

PassReady Prep — NCE Question Bank

Generated: 2026-10-04 · Showing: In review · Total items: 200

How to review

✓ **KEY** marks the correct option. Every option shows why it is right or wrong; check both the key and each distractor.

Mark each item **Approve**, **Revise** (note what to change) or **Retire**. Approved items are published from Admin → NCE Questions.

▀ **Check** notes mark specific facts or citations the writer was less than certain of — verify those first.

Check: one clearly best answer · factual accuracy (theorists, dates, test properties, code sections) · distractors plausible but defensibly wrong · no answer given away by length or wording · references support the key.

NCE domains (NBCC 2023 content outline)

Professional Practice and Ethics 19 · Intake, Assessment, and Diagnosis 19 · Areas of Clinical Focus 47 · Treatment Planning 14 · Counseling Skills and Interventions 48 · Core Counseling Attributes 13 — of 160 scored items

In this copy

Professional Practice and Ethics **24** · Intake, Assessment, and Diagnosis **24** · Areas of Clinical Focus **59** · Treatment Planning **17** · Counseling Skills and Interventions **60** · Core Counseling Attributes **16** · **30** flagged ▀

Professional Practice and Ethics

24 items · NCE weight 12% (19 of 160 scored items)

A 68-year-old client with an intellectual disability lives in a licensed group home. She tells her counselor that night staff often lock the pantry and send her to bed without dinner as punishment. She has lost weight since her last visit. State law names counselors as mandated reporters of abuse and neglect of vulnerable adults. What should the counselor do?

A. Tell the group home administrator, who is responsible for investigating staff

Notifying the facility does not satisfy a statutory duty to report to the designated agency, and the facility has a conflict of interest.

B. Wait until the client shows clear physical injury before making any report

Reporting laws require reasonable suspicion, not proof of injury; withholding food as punishment is itself neglect or abuse.

C. Report the suspected neglect to adult protective services or the named agency ✓ **KEY**

A mandated reporter with reasonable suspicion reports to the agency the statute designates, such as adult protective services.

D. Keep the disclosure confidential unless the client signs a release to share it

Confidentiality yields when law requires disclosure (ACA B.2.a); a release is not needed for a mandated report.

Rationale: Most states require counselors to report suspected abuse, neglect or exploitation of older or vulnerable adults to adult protective services or another designated agency, and the threshold is reasonable suspicion rather than proof. ACA B.2.a recognizes that confidentiality does not apply when legal requirements demand disclosure. Telling the facility alone does not meet the duty.

References: State law — Vulnerable adult / adult protective services mandatory reporting statutes (vary by state) · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve

☐ Revise

☐ Retire

Notes:

A 36-year-old client discloses that an uncle sexually abused her from ages 8 to 11. She does not want to file a report. In passing, she mentions that the uncle now coaches a youth soccer team. How should the counselor approach the question of reporting?

A. Make no report, since child abuse laws cover only victims who are minors now

Some states require or permit reports when the alleged abuser has current access to children, so this is too absolute.

B. Report at once, since abuse of a child must be reported regardless of when it occurred

Many states do not require reports of past abuse disclosed by an adult; the counselor must check the statute.

C. Ask the client to file her own police report before discussing the abuse further

Making the report a condition of counseling ignores client autonomy and does not resolve the counselor's own duty.

D. Check the state statute and consult, weighing that children may be at present risk ✓ **KEY**

The duty depends on state law, and current access to children is the key fact; consultation and the client's involvement guide next steps.

Rationale: Mandated reporting laws are written mainly to protect children who are currently at risk, and states differ on whether a report is required when an adult discloses abuse from childhood. Some states require or allow a report when the alleged abuser still has access to children. The counselor reviews the specific statute, consults, documents the reasoning, and involves the client in the process where possible.

References: State law — Child abuse reporting statutes: adult disclosure of past abuse and current risk to minors · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

► **Check:** Reporting of past abuse disclosed by an adult varies widely by state; confirm the framing that the duty turns on state statute and current risk to children.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has just phoned in a report of suspected child neglect to the state child protection hotline. Which documentation practice is MOST appropriate?

A. Note the date, time, facts reported, agency worker's name and report number ✓ **KEY**

A factual record of what was reported, when and to whom shows that the legal duty was met.

B. Keep no note of the call, so that the family cannot later learn who reported

Failing to document leaves no record that the duty was fulfilled; reporter identity is protected by law.

C. Record the counselor's conclusion about which parent is guilty of the neglect

Records should contain observed facts and disclosures, not conclusions of guilt, which are for investigators.

D. Store the report details only in a personal file kept at the counselor's home

Records belong in the secure clinical record, not in a personal file outside the agency or practice.

Rationale: After a mandated report, the counselor documents objective facts: the date and time of the report, the agency contacted, the name of the person who took it, any reference number, and the information conveyed, including the client's own words where relevant. This record shows the legal duty was met and supports continuity of care. Speculation about guilt does not belong in the note.

References: Wiger (Documentation) — Documenting mandated reports and critical incidents · State law — Child abuse and neglect reporting procedures

☐ Approve ☐ Revise ☐ Retire Notes:

Under the Family Educational Rights and Privacy Act (FERPA), when do the rights to inspect and control a student's education records transfer from the parents to the student?

A. When the student turns 16 and may leave school

FERPA does not tie the transfer of rights to compulsory-attendance age.

B. At age 18, or on entering college at any age ✓ **KEY**

The student becomes an "eligible student" at 18 or on attending a postsecondary institution.

C. On graduating from high school, whatever the age

Graduation alone does not transfer FERPA rights; age 18 or postsecondary attendance does.

D. At age 21, the age set for all federal privacy laws

FERPA sets 18, and there is no single age for all federal privacy laws.

Rationale: FERPA gives parents rights over their child's education records until the student turns 18 or attends a postsecondary institution, at which point the student becomes an "eligible student" and the rights transfer. Schools may still disclose to parents in limited situations, such as when the student is a tax dependent or in a health or safety emergency.

References: FERPA — 34 CFR 99.3 and 99.5: eligible student and transfer of parental rights

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school counselor jots brief notes after student meetings to jog her memory. A parent asks to see "everything the school has" on his son. Under FERPA, which condition would keep the counselor's notes outside the definition of education records?

A. They are typed on a school computer and saved in the student's file

Placing notes in the student file makes them part of the education record.

B. They are shared with the student support team to plan interventions

Once notes are revealed to others, the sole-possession exclusion is lost.

C. They are labeled "confidential" and locked in the main office cabinet

A label and a lock do not change status if others in the office can access them.

D. They are used only as her memory aid and are not shown to anyone else ✓ **KEY**

Sole-possession records kept as a personal memory aid and not shared are excluded from education records.

Rationale: FERPA excludes from "education records" notes that are kept in the sole possession of the maker, used only as a personal memory aid, and not accessible or revealed to any other person except a temporary substitute. Once such notes are shared, placed in the student file, or used to make decisions, they become education records that parents may inspect.

References: FERPA — 34 CFR 99.3 definition of education records: sole-possession exclusion

☐ Approve ☐ Revise ☐ Retire Notes:

A client at a HIPAA-covered clinic reads her records and asks the counselor to change an intake note that lists past alcohol misuse, which she says is wrong. The counselor reviewed the intake carefully and believes the note is accurate and complete. What does the HIPAA Privacy Rule require?

A. Delete the note, because clients control the content of their own records

Clients may request amendment, but the provider is not required to accept a change it finds inaccurate.

B. Refuse to discuss it, because clients may view but not seek changes to records

HIPAA gives individuals an explicit right to request amendment of their records.

C. Answer in writing; if denied, explain why and let her file a disagreement ✓ **KEY**

A denial must be written, give the basis, and tell the client she may submit a statement of disagreement.

D. Amend it only if a supervisor agrees, then send the change to all insurers

Supervisor approval is not the HIPAA standard, and blanket disclosure to insurers violates minimum necessary.

Rationale: Under 45 CFR 164.526, individuals may ask a covered entity to amend their protected health information. The entity may deny the request if the information is accurate and complete, but it must respond in writing within set time limits, state the basis for denial, and explain the right to file a statement of disagreement that is then linked to the record. Disclosures of the amendment follow the minimum necessary standard.

References: HIPAA — 45 CFR 164.526 Amendment of protected health information

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor is licensed in her state, holds the National Certified Counselor (NCC) credential, and is a member of the American Counseling Association. Which ethical standards apply to her professional conduct?

A. The ACA Code, the NBCC Code and her licensing board's rules all apply ✓ **KEY**

Each body binds her through a separate tie: membership, certification and licensure.

B. Only the state board rules, since licensure overrides voluntary codes

Licensure does not cancel obligations she accepted by joining ACA and holding the NCC.

C. Only the NBCC Code, since national certification outranks state law

National certification does not outrank state law; licensure regulates legal practice.

D. Only whichever code is strictest, as the others then fall away entirely

All applicable codes remain in force; she is accountable to each body separately.

Rationale: Counselors are often bound by several codes at once. ACA members agree to follow the ACA Code of Ethics, NCCs and applicants are bound by the NBCC Code of Ethics, and licensees must follow the rules and code adopted by their state board. Each body can act on a violation within its own authority, so the counselor attends to all of them.

References: NBCC Code of Ethics — Application to NCCs and applicants · ACA Code of Ethics — Preamble and Purpose: application to ACA members

☐ Approve ☐ Revise ☐ Retire Notes: _____

A National Certified Counselor receives a public reprimand from her state licensing board for a documentation lapse. She wonders whether she must tell NBCC, since the matter was handled by the state. What does the NBCC Code of Ethics expect?

- A. No notice is needed, as NBCC reviews only complaints that clients file
NBCC can act on licensure discipline as well as on client complaints.
- B. She reports the board action to NBCC within the time the Code sets ✓ **KEY**
The NBCC Code requires NCCs to report disciplinary actions by licensing boards to NBCC.
- C. She tells NBCC only if the board fully revokes or suspends her license
The reporting duty is not limited to revocation or suspension.
- D. She waits for the renewal form and mentions it at her next recertification
The Code sets a reporting window rather than deferring disclosure to renewal.

Rationale: The NBCC Code of Ethics requires NCCs to report to NBCC disciplinary actions taken by licensing or credentialing bodies, along with certain legal matters, within a set period (60 days in recent versions). This lets NBCC decide independently whether action on the certification is warranted.

References: NBCC Code of Ethics — NCC duty to report licensure discipline and legal actions to NBCC

► **Check:** Confirm the current NBCC Code wording on self-reporting duties and the 60-day window cited in the rationale.

☐ Approve ☐ Revise ☐ Retire Notes:

Two months after her father's death, a counselor finds herself tearful and distracted in sessions with several bereaved clients, and she lost track of one client's risk disclosure last week. What is her MOST appropriate course of action?

- A. Keep her full caseload, since staying busy is the best way to work through grief
Continuing unchanged after a missed risk disclosure puts clients at risk of harm.
- B. Quietly close her practice to new and current clients without notice to them
Abrupt closure without notice or transfer amounts to client abandonment.
- C. Seek help and consultation, and limit or transfer cases until she can serve safely ✓ **KEY**
C.2.g directs impaired counselors to seek assistance and limit, suspend or end services as needed.
- D. Disclose her loss to grieving clients so that they can support her in sessions
Using clients for support reverses roles and serves the counselor's needs, not the clients'.

Rationale: ACA C.2.g directs counselors to monitor themselves for signs of impairment, seek assistance for problems that reach the level of impairment, and limit, suspend or terminate professional responsibilities until it is safe to resume. Consultation, personal counseling and a temporary transfer of affected cases protect clients without abandoning them.

References: ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

After three years of working mainly with survivors of violent crime, a counselor notices that she now sees the world as unsafe, distrusts strangers, and has intrusive images from clients' stories. Which term BEST describes this?

A. Vicarious trauma ✓ **KEY**

Vicarious trauma is a lasting change in the helper's beliefs about safety and trust from empathic exposure to trauma.

B. Countertransference

Countertransference is the counselor's reaction to a particular client, often tied to the counselor's own history.

C. Simple job burnout

Burnout is exhaustion and cynicism from workload and conditions, not a shift in trauma-related schemas.

D. Moral distress

Moral distress arises when one cannot act on what one believes is ethically right.

Rationale: Vicarious traumatization, described by McCann and Pearlman, is a cumulative change in the counselor's inner experience and core beliefs, such as safety, trust and control, that results from empathic engagement with clients' trauma. It differs from burnout, which stems from work conditions, and calls for supervision, caseload balance and self-care as part of ethical practice (ACA C.2.g).

References: Gladding (Counseling: A Comprehensive Profession) — Counselor burnout, vicarious trauma and self-care · ACA Code of Ethics — C.2.g Impairment

► **Check:** Confirm the Gladding chapter locator for vicarious trauma vs burnout; the McCann & Pearlman attribution is standard.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants to join the American Counseling Association division that focuses on multicultural counseling and that endorsed the Multicultural and Social Justice Counseling Competencies. Which division should she join?

A. ASGW

The Association for Specialists in Group Work focuses on group work and its best practices.

B. ACES

The Association for Counselor Education and Supervision focuses on training and supervision.

C. IAMFC

The International Association of Marriage and Family Counselors focuses on couple and family work.

D. AMCD ✓ **KEY**

The Association for Multicultural Counseling and Development focuses on multicultural counseling.

Rationale: The Association for Multicultural Counseling and Development (AMCD) is the ACA division devoted to multicultural and diversity issues, and it developed the Multicultural and Social Justice Counseling Competencies with ACA endorsement. ACA divisions each serve a specialty area, such as group work (ASGW), counselor education (ACES) and family counseling (IAMFC).

References: MSJCC (Ratts et al., 2016) — AMCD development and ACA endorsement of the competencies · Gladding (Counseling: A Comprehensive Profession) — Professional organizations and ACA divisions

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator is preparing a slide that matches counseling bodies with their roles. Which pairing is accurate?

A. CACREP: issues licenses that let counselors practice in each state

CACREP accredits graduate programs; it does not license anyone.

B. Chi Sigma Iota: the international honor society of the counseling field ✓ KEY

Chi Sigma Iota is the counseling academic and professional honor society.

C. NBCC: accredits master's and doctoral counseling graduate programs

NBCC certifies individual counselors and administers the NCE; CACREP accredits programs.

D. State licensing board: awards the National Certified Counselor title

The NCC is awarded by NBCC; state boards grant licenses to practice.

Rationale: Chi Sigma Iota is the international honor society for counseling students, professional counselors and counselor educators, promoting scholarship, leadership and excellence. CACREP accredits programs, NBCC grants national certification such as the NCC, and state boards license counselors and enforce state practice law.

References: Gladding (Counseling: A Comprehensive Profession) — Professional organizations: Chi Sigma Iota, NBCC, CACREP and state boards

☐ Approve ☐ Revise ☐ Retire Notes: _____

The California Supreme Court decided Tarasoff v. Regents of the University of California twice, in 1974 and again on rehearing in 1976. How did the 1976 decision change the clinician's obligation?

A. It framed it as a duty to protect, met by warning or other reasonable steps ✓ KEY

Tarasoff II held a duty to use reasonable care to protect the victim, which warning may or may not satisfy.

B. It removed any duty, ruling that therapy privilege outweighs third parties

The court reaffirmed liability, stating that the protective privilege ends where public peril begins.

C. It limited the duty to cases in which the client has already hurt someone

The duty concerns foreseeable future violence against an identifiable victim, not past harm.

D. It required clinicians to call the police after any client voices anger

The duty arises from a serious threat of violence, not ordinary expressions of anger.

Rationale: Tarasoff I (1974) held that a therapist had a duty to warn a foreseeable victim. On rehearing, Tarasoff II (1976) restated this as a broader duty to protect: when a therapist determines, or should determine, that a client poses a serious danger of violence to another, the therapist must use reasonable care to protect the intended victim, which may include warning, notifying police or other steps.

References: State law — Tarasoff v. Regents of the University of California (1974, 1976) and state duty-to-protect law · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor relocates her practice to a new state. A colleague tells her that "duty to warn works the same everywhere because of Tarasoff." What is the MOST accurate response?

A. Tarasoff is a federal ruling, so every state follows the same rule

Tarasoff was a California Supreme Court case, not a federal ruling binding all states.

B. Duty to warn applies only to psychiatrists, not licensed counselors

State duty-to-protect laws commonly cover licensed mental health professionals, including counselors.

C. States differ, so she must learn her new state's duty-to-protect law ✓ **KEY**

Some states mandate action, some permit disclosure, and some have no statute, so she checks local law.

D. The ACA Code sets one duty-to-warn rule that replaces state statutes

The ACA Code defers to legal requirements and does not replace state statutes.

Rationale: Tarasoff was a California decision. States have since adopted different approaches: some impose a mandatory duty to warn or protect, some permit but do not require disclosure, and some have no statute or case law. Counselors must know the law in each state where they practice, including what triggers the duty and what actions discharge it.

References: State law — Duty-to-warn/protect statutes: mandatory, permissive or none · ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements

☐ Approve ☐ Revise ☐ Retire Notes:

The Preamble of the 2014 ACA Code of Ethics lists core professional values of the counseling profession. Which of the following is one of those core values?

A. Protecting the field's public image

Reputation of the profession is not listed among the Preamble's core values.

B. Maximizing cost-effectiveness

Cost-effectiveness is not a core professional value in the Preamble.

C. Deferring to client families

The Code emphasizes client autonomy, not deference to family on treatment.

D. Promoting social justice ✓ **KEY**

Promoting social justice is one of the core professional values listed in the Preamble.

Rationale: The Preamble names the profession's core values: enhancing human development throughout the life span; honoring diversity and embracing a multicultural approach; promoting social justice; safeguarding the integrity of the counselor-client relationship; and practicing in a competent and ethical manner. It also lists the ethical principles of autonomy, nonmaleficence, beneficence, justice, fidelity and veracity.

References: ACA Code of Ethics — Preamble: core professional values and fundamental principles

☐ Approve ☐ Revise ☐ Retire Notes:

A client's partner has twice confronted the counselor in the parking lot after sessions and left threatening voicemails. The counselor no longer feels safe continuing the relationship. Under the ACA Code of Ethics, what may the counselor do?

A. End services by letter right away, giving no referrals, since the threat is not her fault

Even when termination is justified, the counselor offers pretermination counseling and referrals as appropriate.

B. Terminate, since threats from someone close to the client are an accepted reason ✓ **KEY**

A.11.c permits termination when the counselor is in jeopardy of harm by someone in a relationship with the client.

C. Continue indefinitely, because ending services would punish the client for the partner

The Code does not require counselors to keep working while in jeopardy of harm.

D. Invite the partner into joint sessions to resolve the conflict before any decision

Bringing a threatening person into sessions raises risk and is not required before termination.

Rationale: A.11.c states that counselors may terminate when they are in jeopardy of harm by the client or by another person with whom the client has a relationship. Counselors still provide pretermination counseling and recommend other service providers when necessary, and they document the reasons and safety steps taken.

References: ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes: _____

17. nce-s-eth-065 · Professional Counseling Orientation and Ethical Practice · termination and referral · medium · In review

A counselor realizes that a client's severe eating disorder requires specialized care beyond her competence. She offers three referrals to qualified providers, but the client declines all of them and asks to keep seeing her. What does the ACA Code of Ethics direct?

A. Continue, since the client's wish to stay outweighs the competence concern

Client preference does not permit practice outside competence when serious risk is present.

B. Keep working with her but drop eating-related goals from the treatment plan

Ignoring the main clinical problem does not address the risk and is not what the Code directs.

C. Discontinue the counseling relationship once the referrals have been offered ✓ **KEY**

A.11.a states that if clients decline the suggested referrals, counselors discontinue the relationship.

D. Ask a friend with eating-disorder training to coach her by phone each week

Informal coaching is not supervision or consultation and does not establish competence.

Rationale: A.11.a provides that when counselors lack the competence to help a client, they avoid entering or continuing the relationship, know culturally and clinically appropriate referral resources, and suggest them. If the client declines the suggested referrals, the counselor discontinues the relationship, handling the transition with care to avoid abandonment.

References: ACA Code of Ethics — A.11.a Competence Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes: _____

At the final session, a client who immigrated from the Philippines brings her counselor a small box of homemade pastries to thank her, explaining that in her family such gifts express respect. How should the counselor approach this gift?

A. Weigh its value, the client's motive, the relationship and cultural meaning ✓ KEY

A.10.f directs counselors to consider the relationship, monetary value, and the client's and counselor's motives.

B. Decline it politely, since the Code bars all gifts from current clients

The Code recognizes that small tokens of appreciation may be acceptable and does not bar all gifts.

C. Accept it, then offer a gift of equal value back so that the two are even

Reciprocal gift-giving is not required and can blur the professional relationship.

D. Accept it only if the client agrees to put the gift in writing beforehand

The Code sets factors to weigh, not a written agreement, for small gifts.

Rationale: A.10.f recognizes that in some cultures small gifts are a token of respect and gratitude. When deciding whether to accept, counselors consider the therapeutic relationship, the monetary value of the gift, the client's motivation for giving it, and the counselor's motivation for accepting or declining. Refusing a modest, culturally meaningful gift can itself harm the relationship.

References: ACA Code of Ethics — A.10.f Receiving Gifts

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has a guardian's consent to give a career interest inventory to a 17-year-old student. Under the ACA Code of Ethics, what must the counselor explain to the student before the assessment?

A. The raw scoring keys, so she can check the scoring on her own later on

Sharing scoring keys would compromise test security and is not part of informed consent.

B. Nothing until results are in, so that her answers stay honest and unbiased

E.3.a requires the explanation before the assessment, not after.

C. Only the time the inventory will take, since interest tests are low risk

Low risk does not remove the duty to explain the nature, purpose and use of results.

D. Its nature and purpose and who will use the results, in words she grasps ✓ KEY

E.3.a requires explaining nature, purposes and specific use of results in understandable language.

Rationale: E.3.a requires counselors, before an assessment, to explain its nature and purposes and the specific use of results by potential recipients, in terms and language the client can understand. This applies to low-stakes instruments too, and with minors the counselor also involves parents or guardians as appropriate.

References: ACA Code of Ethics — E.3.a Explanation to Clients

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor running a workshop on test anxiety wants to photocopy several items from a published, copyrighted achievement test to use as practice handouts. What is the ethical issue?

A. None, as long as the counselor removes the publisher's name from each page

Removing attribution makes the problem worse; it does not grant permission.

B. Copying items without permission breaks test security and publisher rights ✓ **KEY**

E.10 requires maintaining test security and not reproducing published tests without permission.

C. It is acceptable only if the handouts are collected and shredded at the end

Collecting handouts does not cure unauthorized reproduction or exposure of items.

D. It is allowed for any educational purpose under the fair use of all tests

Exposing secure items can invalidate the test for future takers; fair use does not cover this.

Rationale: E.10 directs counselors to maintain the integrity and security of tests and assessment techniques consistent with legal and contractual obligations, and not to appropriate, reproduce or modify published assessments without acknowledgment and permission from the publisher. Exposing items also threatens the validity of the test for future examinees.

References: ACA Code of Ethics — E.10 Assessment Security

☐ Approve

☐ Revise

☐ Retire

Notes:

21. nce-s-eth-069 · Professional Counseling Orientation and Ethical Practice · supervision and consultation responsibilities · medium · In review

A licensed counselor agrees to provide clinical supervision for a new graduate working toward licensure. Under the ACA Code of Ethics, what should the supervisor do at the start of the relationship?

A. Start case review at once and discuss evaluation only when problems arise

Delaying the discussion of evaluation denies the supervisee informed consent.

B. Ask the supervisee to waive any right to appeal a negative evaluation

Supervisees must be told of due process and appeal mechanisms, not asked to waive them.

C. Review policies, evaluation methods and due process for appeals with her ✓ **KEY**

F.4.a builds informed consent into supervision, including policies and due process appeal.

D. Rely on the graduate program's handbook, since it covers all expectations

Post-degree supervision needs its own informed consent; a school handbook does not cover it.

Rationale: F.4.a requires supervisors to incorporate the principles of informed consent and participation into supervision, informing supervisees of the policies and procedures to which supervisors are to adhere and the mechanisms for due process appeal of individual supervisor actions. Many supervisors put this in a written supervision agreement that also covers evaluation, emergencies and fees.

References: ACA Code of Ethics — F.4.a Informed Consent for Supervision

☐ Approve

☐ Revise

☐ Retire

Notes:

A counselor's close friend asks her to serve as the licensure supervisor for the friend's adult son, who just finished a counseling degree. The counselor is confident she could be fair. What does the ACA Code of Ethics direct?

- A. Accept, provided she documents the friendship in the supervision file
Documentation does not cure the loss of objectivity the Code warns about.
- B. Accept, but have a second supervisor sign his final evaluation as well
A co-signer does not remove the conflict in the ongoing supervisory relationship.
- C. Accept if the son agrees in writing that their personal ties will not matter
Supervisee consent does not resolve the supervisor's inability to stay objective.
- D. Decline, since she may be unable to judge him with full objectivity ✓ KEY
F.3.d bars supervisory relationships with friends or family members where objectivity is at risk.

Rationale: F.3.d states that supervisors are prohibited from engaging in supervisory relationships with individuals with whom they have an inability to remain objective, such as friends or family members. The counselor's own confidence does not settle the matter; the safer course is to decline and help the son find another qualified supervisor.

References: ACA Code of Ethics — F.3.d Friends or Family Members

☐ Approve ☐ Revise ☐ Retire Notes:

A group practice is moving from paper charts to a cloud-based electronic health record. Under the ACA Code of Ethics, what should counselors tell clients about this change?

- A. Nothing, because record storage is an internal business decision only
H.5.a requires informing clients how records are kept electronically.
- B. How records are stored, including encryption and archive retention ✓ KEY
H.5.a lists the type of encryption and security and archival storage as things clients are told.
- C. That electronic records are fully secure and cannot ever be breached
Counselors acknowledge the limits of technology security rather than promising it is perfect.
- D. That clients must sign a waiver of all liability for any data breach
Liability waivers are not part of the Code and do not replace security duties.

Rationale: H.5.a directs counselors to maintain electronic records in accordance with relevant laws and to inform clients how records are maintained electronically, including the type of encryption and security assigned to the records and whether, and for how long, archival storage is kept. HIPAA's Security Rule adds administrative, physical and technical safeguards for electronic records.

References: ACA Code of Ethics — H.5.a Records · HIPAA — Security Rule safeguards for electronic PHI (45 CFR Part 164, Subpart C)

☐ Approve ☐ Revise ☐ Retire Notes:

The mother of a 14-year-old client calls after the third session and asks the counselor to tell her "everything he says," noting that she has legal custody and pays for services. The teen has asked that his talks stay private. What is the counselor's BEST approach?

A. Respect her rights, clarify what will be shared, and keep safety limits clear ✓ KEY

B.5.b asks counselors to respect parental rights while explaining the confidential nature of counseling.

B. Give her full session details, since a parent who pays controls the record

Payment does not entitle a parent to all session content, and full disclosure can damage trust.

C. Refuse to speak with her at all, since the teen asked for complete privacy

Ignoring a custodial parent disregards the parent's legal rights and responsibilities.

D. End counseling, since the parent and teen want different things from it

Differing wishes are common and are worked through, not a reason to terminate.

Rationale: B.5.b directs counselors to inform parents and guardians about the role of counselors and the confidential nature of the counseling relationship, consistent with legal and custodial arrangements, and to respect the inherent rights and responsibilities of parents. Ideally the counselor sets these terms with parent and minor at the outset: general progress may be shared, specific content stays private, and safety concerns are disclosed.

References: ACA Code of Ethics — B.5.b Responsibility to Parents and Legal Guardians

☐ Approve

☐ Revise

☐ Retire

Notes:

Intake, Assessment, and Diagnosis

24 items · NCE weight 12% (19 of 160 scored items)

25. nce-s-ass-049 · Human Growth and Development · differential diagnosis · medium · In review

An 8-year-old is referred for poor concentration, restlessness, and unfinished classwork. His teacher from last year describes him as attentive and settled. The problems began about five months ago, shortly after he witnessed a violent break-in at his home, and he now has nightmares and startles easily. Which finding MOST argues against ADHD as the primary explanation?

A. His restlessness is seen both at home and at school

Symptoms in two or more settings are required for ADHD, so this finding is consistent with ADHD rather than against it.

B. He is a boy referred in the early elementary years

Age and gender describe common referral patterns; they do not help separate ADHD from a trauma response.

C. The symptoms began suddenly after a traumatic event ✓ **KEY**

An abrupt onset after trauma, with no earlier history and with intrusion and arousal symptoms, points toward a trauma-related disorder.

D. His classwork is left unfinished, not done poorly

Failing to finish schoolwork is itself a listed ADHD inattention symptom, so it does not argue against ADHD.

Rationale: ADHD is a neurodevelopmental disorder with a persistent pattern that typically shows up early and across settings. When attention problems and restlessness appear abruptly after a traumatic event in a child who was previously attentive, and they come with nightmares and an exaggerated startle response, a trauma- and stressor-related disorder better explains the picture. Hyperarousal and intrusive memories can look like inattention and hyperactivity, so onset timing and trauma history are key differentiators.

References: DSM-5-TR — Attention-deficit/hyperactivity disorder: Differential Diagnosis · SAMHSA TIP 57 — Trauma-related symptoms that resemble other disorders

☐ Approve ☐ Revise ☐ Retire Notes:

A 35-year-old client reports three weeks of depressed mood, loss of interest, guilt, and poor concentration. His partner says he has been "snappy and irritable" with everyone. He sleeps about four hours a night but wakes exhausted and drags through the day. He reports no past periods of elevated mood or unusual energy. Which diagnosis is BEST supported?

A. Major depressive disorder, single episode ✓ KEY

He has at least five depressive symptoms for two weeks or more; irritability and insomnia with fatigue fit a depressive episode.

B. Bipolar II disorder, current episode depressed

Bipolar II requires at least one past hypomanic episode, and he reports no periods of elevated mood or increased energy.

C. Cyclothymic disorder with depressive features

Cyclothymic disorder requires at least two years of fluctuating subthreshold symptoms, not a three-week episode.

D. Disruptive mood dysregulation disorder

DMDD is diagnosed only in children and adolescents (first diagnosis between ages 6 and 18), with onset before age 10.

Rationale: Irritability is common in depression and does not by itself suggest bipolarity. Hypomania requires a distinct period of abnormally elevated, expansive, or irritable mood together with increased activity or energy. This client sleeps little but feels exhausted, which is insomnia, not the decreased need for sleep seen in hypomania. With no history of hypomanic episodes, the depressive episode is best diagnosed as major depressive disorder.

References: DSM-5-TR — Major depressive disorder: Differential Diagnosis; hypomanic episode Criteria A-B

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old client in a severe depressive episode believes she is being punished for terrible sins and that her organs are rotting. Her family reports that these beliefs appeared only after the depression began, and she has never had psychotic symptoms while her mood was stable. Which diagnosis BEST fits?

A. Schizophrenia, first episode, acute

Psychosis that occurs only during mood episodes points to a mood disorder; schizophrenia is excluded when mood episodes explain it.

B. Schizoaffective disorder, depressive type

Schizoaffective disorder requires delusions or hallucinations for two weeks or more without a major mood episode.

C. Delusional disorder, somatic type

Delusional disorder is not diagnosed when delusions occur only during major depressive or manic episodes.

D. MDD, severe, with psychotic features ✓ KEY

Delusions confined to a major depressive episode are coded with the psychotic features specifier.

Rationale: The deciding question is when the psychotic symptoms occur. If delusions or hallucinations happen only during a major depressive episode, the diagnosis is major depressive disorder with psychotic features; here the content (guilt, punishment, bodily decay) is mood-congruent. Schizoaffective disorder requires a period of at least two weeks of psychosis without a major mood episode, and schizophrenia is ruled out when mood episodes account for the psychosis.

References: DSM-5-TR — Depressive disorders: with psychotic features specifier; schizoaffective disorder Criterion B

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old is brought in after four days of paranoid beliefs and hearing voices. He reports using methamphetamine daily for the past month and stopping two days ago. He has no prior psychiatric history. Which approach to diagnosis is MOST appropriate?

A. Diagnose schizophrenia, as he has delusions and hallucinations

Schizophrenia requires continuous signs for six months and is excluded when a substance explains the symptoms.

B. Consider a substance-induced psychosis and see if it clears ✓ KEY

Onset during heavy stimulant use suggests a substance-induced disorder; persistence well after stopping would point elsewhere.

C. Diagnose brief psychotic disorder, as it has lasted under a month

Brief psychotic disorder is not diagnosed when the disturbance is attributable to a substance.

D. Postpone any psychosis assessment until a 90-day program ends

Psychosis needs prompt assessment for safety and care; waiting months is not a diagnostic strategy.

Rationale: Stimulants such as methamphetamine can produce paranoia and hallucinations. DSM-5-TR asks whether the symptoms began during or soon after substance use and whether they persist for a substantial period (for example, about a month) after intoxication or withdrawal ends. Symptoms that clear with abstinence support a substance/medication-induced psychotic disorder; symptoms that persist, or a history of psychosis without use, suggest an independent psychotic disorder.

References: DSM-5-TR — Substance/medication-induced psychotic disorder: Criterion C and Differential Diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor gives a client a form with stems such as "I regret..." and "My father..." and asks her to finish each one in her own words. Which type of instrument is this?

A. A sentence completion test such as the Rotter ✓ KEY

Incomplete sentence stems that the client finishes freely define the sentence completion method, such as the Rotter Incomplete Sentences Blank.

B. An objective personality inventory like the MMPI-3

The MMPI-3 uses fixed true/false items scored against scales, not open-ended responses.

C. A word association technique in the style of Jung

Word association asks for the first word that comes to mind after a single stimulus word, not completing sentences.

D. A symptom checklist such as the SCL-90-R measure

The SCL-90-R asks clients to rate how much listed symptoms have bothered them, a structured self-report format.

Rationale: Sentence completion tests are often called semi-structured projective techniques: the stem gives some structure, but the client supplies the content, which may reveal attitudes, conflicts, and concerns. The Rotter Incomplete Sentences Blank is the best-known example. Like other projectives, responses are interpreted with caution and alongside other data.

References: Hays (Assessment) — Projective personality assessment: sentence completion techniques

☐ Approve ☐ Revise ☐ Retire Notes:

As part of an evaluation for possible depression, a school counselor asks a 9-year-old to draw a house, a tree, and a person. Afterward, a colleague proposes diagnosing depression because the tree in the drawing has no leaves. Which response BEST reflects sound practice with this technique?

A. Accept the sign, as drawing tests are well validated for mood

Individual drawing signs have weak and inconsistent research support for diagnosing specific disorders.

B. Rescore the drawing with a longer and more detailed checklist

A longer checklist does not fix the underlying validity problem of reading diagnoses from single drawing features.

C. Treat the drawing as a hypothesis to check against other data ✓ KEY

Projective drawings are best used for rapport and hypothesis generation, confirmed by interviews, ratings, and other measures.

D. Have the child draw again and diagnose if the sign is repeated

Repeating the task does not make the sign a valid indicator of depression; the diagnosis still lacks evidence.

Rationale: The House-Tree-Person technique, developed by Buck, asks the client to draw and then talk about the drawings. Interpreting single features (such as a bare tree) as signs of a disorder has limited reliability and validity support. Projectives are best used to build rapport and generate hypotheses that are then checked against interview data, behavior ratings, collateral reports, and standardized measures.

References: Hays (Assessment) — Projective drawings (House-Tree-Person): uses and psychometric limitations

☐ Approve ☐ Revise ☐ Retire Notes:

Over the past year, a client's drinking has met these DSM-5-TR criteria: drinking more than intended, unsuccessful efforts to cut down, craving, continued use despite arguments with his spouse about it, and tolerance. He reports no withdrawal symptoms. Which severity specifier applies?

A. Mild, as no withdrawal symptoms are reported

Withdrawal is not required for any severity level; severity is set by how many criteria are met.

B. Moderate, based on the number of criteria met ✓ KEY

Five criteria fall in the moderate range (4-5 criteria); mild is 2-3 and severe is 6 or more.

C. Severe, as tolerance shows physical dependence

Tolerance is one criterion among eleven; it does not by itself raise the severity level.

D. Unspecified, until withdrawal is also confirmed

An alcohol use disorder can be diagnosed and rated without withdrawal; at least two criteria in 12 months are needed.

Rationale: DSM-5-TR substance use disorders have 11 criteria; at least two within a 12-month period are needed for the diagnosis. Severity is rated by count: mild (2-3 symptoms), moderate (4-5), and severe (6 or more). Physiological features such as tolerance and withdrawal are not required, and neither one alone determines severity.

References: DSM-5-TR — Alcohol use disorder: Criteria A and severity specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

In an integrated primary care clinic, a 44-year-old patient screens positive on the AUDIT-C. What does this result BEST support as the next step?

A. Diagnosing alcohol use disorder, mild severity

A screening tool flags possible risk; it cannot establish whether DSM-5-TR criteria are met.

B. Referring the patient to inpatient detoxification

Level of care depends on a full assessment, such as of withdrawal risk, not on a positive screen alone.

C. Placing the patient in a relapse prevention group

Treatment placement before any diagnostic assessment skips the step that tells what care is needed.

D. Fuller assessment to see if a disorder exists ✓ KEY

A positive screen calls for further evaluation, such as the full AUDIT and a diagnostic interview.

Rationale: The AUDIT-C is the three consumption items of the 10-item AUDIT. Like all screeners, it is built to identify people who may have a problem, trading some precision for speed. A positive result is followed by a fuller assessment (for example, the complete AUDIT, a clinical interview against DSM-5-TR criteria, and review of consequences) before diagnosis or placement.

References: Hays (Assessment) — Substance use screening (AUDIT, AUDIT-C) versus diagnostic assessment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a high school health center wants a brief substance use screening tool that was developed and validated specifically for adolescents. Which instrument BEST fits this need?

A. CRAFFT ✓ KEY

The CRAFFT was developed for adolescents and asks about situations such as riding in a car with an impaired driver.

B. CAGE questionnaire

The CAGE was developed for adults and focuses on lifetime alcohol problems, not adolescent use.

C. Michigan Alcoholism Screening Test

The MAST was developed with adults and screens for alcohol problems only.

D. Addiction Severity Index

The ASI is a longer adult interview measuring problem severity, not a brief adolescent screen.

Rationale: The CRAFFT is a brief screen for adolescent substance use risk. Its name comes from its key words: Car, Relax, Alone, Forget, Friends, Trouble. It covers alcohol and other drugs and fits settings such as schools and pediatric care. Like any screen, a positive result leads to further assessment rather than diagnosis.

References: Hays (Assessment) — Substance use screening with adolescents: CRAFFT

☐ Approve ☐ Revise ☐ Retire Notes:

A parent brings a 6-year-old for an intake because of delayed speech and difficulty playing with peers. Which part of the biopsychosocial history is MOST important to gather in detail at this point?

A. The parent's own job history and career plans

Parent employment can matter for context, but it does not address the child's developmental concerns.

B. The child's scores on recent reading tests

At age 6, formal reading data are limited and do not explain early speech and social delays.

C. Developmental milestones and early health ✓ **KEY**

Timing of speech, motor, and social milestones, plus prenatal and early medical history, is central to these concerns.

D. The family's faith practices and affiliation

Spiritual context can be relevant to a full history, but it is not the priority for speech and social delays.

Rationale: For young children, the developmental history is a core section of the biopsychosocial assessment. When to first words, phrases, walking, and social play emerged, and any prenatal, birth, hearing, or medical issues, help distinguish typical variation from conditions such as language disorder, hearing loss, or autism spectrum disorder, and guide referrals.

References: Hays (Assessment) — Clinical interview: developmental and biopsychosocial history · Berk (Development Through the Lifespan) — Language and social development in early childhood

☐ Approve ☐ Revise ☐ Retire Notes:

During a biopsychosocial intake for her first episode of depression, a 22-year-old mentions that her mother was hospitalized for bipolar disorder. Why is this family history clinically important?

A. It confirms that the client has bipolar disorder herself

Family history raises risk but cannot establish a diagnosis; the client's own history must meet criteria.

B. It raises concern for bipolarity, so screening is key ✓ **KEY**

A first-degree relative with bipolar disorder increases risk, so past hypomania and future mood shifts need careful screening.

C. It is not relevant once the client is an adult living alone

Family psychiatric history keeps its relevance to risk regardless of the client's age or living situation.

D. It shows that the client's depression is mostly learned

Family history reflects genetic and environmental influences; it does not show that symptoms were learned.

Rationale: Family psychiatric history is a standard part of the biopsychosocial assessment. Bipolar disorder is strongly heritable, so a depressed client with a first-degree relative who has bipolar disorder warrants careful questioning about past hypomanic or manic symptoms and monitoring over time. The information also matters for any referral for medication evaluation, because antidepressants alone may carry risk in bipolar disorder.

References: DSM-5-TR — Bipolar I disorder: Risk and Prognostic Factors (genetic and physiological) · Hays (Assessment) — Clinical interview: family history

☐ Approve ☐ Revise ☐ Retire Notes:

Near the end of a biopsychosocial interview, a counselor asks a 58-year-old client about his friendships, his faith community, times he coped well in the past, and things he does well. What is the main purpose of gathering this information?

A. To estimate whether he qualifies for disability benefits

Benefit eligibility depends on functional limits and outside rules, not on a strengths inventory.

B. To screen him for grandiosity and inflated self-esteem

Asking about strengths is not a mania screen; grandiosity is assessed through mood history and the MSE.

C. To set the DSM-5-TR severity specifier for his diagnosis

Severity specifiers depend on symptom criteria, not on the client's supports or strengths.

D. To find resources that the treatment plan can build on ✓ **KEY**

Strengths, supports, and past coping successes are resources that inform goals and interventions.

Rationale: A complete biopsychosocial history covers not only problems and risks but also strengths, social supports, spiritual resources, and past coping. These protective factors shape a realistic, collaborative treatment plan, can support safety planning, and convey to the client that the counselor sees the whole person rather than a list of deficits.

References: Wiger (Documentation) — Biopsychosocial assessment: strengths and resources

☐ Approve

☐ Revise

☐ Retire

Notes:

When structuring an initial intake interview, which sequence is generally recommended for exploring the client's concerns?

A. Begin with closed questions on the most sensitive topics

Starting with sensitive, closed questions before rapport tends to limit disclosure and raise anxiety.

B. Ask about trauma history before the presenting problem

The client's reason for coming usually comes first; detailed trauma questions follow once rapport is built.

C. Start with open questions, then narrow to specifics ✓ **KEY**

This funnel approach lets the client tell the story in their own words, then fills gaps with targeted questions.

D. Read a full symptom checklist before any open question

Leading with a checklist structures the interview around the form rather than the client's concerns.

Rationale: Most intake models use a funnel: after informed consent, the counselor invites the client to describe the presenting concern in their own words with open-ended questions, then narrows to specific questions about onset, duration, severity, and history. This builds rapport, captures the client's view of the problem, and still yields the detail needed for diagnosis and planning.

References: Hays (Assessment) — Clinical interviewing: structure and sequencing of the intake · Gladding (Counseling: A Comprehensive Profession) — The initial interview

☐ Approve

☐ Revise

☐ Retire

Notes:

An agency is deciding between a fully structured diagnostic interview and an unstructured clinical interview for its intakes. Which statement accurately compares the two approaches?

A. Structured interviews yield higher interrater reliability ✓ KEY

Fixed wording and order reduce variation between interviewers, which raises agreement on diagnoses.

B. Unstructured interviews are less open to clinician bias

Unstructured interviews depend more on clinician judgment, which makes them more open to bias.

C. Structured interviews let clinicians freely vary questions

Freedom to vary questions describes unstructured or semi-structured interviews, not fully structured ones.

D. Unstructured interviews are too unreliable to be of use

Unstructured interviews offer flexibility and rapport and remain widely used, despite lower reliability.

Rationale: Fully structured interviews ask set questions in a set order with set scoring, so different interviewers reach more similar conclusions (higher reliability). The tradeoff is less flexibility to follow the client's lead. Unstructured interviews offer flexibility and rapport but rely heavily on the clinician, which lowers reliability and leaves more room for bias. Semi-structured interviews fall between the two.

References: Hays (Assessment) — Clinical interviews: structured, semi-structured and unstructured formats

☐ Approve ☐ Revise ☐ Retire Notes:

During a mental status examination, a client correctly gives her name and says she is in a hospital, but she cannot state the month, the year, or the day of the week. How should orientation be recorded?

A. Fully oriented in all three spheres

She cannot give the date, so she is not oriented to time.

B. Oriented to person and place, not time ✓ KEY

She knows who she is and where she is but not the date, so she is oriented to person and place only.

C. Oriented to time and place, not person

She correctly gave her own name, so orientation to person is intact.

D. Disoriented in all three spheres

She is oriented to both person and place, so she is not fully disoriented.

Rationale: Orientation in the MSE is usually checked for person, place, and time (sometimes also situation) and recorded as, for example, "oriented x2." Orientation to time is typically the first to be lost in delirium and dementia, while orientation to person is usually the last. Any disorientation warrants attention to possible medical or neurocognitive causes.

References: Hays (Assessment) — Mental status examination: sensorium and orientation

☐ Approve ☐ Revise ☐ Retire Notes:

A 39-year-old client says, "I know my drinking is out of control, and it's hurting my kids." Later in the session she mentions that she drove her children to school that morning after drinking. How are insight and judgment BEST described in the mental status examination?

A. Insight is poor, but judgment is good

She recognizes the problem and its effects, so insight is not poor; driving after drinking is not good judgment.

B. Insight and judgment are both intact

Driving her children after drinking shows impaired judgment, despite her awareness.

C. Insight is absent, and judgment is poor

She names the problem and its harm to her children, so insight is present, not absent.

D. Insight fair, but judgment impaired ✓ KEY

She understands the problem (insight) but made a dangerous decision (judgment), so the two are rated differently.

Rationale: In the MSE, insight is the client's awareness that a problem exists and understanding of its causes and effects. Judgment is the ability to make sound decisions and anticipate consequences, best inferred from real recent behavior. The two can diverge: a client may describe a problem accurately yet still act in risky ways, as here.

References: Hays (Assessment) — Mental status examination: insight and judgment

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old client reports a plan to overdose this weekend on medication she has been saving and says she intends to act on it. She adds that her two children "are the reason I'm still here." How should the counselor weigh her children as a protective factor?

A. Note it, but treat risk as high and act on safety now ✓ KEY

Protective factors matter, but a plan with intent and means signals high acute risk that needs immediate action.

B. View it as reducing her risk to low; plan to meet next week

A single protective factor does not offset a specific plan, stated intent, and access to means.

C. Have her sign a no-harm contract, then end the session

No-suicide contracts are not supported as protection and do not address the plan or her access to means.

D. Set it aside, since protective factors do not change risk

Protective factors are part of a sound formulation and can be used in safety planning; they are not ignored.

Rationale: Suicide risk formulation weighs risk factors, warning signs, and protective factors together, but protective factors do not cancel out acute warning signs. Ideation with a specific plan, stated intent, and available means indicates high acute risk and calls for immediate safety steps, such as restricting means, involving supports, and arranging a higher level of care as needed. Her reasons for living can be built into the safety plan.

References: C-SSRS — Suicidal ideation with specific plan and intent · Stanley-Brown SPI — Reasons for living and means restriction

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is assessing an adult client's risk of harming others. Which factor is generally considered the strongest single predictor of future violent behavior?

A. Having any diagnosed mental disorder

Most people with mental disorders are not violent; diagnosis alone is a weak predictor.

B. A score below average on an IQ test

Intelligence test scores are not a strong or direct predictor of violence.

C. A history of past violent behavior ✓ KEY

Past violence is the most consistently supported predictor of future violence.

D. Living alone, away from family members

Living alone is not an established predictor of violence toward others.

Rationale: In violence risk assessment, past behavior is the best predictor of future behavior: a history of violence, especially recent, frequent, or severe violence, carries the most weight. Other factors that raise risk include substance use, stated threats or intent, access to weapons, and certain active symptoms, such as command hallucinations or paranoid beliefs about a specific person.

References: Hays (Assessment) — Risk assessment: violence and harm to others

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old has had delusions, hallucinations, and disorganized speech for two months. The symptoms continue, and it is not yet known whether he will recover within six months. Which diagnosis is MOST appropriate now?

A. Schizophrenia, first episode

Schizophrenia requires continuous signs of disturbance for at least six months.

B. Brief psychotic disorder

Brief psychotic disorder lasts at least one day but less than one month.

C. Schizoaffective disorder

No major mood episode is described, which schizoaffective disorder requires.

D. Schizophreniform disorder (provisional) ✓ KEY

Symptoms of one to six months with recovery not yet known call for a provisional schizophreniform diagnosis.

Rationale: DSM-5-TR uses "provisional" when there is a strong presumption that full criteria will be met but not enough information is yet available. Schizophreniform disorder lasts at least one month but less than six. When the diagnosis must be made before recovery is known, it is labeled provisional; if symptoms reach six months, the diagnosis changes to schizophrenia.

References: DSM-5-TR — Schizophreniform disorder: Diagnostic Criteria B; provisional diagnosis (Use of the Manual)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor on a school evaluation team is documenting autism spectrum disorder for a 7-year-old using DSM-5-TR. How are the severity levels for this diagnosis assigned?

A. As a single overall level based on the child's IQ score

Intellectual impairment is recorded with its own specifier; it does not set autism severity.

B. Separately for social communication and restricted behavior ✓ **KEY**

Each of the two core domains receives its own level, from requiring support to requiring very substantial support.

C. By the total count of criteria the child meets across domains

Autism severity reflects the support needed, not a criteria count as in substance use disorders.

D. By how early in life the symptoms were first noticed by parents

Age of recognition is part of the history, but it does not determine the severity level.

Rationale: For autism spectrum disorder, DSM-5-TR rates current severity separately for social communication and for restricted, repetitive behaviors, using Level 1 ("requiring support"), Level 2 ("requiring substantial support"), and Level 3 ("requiring very substantial support"). The levels can differ between domains and can change over time. Accompanying intellectual or language impairment is recorded with separate specifiers.

References: DSM-5-TR — Autism spectrum disorder: severity levels and specifiers

☐ Approve ☐ Revise ☐ Retire Notes: _____

A clinic wants a single individually administered intelligence test it can use with clients ranging from a 4-year-old to a 70-year-old. Which instrument covers this full age range?

A. Wechsler Intelligence Scale for Children-V

The WISC-V is normed for school-age children (about 6 to 16), not preschoolers or older adults.

B. Wechsler Preschool and Primary Scale-IV

The WPPSI-IV covers young children (about 2½ to 7), not adults.

C. Stanford-Binet Intelligence Scales (SB5) ✓ **KEY**

The SB5 is normed from age 2 through 85 and older, covering both clients.

D. Wechsler Adult Intelligence Scale-IV

The WAIS-IV starts at age 16, so it cannot be used with a 4-year-old.

Rationale: The Stanford-Binet Intelligence Scales, Fifth Edition (SB5), spans roughly ages 2 to 85+ and measures five factors (fluid reasoning, knowledge, quantitative reasoning, visual-spatial processing, and working memory) in both verbal and nonverbal domains. The Wechsler scales instead use separate tests by age band: WPPSI for preschoolers, WISC for school-age children, and WAIS for older adolescents and adults.

References: Hays (Assessment) — Intelligence testing: Stanford-Binet Fifth Edition and the Wechsler scales

► **Check:** Age ranges stated approximately (SB5 2-85+, WISC-V 6-16, WPPSI-IV 2:6-7:7, WAIS-IV 16-90); a WAIS-5 has since been published with a similar adult range. SME to confirm editions.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 10-year-old's WISC-V Full Scale IQ is 112, while her reading standard score on the Woodcock-Johnson IV Tests of Achievement is 78. Both scales have a mean of 100 and a standard deviation of 15. What has this pattern traditionally been taken to suggest?

A. A possible specific learning disability in reading ✓ KEY

A large gap between average-or-better ability and low achievement is the classic ability-achievement discrepancy.

B. Mild intellectual disability that affects reading

An IQ of 112 is above average, which rules out intellectual disability.

C. Invalid testing, as the two scores should match

Ability and achievement measure different things; a gap is meaningful, not proof of invalid testing.

D. Above-average reading skill for a child of her age

A standard score of 78 is about 1.5 SD below the mean, well below average.

Rationale: The ability-achievement discrepancy model compares an intelligence score (such as WISC-V FSIQ) with an achievement score (such as on the WIAT or Woodcock-Johnson). A gap of more than two standard deviations, as here (34 points), was long used to identify a specific learning disability. IDEA 2004 no longer requires schools to use a discrepancy and permits response-to-intervention approaches, and DSM-5-TR does not require a discrepancy.

References: Hays (Assessment) — Achievement testing: ability-achievement discrepancy and learning disabilities

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor wants to use an English-language anxiety inventory with Vietnamese-speaking clients and asks a bilingual staff member to translate it word for word. What is the MOST important limitation of this plan?

A. Any translated test loses its reliability completely

Translation does not erase reliability by definition; it must be re-examined in the new version.

B. Translation alone does not ensure equivalence or norms ✓ KEY

Items may change meaning across languages, and English-speaking norms may not apply to the new group.

C. Clients must take it in English to compare to the norms

Testing in a client's weaker language threatens validity; it is not the solution.

D. Anxiety cannot be assessed across different cultures

Anxiety can be assessed cross-culturally with suitably adapted and validated measures.

Rationale: Culturally fair assessment requires more than literal translation. Adapted tests need linguistic, conceptual, and metric equivalence, usually established through processes such as back-translation and review by bilingual experts, followed by evidence of reliability and validity with the new population. Scores should also be interpreted against appropriate norms; ACA Code E.8 calls for caution when using instruments normed on other populations.

References: ACA Code of Ethics — E.8 Multicultural Issues/Diversity in Assessment · Hays (Assessment) — Culturally fair assessment: test translation and equivalence

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old client of Puerto Rican heritage describes an episode at her father's funeral in which she screamed uncontrollably, trembled, felt heat rising in her chest, and briefly felt detached, then felt like herself again. Which approach BEST reflects culturally fair assessment?

A. Diagnose panic disorder based on the physical symptoms she reports

One episode tied to a stressful event does not meet panic disorder criteria, and cultural meaning is not explored.

B. Record it as a dissociative disorder due to the brief detachment

Brief detachment within a culturally recognized distress episode does not establish a dissociative disorder.

C. Note it as a psychotic episode given the loss of control described

Nothing in the episode suggests delusions or hallucinations; loss of control is not psychosis.

D. Explore it as a possible cultural idiom of distress before labeling ✓ **KEY**

The pattern matches ataque de nervios, a cultural concept of distress, so its meaning should be explored first.

Rationale: DSM-5-TR describes ataque de nervios as a cultural concept of distress common among people of Latin American, and especially Caribbean, heritage. It often follows a stressful family event and can include uncontrollable shouting, crying, trembling, heat rising to the head or chest, and brief dissociation, with a quick return to usual functioning. Exploring the client's own explanation, for example with the Cultural Formulation Interview, guards against misdiagnosing culturally shaped distress.

References: DSM-5-TR — Culture and Psychiatric Diagnosis: cultural concepts of distress (ataque de nervios) · Sue & Sue (Counseling the Culturally Diverse) — Culturally appropriate assessment and diagnosis

☐ Approve

☐ Revise

☐ Retire

Notes:

Areas of Clinical Focus

59 items · NCE weight 29% (47 of 160 scored items)

49. nce-s-clf-119 · Assessment and Testing · depressive disorders · medium · In review

A 31-year-old says that in the week before her period she becomes intensely irritable, tearful, and anxious and argues with her partner. The symptoms ease within a few days after menses begins and are gone the following week. She recalls this pattern for about a year. Under DSM-5-TR, what is needed to CONFIRM a diagnosis of premenstrual dysphoric disorder?

A. Prospective daily symptom ratings over at least two symptomatic cycles ✓ KEY

DSM-5-TR requires confirmation by prospective daily ratings during at least two symptomatic cycles; the diagnosis may be made provisionally before then.

B. Blood tests showing hormone levels outside the expected range for her age

No laboratory test confirms PMDD; hormone levels are typically within normal limits.

C. Her partner's report confirming the same pattern over the previous year

Collateral reports can help, but the criterion calls for the client's own prospective daily ratings.

D. Evidence that she has no co-occurring depressive or anxiety disorders

PMDD may co-occur with other disorders; it must simply not be merely an exacerbation of another disorder.

Rationale: Premenstrual dysphoric disorder requires at least five symptoms, including at least one marked affective symptom, in the final week before menses in most cycles of the past year, improving within a few days after onset and becoming minimal or absent in the week after. Because retrospective recall overstates cyclic patterns, DSM-5-TR requires confirmation by prospective daily ratings during at least two symptomatic cycles, although a provisional diagnosis may be given first.

References: DSM-5-TR — Premenstrual Dysphoric Disorder, Criterion G (prospective daily ratings)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 76-year-old is referred for "memory problems" after several months of low mood and withdrawal. During a cognitive screen, which observation MOST suggests that depression, rather than a neurodegenerative neurocognitive disorder, is driving her poor performance?

A. She works hard at each task and covers missed items with plausible guesses

Effortful attempts with confabulated or near-miss answers are more typical of a neurodegenerative disorder.

B. She quickly answers "I don't know" and stresses how poorly she is doing ✓ **KEY**

Low effort, quick "don't know" answers and highlighting of her own failures are classic signs of depression-related cognitive impairment.

C. Her family dates a slow, steady decline back over the past several years

An insidious, gradual course over years points toward a neurodegenerative process such as Alzheimer's disease.

D. She seems unconcerned, and her daughter notices more lapses than she does

Reduced awareness of deficits, with family noticing more than the client, is more typical of neurocognitive disorder.

Rationale: Depression in later life can produce cognitive complaints (sometimes called pseudodementia). Clients with depression tend to report and emphasize their deficits, give up quickly with "I don't know" answers, and show a more recent onset tied to mood change, whereas clients with neurodegenerative disorders often try hard, minimize or are unaware of deficits, and have a gradual course. The distinction guides referral for both mood treatment and a medical workup.

References: DSM-5-TR — Major or Mild Neurocognitive Disorder: differential diagnosis (major depressive disorder) · Berk (Development Through the Lifespan) — Late adulthood: depression and cognitive decline

► **Check:** The effort/awareness contrasts are classic clinical teaching ("pseudodementia"), not DSM-5-TR criteria; SME to confirm the Berk chapter locator.

☐ Approve ☐ Revise ☐ Retire Notes:

Over the past 12 months, a client with bipolar I disorder has had one manic episode, two major depressive episodes, and one hypomanic episode, each separated by a period of remission. Which DSM-5-TR specifier applies?

A. With mixed features

Mixed features describes opposite-pole symptoms occurring within a single episode, not the number of episodes.

B. With seasonal pattern

Seasonal pattern requires a regular link between episodes and a time of year, which is not described.

C. With rapid cycling ✓ **KEY**

Rapid cycling is specified when at least four mood episodes occur within 12 months.

D. With psychotic features

No delusions or hallucinations are described.

Rationale: The rapid cycling specifier applies to bipolar I or II disorder when at least four mood episodes (manic, hypomanic, or major depressive) occur in the previous 12 months. Episodes must be demarcated by partial or full remission of at least 2 months or by a switch to an episode of the opposite polarity. Rapid cycling is linked to a poorer prognosis and has treatment implications.

References: DSM-5-TR — Specifiers for Bipolar and Related Disorders: with rapid cycling

☐ Approve ☐ Revise ☐ Retire Notes:

For 8 days, a 25-year-old has had elevated mood, little need for sleep, and racing thoughts. She still works her shifts, but she now believes a television news anchor is sending her coded messages. A colleague suggests hypomania because her work has not suffered. Which statement is MOST accurate?

A. Hypomania fits, since her job performance has stayed largely intact

Preserved work functioning does not override the rule that psychotic features make an episode manic.

B. No diagnosis is possible until symptoms have lasted at least 2 weeks

Two weeks is the duration for a major depressive episode; mania requires 1 week, which is met.

C. Her beliefs point to schizophrenia rather than to any mood episode

Psychotic symptoms limited to a mood episode are part of the mood disorder, not evidence of schizophrenia.

D. Psychotic features mean the episode cannot be classed as hypomanic ✓ KEY

DSM-5-TR states that if psychotic features are present, the episode is, by definition, manic.

Rationale: A hypomanic episode is not severe enough to cause marked impairment or require hospitalization, and it has no psychotic features. DSM-5-TR states that if psychotic features are present, the episode is by definition manic. Here the 1-week duration for mania is met and the delusion makes the episode a manic episode with psychotic features, supporting bipolar I disorder.

References: DSM-5-TR — Bipolar I Disorder: Manic Episode Criterion C; Hypomanic Episode Criterion E

☐ Approve ☐ Revise ☐ Retire Notes:

For 9 months, a 50-year-old has been convinced he has undiagnosed heart disease despite normal cardiac workups. He has few physical symptoms but checks his pulse many times a day and reads medical websites for hours. He reports little worry about other areas of life. Which diagnosis BEST fits?

A. Illness anxiety disorder ✓ KEY

Preoccupation with having a serious illness for at least 6 months, with absent or mild somatic symptoms and excessive checking, fits illness anxiety disorder.

B. Generalized anxiety disorder

GAD involves worry about many areas of life, whereas his worry is focused on having one serious disease.

C. Somatic symptom disorder

Somatic symptom disorder centers on distressing somatic symptoms; his symptoms are minimal.

D. Panic disorder

No recurrent unexpected panic attacks are described.

Rationale: Illness anxiety disorder involves preoccupation with having or acquiring a serious illness for at least 6 months, with somatic symptoms absent or only mild, high health anxiety, and excessive health-related behaviors (care-seeking type) or maladaptive avoidance (care-avoidant type). GAD is distinguished by worry spanning multiple events or activities, of which health may be only one.

References: DSM-5-TR — Illness Anxiety Disorder: criteria and differential diagnosis (GAD)

☐ Approve ☐ Revise ☐ Retire Notes:

A client with PTSD has sudden surges of intense fear with a racing heart, sweating, and trembling whenever she encounters reminders of the assault. She does not worry about having attacks out of the blue and has not changed her routine to prevent them. How should these attacks be documented?

A. Add a separate diagnosis of panic disorder

Panic disorder requires recurrent unexpected attacks plus a month of worry or behavior change, neither of which is present.

B. Replace PTSD with panic disorder with agoraphobia

The attacks are cued by trauma reminders and agoraphobia is not described; DSM-5-TR also diagnoses panic disorder and agoraphobia separately.

C. Record PTSD with the "with panic attacks" specifier ✓ KEY

DSM-5-TR allows "with panic attacks" as a specifier for any mental disorder in which panic attacks occur.

D. Leave them out, as expected attacks are not recorded

Expected panic attacks can be recorded through the specifier; they are clinically meaningful.

Rationale: In DSM-5-TR, panic attacks can be expected or unexpected and are not themselves a mental disorder. When they occur in the context of another disorder, the "with panic attacks" specifier may be added (for example, PTSD with panic attacks). Panic disorder is reserved for recurrent unexpected attacks followed by at least 1 month of persistent concern or maladaptive behavior change.

References: DSM-5-TR — Panic Attack Specifier

☐ Approve ☐ Revise ☐ Retire Notes:

For 5 weeks, a 9-year-old has refused sleepovers, had nightmares about her mother being hurt, and sobbed whenever her mother leaves for work. The symptoms are causing problems at home and school. Her father asks whether a diagnosis of separation anxiety disorder requires symptoms to last 6 months. What is the minimum duration for a child?

A. Six months, the same as for adults

Six months is the typical duration for adults; children need a shorter duration.

B. At least 4 weeks ✓ KEY

DSM-5-TR requires at least 4 weeks of symptoms in children and adolescents.

C. At least 3 months

Three months is not the DSM-5-TR duration for separation anxiety disorder.

D. At least 2 weeks

Two weeks is the major depressive episode duration, not the separation anxiety duration.

Rationale: Separation anxiety disorder requires developmentally inappropriate and excessive fear about separation from attachment figures, shown by at least three of the listed symptoms, lasting at least 4 weeks in children and adolescents and typically 6 months or more in adults. DSM-5 removed the requirement that onset occur before age 18, so the disorder can also be diagnosed in adults.

References: DSM-5-TR — Separation Anxiety Disorder, Criterion B (duration)

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old who endured years of childhood abuse reports intrusive memories and avoidance, along with pervasive shame, a sense of being worthless, great difficulty calming strong emotions, and pulling away from close relationships. A colleague wants to give a DSM-5-TR diagnosis of "complex PTSD." Which statement is MOST accurate?

A. Complex PTSD is a DSM-5-TR subtype of PTSD that is recorded with a specifier

DSM-5-TR has no complex PTSD subtype or specifier; its PTSD specifiers are dissociative symptoms and delayed expression.

B. Shame and emotion dysregulation rule out PTSD, so another diagnosis is needed

Negative self-beliefs, persistent negative emotions, and detachment are part of PTSD Criterion D.

C. Borderline personality disorder must be diagnosed when these features appear

These features overlap with borderline personality disorder but do not require that diagnosis; a full assessment is needed.

D. Complex PTSD is an ICD-11 diagnosis; in DSM-5-TR, PTSD covers these features ✓ **KEY**

ICD-11 includes complex PTSD; DSM-5-TR captures these features within PTSD, chiefly Criterion D.

Rationale: ICD-11 lists complex PTSD, which adds disturbances in self-organization (affect dysregulation, negative self-concept, relational difficulties) to core PTSD symptoms, usually after prolonged or repeated trauma. DSM-5-TR has no separate complex PTSD diagnosis; its broadened PTSD criteria, especially negative alterations in cognitions and mood, capture many of these features. Counselors should know both systems when reading records and research.

References: ICD-11 — 6B41 Complex post-traumatic stress disorder · DSM-5-TR — Posttraumatic Stress Disorder, Criterion D and specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A 40-year-old with PTSD after an armed robbery at her store is ready for trauma-focused treatment. She keeps telling herself, "It was my fault for staying open late," and says she would rather not retell the event in detail session after session. Which evidence-based approach BEST matches her needs and preferences?

A. Prolonged exposure, which centers on repeated imaginal retelling

PE is effective, but repeated imaginal exposure to the memory is its core, which she prefers to avoid.

B. Critical incident stress debriefing, held in a single group session

Single-session debriefing is not an evidence-based treatment for PTSD.

C. Cognitive processing therapy, which targets stuck points like self-blame ✓ **KEY**

CPT focuses on identifying and challenging stuck points such as self-blame, and a written account is optional.

D. Psychological first aid, focused on early safety, comfort, and practical help

PFA is an early post-event support approach, not a treatment for established PTSD.

Rationale: Both prolonged exposure and cognitive processing therapy are strongly recommended for PTSD. PE uses repeated imaginal exposure to the trauma memory and in vivo exposure to avoided situations. CPT helps clients identify and challenge "stuck points," such as self-blame, and can be delivered without a written trauma account. Client preference is a legitimate factor in choosing among effective treatments.

References: APA CPG — Clinical Practice Guideline for the Treatment of PTSD (2017): CPT and PE recommendations

☐ Approve ☐ Revise ☐ Retire Notes:

A paramedic reports intrusive images, avoidance, and constant hypervigilance after years of responding to fatal crashes, including the deaths of children. He was never in danger himself. A peer tells him he cannot have PTSD because his own life was never threatened. Which statement is MOST accurate?

A. Repeated or extreme exposure to aversive details on the job can meet Criterion A ✓ KEY

Criterion A4 includes repeated or extreme exposure to aversive details, as with first responders.

B. Only a direct threat to his own life or body can satisfy the stressor criterion

Criterion A also includes witnessing, learning about a close other's trauma, and repeated work exposure.

C. Secondary traumatic stress is the DSM-5-TR diagnosis that applies in its place

Secondary traumatic stress is a useful concept but is not a DSM-5-TR diagnosis.

D. His symptoms are better classed as adjustment disorder because of his job role

Occupational exposure does not shift the diagnosis to adjustment disorder when PTSD criteria are met.

Rationale: PTSD Criterion A can be met by direct exposure, witnessing, learning that a traumatic event happened to a close family member or friend, or experiencing repeated or extreme exposure to aversive details of traumatic events, as with first responders or police. Exposure through electronic media counts only when it is work related. First responders face cumulative exposure and should be assessed accordingly.

References: DSM-5-TR — Posttraumatic Stress Disorder, Criterion A4

☐ Approve ☐ Revise ☐ Retire Notes:

A 23-year-old spends hours each day picking at small bumps on her face and arms, causing open sores. She has tried many times to stop, feels ashamed, and wears long sleeves to hide the marks. A physician found no skin condition, and she uses no stimulants. Which diagnosis is MOST likely?

A. Trichotillomania

Trichotillomania involves recurrent hair pulling, not skin picking.

B. Excoriation disorder ✓ KEY

Recurrent skin picking causing lesions, with repeated attempts to stop and distress, fits excoriation disorder.

C. Body dysmorphic disorder

BDD picking aims to fix a perceived appearance flaw; no such preoccupation is described.

D. Nonsuicidal self-injury

NSSI involves intentional self-injury for relief or other functions, not a body-focused repetitive habit.

Rationale: Excoriation (skin-picking) disorder, in the obsessive-compulsive and related disorders chapter, requires recurrent skin picking resulting in skin lesions, repeated attempts to decrease or stop, and clinically significant distress or impairment. The picking must not be due to a substance, such as stimulants, or a dermatologic condition, and must not be better explained by another disorder, such as BDD.

References: DSM-5-TR — Excoriation (Skin-Picking) Disorder: criteria and differential diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor and a client with contamination-focused OCD are building an exposure and response prevention (ERP) hierarchy. Which practice is MOST consistent with ERP?

A. Start with the most feared item so that change comes quickly

Beginning at the top of the hierarchy is flooding and can reduce willingness to continue; ERP usually starts lower.

B. Allow one quick wash after each exposure to keep distress low

Washing after exposure is a compulsion that undercuts response prevention.

C. End each exposure as soon as the client's distress starts to rise

Ending exposures when distress rises rewards escape and prevents new learning.

D. Begin with tasks rated as moderately distressing and work upward ✓ **KEY**

Graded ERP typically starts with moderately distressing items and moves up the hierarchy.

Rationale: In ERP, the client rates feared situations (often with subjective units of distress) to build a graded hierarchy, then repeatedly confronts them while refraining from compulsions, including subtle or mental rituals. Exposures usually begin with moderately distressing items and progress upward. Remaining in the situation without ritualizing lets the client learn that feared outcomes do not occur and that distress can be tolerated.

References: NICE guidelines — CG31 Obsessive-compulsive disorder and body dysmorphic disorder: CBT including ERP

► **Check:** NICE CG31 supports ERP; the graded-hierarchy details are standard ERP practice rather than guideline text. No OCD-specific treatment manual is on the approved list.

☐ Approve ☐ Revise ☐ Retire Notes:

61. nce-s-clf-131 · Counseling and Helping Relationships · substance use and addictive disorders · easy · In review

A 48-year-old who has drunk heavily every day for years plans to quit "cold turkey" this weekend while home alone. He mentions morning shakes and sweats that go away once he has a drink. What should the counselor do FIRST?

A. Affirm his plan and schedule a check-in call for Monday

Affirming an unsupervised stop ignores the risk of dangerous withdrawal.

B. Start a relapse prevention plan focused on his triggers

Relapse prevention matters later; immediate medical safety comes first.

C. Refer him for medical evaluation before he stops drinking ✓ **KEY**

Signs of physical dependence call for medical assessment and supervised withdrawal management.

D. Suggest he taper on his own by cutting back day by day

An unsupervised self-taper still carries withdrawal risk; the decision belongs with a medical provider.

Rationale: Alcohol withdrawal can progress to seizures and withdrawal delirium, which can be fatal. Morning tremor and sweating relieved by drinking suggest physical dependence, so the counselor should arrange medical evaluation to determine the appropriate withdrawal management setting before the client stops. ASAM Dimension 1 (acute intoxication and withdrawal potential) guides this decision.

References: ASAM Criteria — Dimension 1: acute intoxication and/or withdrawal potential · DSM-5-TR — Alcohol Withdrawal: features and complications

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old is referred after cannabis is found in his backpack. He uses most days and his grades are slipping. He says, "Weed is natural. It doesn't hurt anybody." Which counselor response BEST reflects a motivational rather than a confrontational approach?

A. "Research shows cannabis harms the teen brain, so you need to stop now."

Leading with warnings and demands is the righting reflex and tends to increase resistance.

B. "Until you admit you have a problem, counseling will not help you much."

Requiring the client to accept a label is a confrontational stance MI avoids.

C. "It feels pretty harmless to you. What, if anything, concerns you about it?" ✓ **KEY**

A reflection followed by an open question rolls with discord and invites the teen's own reasons for change.

D. "Your parents and school are worried, so let's agree that you quit today."

Pressing for a commitment the teen has not voiced bypasses his ambivalence and autonomy.

Rationale: Motivational interviewing avoids arguing, labeling, and the righting reflex, which tend to provoke sustain talk, especially in adolescents sensitive to autonomy. Instead, the counselor reflects the client's view and asks open questions that evoke the client's own concerns and reasons for change. Research does not support confrontational approaches for substance use.

References: Miller & Rollnick (MI) — The righting reflex; responding to sustain talk and discord · SAMHSA TIP 35 — Motivational approaches with adolescents

☐ Approve ☐ Revise ☐ Retire Notes:

A 32-year-old seeks an ADHD evaluation for lifelong disorganization, forgetfulness, and trouble finishing tasks at work and home. Which statement about diagnosing ADHD in adults under DSM-5-TR is accurate?

A. Five symptoms in a domain are needed for those aged 17 and older ✓ **KEY**

DSM-5-TR lowers the threshold to five symptoms in a domain for older adolescents and adults.

B. Six symptoms in a domain are needed at every age, adults included

Six symptoms are required for children up to age 16; five for those 17 and older.

C. Impairment from symptoms must be documented from before age 7

DSM-5 replaced age 7 with several symptoms present before age 12; impairment need not be documented then.

D. Symptoms need to appear in only one setting, such as the workplace

Several symptoms must be present in two or more settings.

Rationale: For ADHD, children up to age 16 need six or more symptoms of inattention and/or hyperactivity-impulsivity, while adolescents aged 17 and older and adults need five or more. Several symptoms must have been present before age 12 and in two or more settings. Adult presentations often show more inattention and inner restlessness than overt hyperactivity.

References: DSM-5-TR — Attention-Deficit/Hyperactivity Disorder, Criterion A (symptom thresholds by age)

☐ Approve ☐ Revise ☐ Retire Notes:

An autistic 8-year-old speaks in full sentences but struggles with back-and-forth conversation and making friends. His insistence on sameness and great distress at small changes markedly interfere with functioning across settings. How should severity be recorded under DSM-5-TR?

A. Assign one overall level, based mainly on his measured IQ

Severity is not based on IQ; intellectual impairment is recorded with a separate specifier.

B. Rate social communication and restricted behaviors separately ✓ **KEY**

DSM-5-TR rates the level of support needed separately for each of the two domains.

C. Assign a support level only if he has intellectual disability

Severity levels are recorded for all autistic individuals, with or without intellectual disability.

D. Use only the more impaired domain to set one combined level

The domains are rated separately, so a child may need different levels of support in each.

Rationale: DSM-5-TR asks clinicians to specify current severity of autism spectrum disorder separately for social communication and for restricted, repetitive behaviors, using levels 1 (requiring support), 2 (requiring substantial support), and 3 (requiring very substantial support). Severity can change across contexts and over time, and it is recorded alongside specifiers for intellectual and language impairment.

References: DSM-5-TR — Autism Spectrum Disorder: severity levels for each domain

☐ Approve ☐ Revise ☐ Retire Notes:

Two weeks after giving birth, a 28-year-old suddenly develops delusions and disorganized speech. She has no manic or depressive symptoms. With treatment, she returns fully to her prior level of functioning after 10 days. Which diagnosis BEST fits?

A. Schizophreniform disorder, with good prognostic features

Schizophreniform disorder requires symptoms lasting at least 1 month.

B. Major depressive disorder, with peripartum onset

No depressive episode is described.

C. Schizophrenia, first episode, currently in acute episode

Schizophrenia requires continuous signs for at least 6 months.

D. Brief psychotic disorder, with peripartum onset ✓ **KEY**

Psychotic symptoms lasting at least 1 day but less than 1 month, beginning within 4 weeks postpartum, fit this diagnosis and specifier.

Rationale: Brief psychotic disorder requires at least one of delusions, hallucinations, or disorganized speech (plus possibly grossly disorganized behavior), lasting at least 1 day but less than 1 month, with full return to premorbid functioning. The "with peripartum onset" specifier applies when onset is during pregnancy or within 4 weeks postpartum. Postpartum psychosis is a psychiatric emergency, and clinicians should also consider an underlying bipolar disorder.

References: DSM-5-TR — Brief Psychotic Disorder: criteria and specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 17-year-old report that over 8 months he has pulled away from friends, his grades have dropped, he neglects hygiene, and he has become suspicious and voices odd ideas. He has no clear hallucinations or fixed delusions and does not use substances. What is the MOST appropriate action?

A. Refer for a full evaluation, as these may be early signs of psychosis ✓ KEY

A decline in functioning with emerging odd beliefs can signal a prodrome; early evaluation and intervention improve outcomes.

B. Reassure the parents that these are typical changes during adolescence

A marked, sustained decline with suspiciousness goes beyond typical adolescent change.

C. Diagnose schizophrenia, since negative signs have lasted over 6 months

Schizophrenia requires an active phase of at least 1 month of Criterion A symptoms, which is not present.

D. Wait to act until hallucinations or delusions have clearly appeared

Waiting for frank psychosis delays care; a long duration of untreated psychosis worsens outcomes.

Rationale: Prodromal signs of schizophrenia often include social withdrawal, declining school or work performance, poor self-care, and attenuated positive symptoms such as unusual beliefs or suspiciousness. These signs do not establish a diagnosis, but they warrant prompt referral for comprehensive evaluation, because shorter duration of untreated psychosis is associated with better outcomes. DSM-5-TR lists attenuated psychosis syndrome as a condition for further study.

References: DSM-5-TR — Schizophrenia: development and course (prodromal phase) · DSM-5-TR — Conditions for Further Study: Attenuated Psychosis Syndrome

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old cannot decide what to wear or what to order at a restaurant without reassurance from her mother. She agrees with others even when she disagrees, for fear of losing their support, feels helpless when alone, and quickly began a new relationship after a breakup. Which personality disorder BEST fits?

A. Avoidant personality disorder

Avoidant PD centers on fear of criticism and rejection leading to social avoidance, not a need to be taken care of.

B. Histrionic personality disorder

Histrionic PD involves attention seeking and shallow, shifting emotion.

C. Dependent personality disorder ✓ KEY

Difficulty making everyday decisions, fear of disagreeing, and urgently seeking a new relationship fit dependent PD.

D. Borderline personality disorder

Borderline PD involves unstable identity, affect, and relationships, not submissive reliance for everyday decisions.

Rationale: Dependent personality disorder, in Cluster C, is a pervasive and excessive need to be taken care of that leads to submissive, clinging behavior and fears of separation. Criteria include difficulty making everyday decisions without excessive advice, difficulty expressing disagreement, feeling helpless when alone, and urgently seeking another relationship when one ends.

References: DSM-5-TR — Dependent Personality Disorder: criteria and differential diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old manager follows rules rigidly, misses deadlines because work is never perfect enough, will not delegate, is reluctant to spend money, and has let friendships lapse because of work. He sees his ways as simply "the right way." He reports no intrusive thoughts or rituals. Which statement is MOST accurate?

A. OCD fits best, since his perfectionism counts as an obsession

Perfectionism that is experienced as correct is a trait, not an intrusive, unwanted obsession.

B. OCPD fits; his traits form a pattern without true obsessions ✓ KEY

His pervasive, ego-syntonic perfectionism, rigidity, and miserliness, without obsessions or compulsions, fit OCPD.

C. OCPD can be diagnosed only when OCD is also present with it

OCPD does not require OCD; the two may co-occur but are distinct.

D. Narcissistic PD fits, since he sees his own way as superior

His focus is on order and control, not grandiosity or a need for admiration.

Rationale: Obsessive-compulsive personality disorder is a pervasive pattern of preoccupation with orderliness, perfectionism, and control at the expense of flexibility and efficiency, including excessive devotion to work, reluctance to delegate, and miserly spending. Unlike OCD, it involves no true obsessions or compulsions, and the traits are usually ego-syntonic. The two can co-occur, but neither requires the other.

References: DSM-5-TR — Obsessive-Compulsive Personality Disorder: differential diagnosis (OCD)

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 7-year-old say he eats only a handful of familiar foods and refuses new textures. A pediatrician has ruled out a medical cause. Which finding would MOST support avoidant/restrictive food intake disorder (ARFID) rather than ordinary picky eating?

A. He refuses most vegetables and prefers plain pasta at dinner

Food preferences like this are common in young children and do not show clinical significance.

B. He eats more readily when meals are served in a favorite bowl

This is a typical childhood preference with no sign of nutritional or functional impact.

C. He complains that many new foods look strange or smell bad to him

Sensory dislikes occur in both picky eating and ARFID, so they do not distinguish them.

D. He now relies on nutritional supplements and growth has faltered ✓ KEY

Faltering growth and dependence on supplements are among the consequences that define ARFID.

Rationale: ARFID requires an eating disturbance that leads to at least one of: significant weight loss or faltering growth in children, significant nutritional deficiency, dependence on enteral feeding or oral nutritional supplements, or marked interference with psychosocial functioning. Picky eating without these consequences is developmentally common. ARFID also differs from anorexia nervosa in lacking body image disturbance.

References: DSM-5-TR — Avoidant/Restrictive Food Intake Disorder, Criterion A

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old in outpatient counseling for anorexia nervosa says her recent blood work was "totally normal," so she no longer needs to see her physician. Which counselor response is MOST accurate?

A. Normal labs do not rule out serious risk, so medical monitoring should continue ✓ **KEY**

Laboratory results can be normal despite serious medical risk, so ongoing medical monitoring remains part of care.

B. Normal labs show she is medically stable and can pause physician visits for now

Normal labs do not establish medical stability in anorexia nervosa.

C. Medical monitoring is needed only if she begins to report purging behaviors

Restriction alone carries significant medical risk; monitoring is not limited to purging.

D. The counselor can track her physical health during sessions in place of a doctor

Medical monitoring is outside counseling scope and requires a qualified medical provider.

Rationale: Anorexia nervosa has one of the highest mortality rates among mental disorders, and laboratory values can remain normal even when risk is serious. Treatment is multidisciplinary: the counselor coordinates with a medical provider who monitors physical health throughout care, and helps the client see monitoring as part of recovery rather than a sign of failure.

References: NICE guidelines — NG69 Eating disorders: recognition and treatment, physical health monitoring · DSM-5-TR — Anorexia Nervosa: functional consequences and laboratory findings

☐ Approve ☐ Revise ☐ Retire Notes:

A nursing home resident with moderate Alzheimer's disease has, over the past 2 days, become much more confused, drowsy at times and agitated at others, and unable to keep her attention on a conversation. Staff say, "Her dementia is just getting worse." What should the counselor do?

A. Agree that this is expected decline and update her care plan

Alzheimer's disease progresses gradually; an abrupt change over days is not expected progression.

B. Begin reminiscence sessions to help reorient her to the past

Reminiscence does not address a possible acute medical cause of the change.

C. Alert medical staff promptly, as this may signal a delirium ✓ **KEY**

Acute onset, fluctuating alertness, and inattention suggest delirium, which needs urgent medical evaluation.

D. Repeat a cognitive screen in a month to compare the scores

Waiting a month delays care for a potentially serious, treatable medical cause.

Rationale: Delirium is a disturbance in attention and awareness that develops over hours to days, fluctuates, and is caused by a medical condition, substance, or medication. People with major neurocognitive disorder are at higher risk, and delirium superimposed on dementia is often missed because staff attribute the change to the dementia. Any abrupt change in cognition calls for prompt medical evaluation.

References: DSM-5-TR — Delirium: criteria, risk factors, and differential diagnosis (major neurocognitive disorder)

☐ Approve ☐ Revise ☐ Retire Notes:

A month after her grandfather died, a 4-year-old keeps asking when he will be finished being dead and come home. Her parents worry that something is wrong. What BEST explains her questions?

A. Her questions show a denial that calls for treatment

This reflects normal cognitive development, not pathological denial.

B. Young children often do not yet see death as permanent ✓ **KEY**

Preschoolers commonly view death as temporary or reversible; irreversibility is understood later.

C. She is showing early signs of prolonged grief disorder

Her questions are developmentally expected, and too little time has passed for that diagnosis.

D. She is too young to grieve, so the topic should wait

Young children do grieve and benefit from simple, honest explanations.

Rationale: Children gradually develop an understanding of death's components: irreversibility (permanence), nonfunctionality (the body stops working), universality (all living things die), and causality. Preschoolers often think of death as temporary, like sleep or a trip. Adults can help by using clear, concrete words such as "died" rather than euphemisms, answering repeated questions patiently, and including children in mourning rituals as appropriate.

References: Berk (Development Through the Lifespan) — Death, dying, and bereavement: children's understanding of death · Worden (Grief Counseling) — Grieving children: developmental understanding of death

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old's former husband, with whom she stayed close after their divorce, died suddenly. His family did not invite her to the funeral, coworkers seem puzzled that she is upset, and she says she has "no right" to grieve. Which concept BEST describes her experience?

A. Anticipatory grief

Anticipatory grief occurs before an expected death; this death was sudden.

B. Prolonged grief disorder

Nothing indicates the duration or impairment required for prolonged grief disorder.

C. Chronic grief

Chronic grief refers to excessively long mourning, not grief that lacks social recognition.

D. Disenfranchised grief ✓ **KEY**

Grief that is not openly acknowledged, socially supported, or publicly mourned is disenfranchised grief.

Rationale: Disenfranchised grief, a term introduced by Doka, describes grief that is not openly acknowledged or socially supported, such as the death of a former spouse, an unrecognized partner, a pet, or a pregnancy loss. Lack of social recognition removes supports and rituals and can complicate mourning. Counselors can validate the loss and help clients create their own ways to mourn.

References: Worden (Grief Counseling) — Mediators of mourning: social mediators and socially unrecognized losses

► **Check:** The term is Doka's; Worden discusses socially unspeakable/negated losses among social mediators. SME to confirm the Worden locator wording.

☐ Approve ☐ Revise ☐ Retire Notes:

The husband of a woman with advancing ALS says he already grieves the losses of her voice and their plans for retirement. He feels guilty "mourning someone who is still here." Which counselor response is MOST helpful?

A. Normalize his anticipatory grief and help him face both loss and time left ✓ **KEY**

Anticipatory grief is a normal response to expected loss; counseling can attend to it while supporting time together.

B. Advise him to set the grief aside until after she has died, to stay strong

Suppressing grief is not recommended and adds to his guilt and isolation.

C. Explain that grieving now shows a detachment that could harm her care

Anticipatory grief does not indicate harmful detachment and should not be pathologized.

D. Assure him that grieving now means he will grieve far less after she dies

Anticipatory grief does not reliably reduce grief after the death, so this promise is misleading.

Rationale: Anticipatory grief is mourning that begins before an expected death, often in response to a series of losses during a progressive illness. It is normal and may include guilt, sadness, and fatigue. Worden notes that it does not necessarily shorten or lessen grief after the death; counselors can normalize it, address the losses already occurring, and help the family make meaningful use of the time remaining.

References: Worden (Grief Counseling) — Anticipatory grief

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor returns from a meeting to four voicemails from clients. Which client should the counselor contact FIRST?

A. A client who reports her panic attacks are back and she cannot sleep

This is distressing and needs follow-up, but it shows no imminent danger.

B. A client who is furious about a custody ruling and wants to vent today

Anger about a court ruling warrants a call but shows no stated risk of harm.

C. A client who has written goodbye letters and given away her pet cats ✓ **KEY**

Goodbye letters and giving away belongings are preparatory behaviors that signal acute suicide risk.

D. A client who missed his appointment and would like to reschedule it

A routine scheduling request has the lowest priority.

Rationale: Crisis triage puts imminent risk to life first. Preparatory behaviors, such as writing goodbye notes or giving away valued possessions, are among the strongest warning signs of suicide and call for immediate contact and risk assessment, for example with the C-SSRS. Other clients' needs are real but can be addressed afterward.

References: C-SSRS — Suicidal behavior: preparatory acts or behavior

☐ Approve ☐ Revise ☐ Retire Notes:

After a high school student dies by suicide, the principal asks the school counselor to lead the response. Which action BEST reduces the risk of suicide contagion among students?

A. Set up a lasting memorial on campus where students can gather and grieve

Permanent or prominent memorials can glamorize the death; postvention guidance favors time-limited, low-key options.

B. Share facts without method details and screen students most at risk ✓ **KEY**

Safe messaging plus identifying and reaching out to vulnerable students are core postvention steps.

C. Avoid any mention of the death so that students do not become upset

Silence fuels rumors and leaves grieving students without support.

D. Explain that stress caused the death, so students have a simple answer

Attributing suicide to a single cause oversimplifies it and can lead others to identify with it.

Rationale: Postvention aims to support grief and prevent contagion. Recommended steps include sharing accurate information in small groups or classrooms without describing the method, avoiding glamorizing or simplifying the death, offering time-limited and low-key memorial options, and actively identifying and reaching out to students at elevated risk, such as close friends, those with prior suicidal behavior, and those exposed to the death.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling in schools: responding after a suicide

► **Check:** Content follows AFSP/SPRC "After a Suicide: A Toolkit for Schools," which is not on the approved source list; the Gladding locator is a general crisis-counseling citation.

☐ Approve ☐ Revise ☐ Retire Notes:

A client who recently left an abusive partner says he keeps appearing wherever she goes and texts her details about her day. She plans to change all her passwords and delete her accounts tonight. Which counselor response is MOST appropriate?

A. Plan with her first, since sudden changes may alert him and raise risk ✓ **KEY**

Abrupt loss of access can signal to an abuser that the victim is acting and may escalate danger, so changes are planned as part of safety planning.

B. Urge her to block him and delete every account tonight to cut off his access

This may feel protective but can provoke escalation and destroy evidence of stalking.

C. Reassure her that tracking her is unlikely unless he holds her phone

Shared accounts, spyware, location sharing, and tracking devices allow monitoring without physical access.

D. Suggest she meet with him in person to demand that he stop following her

Confronting a stalking ex-partner can increase risk; contact decisions belong in safety planning.

Rationale: Technology-facilitated abuse and stalking are common after separation, which is itself a high-risk period. Safety planning considers how the abuser may be monitoring the client (shared accounts, location sharing, spyware, trackers), uses a safer device for sensitive contacts, documents incidents for possible legal action, and times changes so they do not alert the abuser. Connecting the client with a domestic violence advocate adds specialized expertise.

References: SAMHSA TIP 57 — Trauma-informed principles: safety and client choice · Gladding (Counseling: A Comprehensive Profession) — Counseling survivors of intimate partner violence

► **Check:** Technology-safety guidance comes from NNEDV Safety Net materials, which are not on the approved source list; citations are general.

☐ Approve ☐ Revise ☐ Retire Notes:

A mother says her 6-year-old was "asleep upstairs" during her partner's assaults on her and was never hit, so the girl has not been affected. Since the mother left, the child has had nightmares, startles easily, and clings to her at bedtime. Which statement is MOST accurate?

A. Children who are not hit themselves seldom show lasting effects

Children exposed to IPV show emotional and behavioral effects comparable to those of directly abused children.

B. Her symptoms most likely reflect a normal phase at her age

Nightmares, startle, and clinging after exposure to violence are trauma-related signs, not a typical phase.

C. Young children forget such events, so focus on the mother

Young children are affected by violence they overhear, and they need their own assessment and support.

D. Hearing or seeing IPV can cause trauma reactions in children ✓ KEY

Witnessing or overhearing violence against a caregiver is a traumatic exposure that can produce trauma symptoms.

Rationale: Children often see or hear IPV even when parents believe they are asleep or unaware. DSM-5-TR PTSD criteria for children 6 years and younger specifically include witnessing traumatic events occurring to others, especially primary caregivers. Exposed children may show sleep problems, hyperarousal, regression, and clinginess, and benefit from trauma-informed assessment and a strengthened caregiver relationship.

References: DSM-5-TR — Posttraumatic Stress Disorder for Children 6 Years and Younger, Criterion A2 · SAMHSA TIP 57 — Trauma across the life span: children exposed to violence

☐ Approve ☐ Revise ☐ Retire Notes:

A 53-year-old client says that since her youngest child left for college, she wanders the house feeling useless. Her husband, by contrast, is enjoying the extra time together. Based on life-span research on the empty nest, which factor MOST increases the risk that launching children will be a painful transition?

A. Having a spouse who adjusts to the change faster

A spouse adapting faster may highlight the contrast, but research does not identify it as the main risk factor.

B. An identity built mainly around the parenting role ✓ KEY

Empty-nest distress is most likely when a parent has invested identity chiefly in parenting and has few other roles to turn to.

C. Launching the last child after the age of fifty

Timing that fits social expectations (on-time launching) is usually easier; age over fifty is not itself the risk factor.

D. Children who move to a college far from the home

Distance may add sadness, but most parents adjust well regardless; the key risk lies in how central parenting is to the self.

Rationale: Most parents adjust well to the empty nest, and many report relief and improved marital satisfaction. Distress is more likely when parenting has been the core of a person's identity, when other roles are thin, or when children leave off-time. Counseling can help the client build roles and meaning beyond active parenting.

References: Berk (Development Through the Lifespan) — Middle adulthood: launching children and the empty nest

☐ Approve ☐ Revise ☐ Retire Notes:

Parents worry that their 22-year-old son has "failed to launch." In two years he has moved apartments three times, switched jobs twice, and ended two relationships. He is curious and hopeful about his future. Within Arnett's theory of emerging adulthood, how is this pattern BEST understood?

A. Instability, a typical feature of emerging adulthood ✓ KEY

Arnett names instability in love, work and residence as one of the defining features of emerging adulthood.

B. Identity diffusion that shows his development stalled

Diffusion (Marcia) means neither exploring nor committing; he is actively exploring, so the label does not fit.

C. Role confusion left over from an unresolved adolescence

Arnett frames these moves as normative for the age period, not as a failure of Erikson's adolescent stage.

D. Delayed launching caused by overinvolved, enmeshed parents

Nothing in the stem points to enmeshment; frequent change is expected in this period.

Rationale: Arnett describes emerging adulthood (roughly ages 18–25) by five features: identity explorations, instability, self-focus, feeling in-between, and possibilities. Frequent changes of residence, work and partners are expected. Counselors can reframe parents' worry while still supporting the young adult's goal setting.

References: Berk (Development Through the Lifespan) — Emerging adulthood (Arnett): features of the period

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old client works full time, is raising two teenagers, and now helps her mother, who has early dementia, with meals, bills and appointments. She is exhausted, snaps at her children, and feels guilty that she is "failing everyone." What should the counselor do FIRST?

A. Urge her to move her mother into a memory care facility

Placement may become an option, but directing it first ignores her values and skips assessment.

B. Explore early conflicts with her mother that fuel the guilt

Family history may matter later, but her current overload and well-being take priority.

C. Assess her caregiving load and link her to respite support ✓ KEY

For a "sandwich generation" caregiver, assessing strain and connecting her with respite, services and shared care is the practical first step.

D. Advise her to cut back her hours at work to free up time

Reducing work may add financial stress and is a decision for her after options are explored.

Rationale: Midlife adults who care for both children and aging parents (the "sandwich generation") face role overload that raises the risk of depression and burnout. Counselors first assess the demands and resources, then help the client mobilize respite, family sharing of tasks and community services, and address guilt and self-care.

References: Berk (Development Through the Lifespan) — Middle adulthood: caring for aging parents

☐ Approve ☐ Revise ☐ Retire Notes:

Four months after the birth of their first child, a couple seeks counseling. They argue about night feedings and chores, and both say they feel less close than before. Which statement BEST reflects research on the transition to parenthood?

A. Their conflict signals a poor match that a baby has exposed

A dip in satisfaction after a first birth is common and does not by itself show the couple is mismatched.

B. Satisfaction usually rises soon after a first child arrives

On average, relationship satisfaction declines modestly rather than rises after the first birth.

C. Strain mostly reflects postpartum mood in the birth parent

Postpartum mood should be screened, but strain on the couple is common even without a mood disorder.

D. Satisfaction often dips, less so when care is shared fairly ✓ KEY

Research shows a common decline in satisfaction that is smaller when partners share caregiving and chores in a way both see as fair.

Rationale: Studies of the transition to parenthood find that relationship satisfaction often declines after a first child, driven by sleep loss, role change and disputes over the division of labor. Couples who share child care and household tasks fairly, and who hold realistic expectations, show less decline. Counseling can target these modifiable factors.

References: Berk (Development Through the Lifespan) — Early adulthood: transition to parenthood and marital satisfaction

☐ Approve ☐ Revise ☐ Retire Notes:

A 39-year-old client with relapsing-remitting multiple sclerosis had seemed at peace with her diagnosis. After a new relapse, she is again angry, tearful and asking "why me?" Her counselor wonders if earlier progress was lost. Which understanding of adjustment to chronic illness BEST fits?

A. Adjustment is often cyclical, and reactions return as the illness changes ✓ KEY

Contemporary adjustment models describe nonlinear, recurring reactions, especially when the illness course shifts.

B. Her anger shows she skipped a stage and must now go back and finish it

Rigid stage sequences are not supported; people do not need to complete stages in a fixed order.

C. Once a client reaches acceptance, a return of grief signals a new disorder

Renewed grief after a relapse is expected, not evidence of a new mental disorder.

D. Her distress points to a failed treatment that calls for a higher level of care

An expected emotional response to a relapse does not by itself warrant a higher level of care.

Rationale: Models of psychosocial adaptation to chronic illness and disability describe reactions such as shock, anxiety, depression, anger and acknowledgment as overlapping and recurring rather than a fixed sequence. Changes in the illness, such as a relapse, can bring earlier reactions back. Counselors normalize this cycling and help the client re-engage coping resources.

References: Gladding (Counseling: A Comprehensive Profession) — Rehabilitation counseling: psychosocial adaptation to chronic illness and disability

► **Check:** Confirm Gladding (8th ed.) covers nonlinear psychosocial adaptation (e.g., Livneh & Antonak); a rehabilitation counseling text may be a better citation.

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old with chronic low back pain for three years also meets criteria for major depressive disorder. He tells the counselor, "Once the doctors fix my back, my mood will fix itself, so counseling can wait." Which response BEST reflects current understanding of chronic pain and depression?

A. Agree to pause counseling until medical treatment of the pain ends

Waiting leaves the depression untreated, and the two conditions reinforce each other.

B. Explain that his pain is likely psychological and needs no medical care

Chronic pain is real and needs medical care; implying it is "all in his head" damages trust.

C. Explain that pain and low mood feed each other and both can be treated ✓ **KEY**

Pain and depression are linked in both directions; psychological care can lift mood and lessen pain's grip on daily life.

D. Refer him to a pain clinic and close his counseling case for now

Coordinating with a pain clinic can help, but closing the case abandons treatment of his depression.

Rationale: Chronic pain and depression are closely linked, each worsening sleep, activity, and outlook in a cycle. Guidelines for chronic pain recommend psychological approaches such as CBT or ACT alongside medical care to reduce distress and disability. Counselors treat the depression now and coordinate with the medical team rather than waiting for the pain to resolve.

References: NICE guidelines — NG193 Chronic pain (primary and secondary) in over 16s: psychological therapy (CBT, ACT)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor works with a 68-year-old man who has cared for his wife with Parkinson's disease for six years. He reports feeling trapped, worn out and resentful, and the counselor wants a standard measure of how much strain caregiving places on him. Which instrument is designed for this purpose?

A. Mini-Mental State Examination

The MMSE screens cognitive impairment, not the strain caregiving places on a caregiver.

B. Zarit Burden Interview ✓ **KEY**

The Zarit Burden Interview measures perceived caregiver burden across health, finances, social life and the relationship.

C. Geriatric Depression Scale

The GDS screens for depression in older adults; it does not measure caregiving strain.

D. Beck Hopelessness Scale

The BHS measures negative expectations about the future, not caregiver burden.

Rationale: Caregiver burden is the physical, emotional, social and financial strain of caring for an ill or disabled person. The Zarit Burden Interview is a widely used self-report measure of it. High burden predicts caregiver depression and poorer health, so counselors screen for it and link caregivers to respite, support groups and services.

References: Hays (Assessment) — Assessment with older adults and caregivers: caregiver burden measures

► **Check:** Confirm Hays (Assessment) names the Zarit Burden Interview; the fact is standard but the citation locator is uncertain.

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses the FICA tool to take a brief spiritual history at intake. The letters stand for Faith or belief, Importance or influence, a third element, and Address in care. What does the "C" ask about?

A. Conflict between faith and treatment

Conflicts may surface, but "C" in FICA does not stand for conflict.

B. Coping skills learned from faith

Faith-based coping may arise in the interview, but it is not what "C" names.

C. Commitment to religious practice

Level of practice may come up under importance, but "C" is not commitment.

D. Community the person belongs to ✓ KEY

The "C" in FICA asks whether the client is part of a spiritual or religious community and whether it is a support.

Rationale: Puchalski's FICA is a brief spiritual history: Faith or belief (what gives meaning), Importance or influence (how beliefs shape life and health choices), Community (membership in and support from a spiritual community), and Address in care (how the client wants these addressed in treatment). It lets counselors assess spirituality respectfully without imposing values.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: spiritual assessment

► **Check:** FICA (Puchalski) components are standard; confirm Gladding mentions FICA or substitute a spirituality-in-counseling source.

☐ Approve ☐ Revise ☐ Retire Notes:

After a cancer diagnosis, a 61-year-old devout client says, "God is punishing me for my sins. Maybe this is what I deserve." She has stopped attending services and feels abandoned. In Pargament's research on religious coping, how is her response BEST classified, and what is it linked to?

A. Negative religious coping, tied to poorer adjustment ✓ KEY

Punishing-God reappraisals and feeling abandoned by God are negative religious coping, which predicts more distress.

B. Positive religious coping, tied to better adjustment

Positive coping includes benevolent reappraisals and seeking spiritual support, not a punishing-God view.

C. A deferring coping style, tied to passive acceptance

Deferring means leaving problems to God; her statement is about punishment and abandonment.

D. Spiritual bypassing, tied to avoidance of emotion

Spiritual bypassing uses spiritual ideas to avoid feelings; she is expressing distress, not avoiding it.

Rationale: Pargament distinguished positive religious coping (benevolent reappraisal, seeking spiritual support, collaborative problem solving) from negative religious coping (punishing-God reappraisal, feeling abandoned by God, questioning God's power). Negative religious coping is associated with greater depression and poorer health outcomes. Counselors assess for it and may collaborate with clergy when the client wishes.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: religious coping

► **Check:** Pargament's positive/negative religious coping (RCOPE) is well established; confirm the Gladding locator or cite a spirituality-specific source.

☐ Approve ☐ Revise ☐ Retire Notes:

Eight months after her infant died, a client who has always been devout says she is furious at God and ashamed of feeling that way. She fears her anger means she has lost her faith. The counselor does not share her religion. Which response is MOST appropriate?

A. Suggest she set aside spiritual topics until her grief eases

Avoiding her spiritual struggle leaves a central part of her grief unaddressed.

B. Reassure her that a loving God would not want her to be angry

This imposes a theological view and may deepen her shame.

C. Normalize anger at God as a common struggle and explore it ✓ **KEY**

Spiritual struggles such as anger at God are common after loss; exploring them openly reduces shame and supports meaning making.

D. Refer her to clergy since faith is outside the counselor's role

Clergy consultation may help with consent, but spiritual concerns are within a counselor's scope when they affect well-being.

Rationale: Spiritual struggles, such as anger at God, doubt, or conflict with a faith community, are common after trauma and loss and are linked to distress when unaddressed. Counselors respond within the client's own framework, normalize the struggle, and explore it without imposing beliefs. Collaboration with clergy is an option the client can choose.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: addressing spiritual concerns · ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes:

A veteran with PTSD has the same trauma-related nightmare several nights a week. His counselor has him write down the dream, change the storyline in any way he chooses, and rehearse the new version in imagery for a few minutes each day. Which intervention is this?

A. Cognitive processing therapy

CPT targets stuck points in beliefs about the trauma; it does not rescript dreams.

B. Imagery rehearsal therapy ✓ **KEY**

Imagery rehearsal therapy has clients change a recurring nightmare and rehearse the new version while awake.

C. Stimulus control therapy

Stimulus control re-links the bed with sleep; it does not change dream content.

D. Systematic desensitization

Systematic desensitization pairs relaxation with a fear hierarchy, not dream rescripting.

Rationale: Imagery rehearsal therapy (IRT) treats recurring nightmares, including PTSD-related ones. The client rewrites the nightmare in any way desired and rehearses the new script while awake, which often reduces nightmare frequency and distress. It can be used alongside trauma-focused therapy.

References: VA/DoD CPG — Management of PTSD: treatment of trauma-related nightmares (imagery rehearsal therapy)

► **Check:** Confirm the strength of the current VA/DoD PTSD CPG recommendation on IRT for nightmares; the item only asks for the definition.

☐ Approve ☐ Revise ☐ Retire Notes:

A 56-year-old client being treated for depression says he sleeps eight hours yet nods off while driving. His wife says he snores loudly and sometimes stops breathing and gasps at night. He also wakes with headaches and has high blood pressure. What should the counselor do?

A. Begin CBT-I with sleep restriction to consolidate sleep

Sleep restriction can worsen sleepiness and is not suited to a likely breathing disorder.

B. Teach sleep hygiene and review it again in two weeks

Sleep hygiene does not treat a possible breathing disorder, and the delay is unsafe given drowsy driving.

C. Treat the sleepiness as a symptom of his depression

Snoring, apneas and morning headaches point to a medical sleep problem, not only depression.

D. Refer him to his physician for a sleep apnea evaluation ✓ KEY

Loud snoring, witnessed breathing pauses, sleepiness and hypertension suggest obstructive sleep apnea, which needs medical evaluation.

Rationale: Obstructive sleep apnea hypopnea is a breathing-related sleep disorder marked by snoring, gasping or breathing pauses during sleep, and daytime sleepiness despite adequate sleep time. It is confirmed by polysomnography and treated medically. Untreated apnea can worsen mood and raises accident risk, so counselors refer promptly and continue mental health care.

References: DSM-5-TR — Breathing-related sleep disorders: obstructive sleep apnea hypopnea

☐ Approve ☐ Revise ☐ Retire Notes:

A 66-year-old retired engineer spent his first months of retirement traveling and finishing house projects. Now, about a year in, he feels let down, bored and unsure what his days are for. In Atchley's phases of retirement, which phase is he MOST likely in?

A. Honeymoon

The honeymoon phase is the early period of freedom and activity he has already moved past.

B. Reorientation

Reorientation follows disenchantment, as the retiree builds a realistic new routine.

C. Disenchantment ✓ KEY

Disenchantment is the letdown that can follow the honeymoon, when retirement does not meet expectations.

D. Termination

Termination is the end of the retirement role, often through illness or a return to work.

Rationale: Atchley described retirement phases: preretirement, honeymoon, disenchantment, reorientation, stability (routine), and termination. Not every retiree passes through each phase. Counselors can normalize disenchantment and help the client find meaningful roles such as volunteering, part-time work or learning.

References: Sharf (Career Development Theory) — Late career and retirement: adjustment to retirement

► **Check:** Atchley's phases are standard NCE content; confirm Sharf (6th ed.) presents them or cite Berk (late adulthood: retirement).

☐ Approve ☐ Revise ☐ Retire Notes:

A 63-year-old accountant plans to leave her full-time firm job next year. She does not want to stop working completely, so she is lining up part-time tax preparation work to ease the move into full retirement. What is this type of work called?

A. Bridge employment ✓ KEY

Bridge employment is paid work after leaving a career job and before full retirement.

B. Career plateauing

A plateau is a point where advancement stalls within a career, not a step toward retirement.

C. Role disengagement

Disengagement theory describes withdrawal from roles; she is staying engaged in work.

D. Maintenance stage

Super's maintenance stage involves holding one's place in a career, not shifting to new part-time work.

Rationale: Bridge employment is part-time, temporary or self-employed work that links a career job and full retirement. It can provide income, structure and social contact, and it is linked to better adjustment for many older workers. Career counselors help older clients weigh bridge options alongside finances, health and values.

References: Sharf (Career Development Theory) — Late career: older workers and bridge employment

► **Check:** Confirm Sharf uses the term bridge employment; Berk (late adulthood, retirement) is an alternative locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old tells her school counselor, "My uncle does things to me at night that I don't like, but you can't tell anyone." She looks scared and stops talking. Which response BEST fits recommended practice for handling a sexual abuse disclosure?

A. Promise to keep her secret so she will feel safe telling more

The counselor is a mandated reporter and cannot promise secrecy; a broken promise harms trust.

B. Stay calm, thank her for telling, avoid leading questions, report ✓ KEY

A calm, supportive response with minimal open questions protects the child and later investigation; reporting is required.

C. Ask if her uncle touched her private parts so the facts are clear

This is a leading question that can contaminate her account for any later forensic interview.

D. Gather a full, detailed account now so she will not need to retell

Detailed questioning is the role of trained forensic interviewers, not the school counselor.

Rationale: When a child discloses sexual abuse, counselors stay calm, believe and thank the child, use the child's own words, ask only minimal open-ended questions needed to establish reasonable suspicion, explain honestly that they must tell people who can help, and report promptly. Detailed or leading questioning can distress the child and compromise the investigation.

References: ACA Code of Ethics — B.2.a Serious and Foreseeable Harm and Legal Requirements · State law — Mandated reporting of suspected child abuse

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has seen a 7-year-old for six months for anxiety. After a report of possible abuse, the CPS caseworker asks the counselor to conduct the investigative interview, since "she already trusts you." The counselor has no forensic interview training. What should the counselor do?

A. Agree, since an existing alliance will help her disclose more

Trust does not make a treating counselor a proper forensic interviewer; mixing roles harms both.

B. Agree if the caseworker watches and records the session live

Observation does not solve the dual role or the lack of forensic training.

C. Interview her alone first, then pass the notes on to the caseworker

Investigative questioning by the therapist risks contaminating evidence and the therapy relationship.

D. Decline the forensic role and support a trained interviewer's work ✓ KEY

Treating counselors avoid forensic roles with their own clients; a trained interviewer, often at a child advocacy center, conducts it.

Rationale: Forensic interviews are fact-finding interviews conducted by trained interviewers using structured, nonleading protocols, often at a child advocacy center. A treating counselor's role is therapeutic, and the ACA Code bars counselors from evaluating their own current or former clients for forensic purposes. The counselor can support the child and cooperate within the law while keeping roles separate.

References: ACA Code of Ethics — E.13.c Client Evaluation Prohibited

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in a small rural town sees a 34-year-old Army National Guard member just home from deployment. Compared with many active-duty service members, which stressor is MORE characteristic of Guard and Reserve members and their families?

A. Frequent moves between distant bases that uproot the children

Frequent permanent change of station moves are more typical of active-duty families.

B. Living in base housing within a closely knit military unit

Base housing and on-post community are features of active-duty life, not Guard life.

C. Distance from military supports and strain on civilian jobs ✓ KEY

Guard and Reserve families often live far from installations and must leave and return to civilian jobs.

D. Fewer total deployments over the course of an enlistment

Fewer deployments would lessen stress; the distinct burden lies in the civilian-military shift and limited support.

Rationale: National Guard and Reserve members move rapidly between civilian and military life. Their families often live far from installations and military support services, may be surrounded by civilians who do not understand deployment, and face disruption to civilian employment and income. Counselors in community settings help connect them to resources and address these transition stressors.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling military populations: Guard and Reserve families

► **Check:** Content is standard; confirm Gladding addresses Guard/Reserve distinctions or cite a military counseling source.

☐ Approve ☐ Revise ☐ Retire Notes:

An 11-year-old lives with her father and stepmother on weekends. She enjoys baking with her stepmother but then feels guilty and turns cold toward her, saying, "Mom would be hurt if she knew I liked her." Which intervention BEST addresses this loyalty conflict?

A. Help her mother give clear permission to like her stepmother ✓ KEY

Explicit permission from the biological parent eases the child's loyalty bind most directly.

B. Have her stepmother step in as her primary mother on weekends

Asking the stepmother to replace her mother intensifies the loyalty conflict.

C. Advise the father to limit contact between the two households

Cutting contact does not resolve her guilt and may harm her relationship with her mother.

D. Tell her the guilt is a sign the stepfamily is not yet working

Loyalty conflicts are common in stepfamilies and do not mean the family is failing.

Rationale: Children in stepfamilies often feel that caring for a stepparent betrays the absent biological parent. These loyalty conflicts ease when both biological parents explicitly permit the child to have a good relationship with the stepparent, and when the stepparent is presented as an additional adult rather than a replacement.

References: Goldenberg (Family Therapy) — Remarried families: loyalty conflicts and stepfamily development

☐ Approve ☐ Revise ☐ Retire Notes:

A 64-year-old woman has taken custody of her two grandchildren because her daughter has a severe opioid use disorder. She loves them but says, "This was supposed to be my time." She is tired, short on money, and lies awake worrying about her daughter. Which concern is MOST characteristic of custodial grandparents?

A. A loss of interest in the grandchildren caused by aging

Nothing suggests lost interest; she loves the children and is stressed by the load.

B. An early sign of cognitive decline from added demands

Fatigue and worry do not indicate cognitive decline.

C. Rivalry with the grandchildren's other set of grandparents

Rivalry is not a characteristic theme for custodial grandparents.

D. Off-time role strain together with grief over her daughter ✓ KEY

Raising grandchildren is an off-time role, often with financial strain and grief or anger about the adult child.

Rationale: Grandparents raising grandchildren in skipped-generation households take on an unexpected, off-time parenting role. They often face financial strain, health demands, legal and school hurdles, social isolation from peers, and grief, anger or worry about the adult child whose problems led to the arrangement. Counselors address both the practical load and the losses.

References: Berk (Development Through the Lifespan) — Late adulthood: grandparents rearing grandchildren (skipped-generation families)

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old client says, "I'm probably gay. I've started going to an LGBTQ+ student group so I'm not so alone, but I'm not ready to say I like it about myself, and I haven't told my family." In Cass's model of homosexual identity formation, which stage BEST fits?

A. Identity comparison

In comparison the person first considers "I might be gay" and weighs the alienation; she has moved past this to seeking out community.

B. Identity tolerance ✓ KEY

In tolerance the person accepts "I probably am gay" and seeks contact with gay communities, tolerating rather than fully accepting the identity.

C. Identity acceptance

Acceptance brings a more positive view of the identity and more contact; she still only tolerates it.

D. Identity pride

Pride involves strong immersion and often anger at heterosexist society, which she does not show.

Rationale: Cass described six stages: identity confusion, comparison, tolerance, acceptance, pride, and synthesis. In tolerance, the person acknowledges a probable gay identity and seeks out others to reduce isolation, while still merely tolerating the identity. Stage models are useful lenses but not every client follows them in order.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: sexual identity development (Cass model)

☐ Approve ☐ Revise ☐ Retire Notes:

Religious parents of a 15-year-old who has come out as lesbian tell the counselor they cannot approve of her identity, but they love her and want to keep her safe. Which statement BEST reflects family acceptance research on LGBTQ+ youth?

A. Only her parents' full agreement with her identity lowers risk

Research shows benefit from reducing rejecting behaviors even before full acceptance.

B. Parental views matter little once teens find support from peers

Family acceptance and rejection have strong effects on youth health even with peer support.

C. Reducing rejecting behaviors lowers risk, even without agreement ✓ KEY

Family rejecting behaviors predict higher suicide, depression and substance risk; reducing them helps even without full agreement.

D. Teens facing rejection mainly benefit from moving out of the home early

Leaving home early raises risks such as homelessness; strengthening family support is preferred.

Rationale: Research from the Family Acceptance Project found that LGBTQ+ youth from highly rejecting families had far higher rates of suicide attempts, depression and substance use, while family acceptance was protective. Because even modest reductions in rejecting behaviors help, counselors can work with ambivalent families on specific behaviors, such as stopping slurs and supporting the teen at school, while respecting their values.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: family acceptance and rejection

► **Check:** Finding is from Ryan et al. (Family Acceptance Project); confirm Sue & Sue (9th ed.) discusses it or add a source with the original study.

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old woman resettled as a refugee three months ago after fleeing armed conflict. Through an interpreter she reports nightmares and poor sleep, and says she is worried about losing her apartment and about relatives still in a refugee camp. What should the counselor focus on FIRST?

A. Address safety and resettlement needs while building trust ✓ KEY

Trauma-informed, phase-oriented care starts with safety, stability and trust, including practical resettlement needs.

B. Begin detailed trauma narration to process her war memories

Detailed trauma processing comes after stabilization, not at the start amid housing insecurity.

C. Encourage her to adopt local customs to speed her adjustment

Pushing assimilation ignores her culture and can raise acculturative stress.

D. Teach her to set aside thoughts of her relatives in the camp

Her worry is realistic; suppressing it is neither feasible nor helpful.

Rationale: Refugees carry pre-migration trauma, migration losses and ongoing post-migration stressors such as housing, finances and separated family. Trauma-informed care begins with safety, stabilization and trust, uses qualified interpreters, and attends to practical needs before any trauma-focused work. Current stressors often drive distress as much as past events.

References: SAMHSA TIP 57 — Trauma-informed principles: safety and trustworthiness; refugee and immigrant populations

☐ Approve ☐ Revise ☐ Retire Notes:

A Korean immigrant family has lived in the United States for six years. The 15-year-old son speaks mostly English, wants to date, and argues that his parents' rules are "old-fashioned." His parents, who still speak mainly Korean, feel they are losing him. Which concept BEST describes this family conflict?

A. Typical adolescent rebellion unrelated to culture

Developmental strivings play a role, but the cultural gap is central to this conflict.

B. Marginalization of the son from both of his cultures

He is engaging with the host culture, not rejected by both cultures.

C. Cultural encapsulation of the parents' worldview

Cultural encapsulation (Wrenn) describes counselors who view others only through their own culture.

D. An intergenerational acculturation gap in the family ✓ KEY

Children often acculturate faster than parents, creating gaps in values and language that fuel conflict.

Rationale: An intergenerational acculturation gap arises when children adopt the host culture's language and values faster than their parents. The gap can strain communication, shift family roles, and heighten parent-child conflict. Counselors help family members understand each other's positions and find bicultural ground.

References: Sue & Sue (Counseling the Culturally Diverse) — Immigrant and refugee families: acculturation conflicts between generations

☐ Approve ☐ Revise ☐ Retire Notes:

Within two weeks, four ninth graders at a high school are found to be cutting after one student posted photos of her wounds online. The principal asks the school counselor how the school should respond. Which plan is MOST consistent with best practice for self-injury in schools?

A. Hold an assembly showing the warning signs and methods of cutting

Describing methods to large groups can spread self-injury through social contagion.

B. Meet each student alone, assess risk, inform parents, limit spread ✓ **KEY**

Individual assessment, parent notification and steps to curb contagion are core to a school protocol.

C. Start a support group where the four students share their cutting

Groups focused on sharing self-injury can reinforce it and spread it among members.

D. Keep the cutting from parents so students keep trusting staff

Parents of minors at risk should be informed; secrecy leaves students without home support.

Rationale: School responses to self-injury emphasize individual risk assessment (including suicide risk), parent notification, referral for treatment, and limiting social contagion. Counselors avoid large-group presentations about methods and process groups centered on self-injury, and they may address related online sharing. Students are offered support in a calm, nonjudgmental way.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: responding to self-injury and contagion · ACA Code of Ethics — B.5.b Responsibility to Parents and Legal Guardians

► **Check:** Best-practice content is standard (e.g., Walsh; ASCA); confirm the Gladding locator or cite a school self-injury protocol source.

☐ Approve ☐ Revise ☐ Retire Notes: _____

103. nce-s-clf-173 · Counseling and Helping Relationships · psychopharmacology basics for counselors · hard · In review

A counselor coordinating care for a client with bipolar I disorder learns that the psychiatrist is switching her from lithium to valproate. To support her in keeping medical appointments, the counselor wants to understand the lab monitoring involved. Which monitoring is MOST closely tied to valproate rather than lithium?

A. Kidney function and thyroid hormone levels

Kidney and thyroid monitoring is a hallmark of long-term lithium treatment.

B. Hydration status and daily salt intake

Dehydration and salt changes affect lithium levels, not valproate.

C. Liver function tests and platelet counts ✓ **KEY**

Valproate carries risks to the liver and blood platelets, so these are monitored.

D. Regular white blood cell counts for safety

Routine white cell monitoring is linked to clozapine, not valproate.

Rationale: Lithium has a narrow therapeutic range; monitoring includes blood levels, kidney and thyroid function, and attention to hydration and salt. Valproate monitoring focuses on liver function and blood counts, including platelets, and it carries serious risks in pregnancy. Counselors do not interpret labs but can encourage adherence to monitoring and communicate concerns to the prescriber.

References: NICE guidelines — CG185 Bipolar disorder: monitoring physical health with lithium and valproate

☐ Approve ☐ Revise ☐ Retire Notes: _____

A client who takes sertraline started St. John's wort on her own last week. In session she is restless and sweating, with shivering, muscle twitching, a racing heart and diarrhea. She says the symptoms began yesterday and are getting worse. What should the counselor recognize and do?

A. Possible serotonin syndrome needing urgent medical care ✓ KEY

Combining serotonergic agents can cause serotonin syndrome, which can become life-threatening and needs urgent care.

B. A panic attack to treat with breathing and grounding

Muscle twitching, diarrhea and sweating after adding a serotonergic herb point to a medical reaction, not panic.

C. SSRI discontinuation symptoms that will pass on their own

She has not stopped the SSRI; she added another serotonergic agent.

D. Neuroleptic malignant syndrome from her antidepressant

NMS is linked mainly to antipsychotics and features severe rigidity, not twitching after adding an herb.

Rationale: Serotonin syndrome results from excess serotonergic activity, often when an SSRI is combined with another serotonergic agent such as St. John's wort, tramadol, triptans or an MAOI. Signs include agitation, sweating, tremor or twitching, rapid heart rate, diarrhea and fever. Counselors do not manage medications but should recognize warning signs and direct the client to urgent medical care.

References: Gladding (Counseling: A Comprehensive Profession) — Psychopharmacology for counselors: antidepressant adverse effects

► **Check:** Clinical facts are standard; confirm Gladding covers serotonin syndrome or cite a psychopharmacology reference.

☐ Approve ☐ Revise ☐ Retire Notes:

A client who has taken a prescribed benzodiazepine daily for six years tells her counselor she is "done with pills" and plans to stop it all at once this weekend. Why should the counselor urge her to talk with her prescriber before stopping?

A. Stopping will cause relapse of her anxiety in all cases

Anxiety may return, but that is not certain and is not the main safety concern.

B. Stopping suddenly will lower her tolerance to alcohol use

This is not the key risk of abrupt benzodiazepine discontinuation.

C. Stopping is unsafe only if she took more than prescribed

Dependence develops with long-term use at prescribed doses too.

D. Abrupt withdrawal can bring severe effects like seizures ✓ KEY

Sedative withdrawal after long-term use can include seizures, so a supervised taper is needed.

Rationale: Long-term benzodiazepine use produces physiological dependence even at prescribed doses. DSM-5-TR sedative, hypnotic, or anxiolytic withdrawal can include autonomic hyperactivity, tremor, insomnia, anxiety, hallucinations and grand mal seizures. Counselors support the client's goal and urge a medically supervised taper.

References: DSM-5-TR — Sedative, hypnotic, or anxiolytic withdrawal: criteria

☐ Approve ☐ Revise ☐ Retire Notes:

Two months into her husband's deployment, a mother says their 4-year-old has started wetting the bed again, clings to her at daycare drop-off, and asks every night when Daddy is coming home. What guidance should the counselor give?

A. These signs point to a likely trauma disorder needing intensive care

Regression and clinging are common reactions to a parent's absence and do not by themselves suggest a trauma disorder.

B. Brief regression is common; keep routines and give honest reassurance ✓ **KEY**

Young children often regress during deployment; stable routines, simple honest answers and connection help.

C. Avoid mentioning her father so she is not reminded of his absence

Avoiding talk of the parent increases confusion; keeping him present through photos and messages helps.

D. Tell her Daddy will be home soon to calm her fears about his return

False reassurance can backfire; age-appropriate, honest information is better.

Rationale: Children's reactions to deployment vary by age. Preschoolers often show regression, clinginess and sleep problems; school-age children may show irritability or school problems; adolescents may take on adult roles or act out. Counselors help the at-home parent keep routines, give honest age-appropriate explanations, maintain connection with the deployed parent, and watch for persistent difficulties.

References: Berk (Development Through the Lifespan) — Early childhood: stress, separation and regression in young children

► **Check:** Content is standard deployment-cycle guidance; Berk does not address deployment directly, so a military family source may be preferable.

☐ Approve ☐ Revise ☐ Retire Notes:

A 14-year-old has missed 15 school days this term. Each morning he leaves home on time, then spends the day at a friend's house playing video games. He forged notes so his parents would not find out, and he shows no worry about school itself. Which pattern does this BEST fit?

A. School refusal due to anxiety about being away from home

School refusal involves emotional distress, often staying home with parents' knowledge.

B. School refusal driven by fear of a specific school setting

He shows no fear of school; he is avoiding it to do something more appealing.

C. Truancy, hidden from parents and without school anxiety ✓ **KEY**

Truancy is typically concealed from parents, lacks marked anxiety, and may involve conduct problems.

D. Avoidance linked to depression and loss of interest at school

Nothing points to depressed mood; he is actively seeking a rewarding activity elsewhere.

Rationale: School refusal involves emotional distress about attending, and the child usually stays home with the parents' knowledge. Truancy involves absence concealed from parents, little anxiety about school, and often conduct or peer-related problems. The distinction guides intervention: anxiety-focused exposure for refusal, versus behavioral contracts, parental monitoring and school engagement for truancy.

References: Gladding (Counseling: A Comprehensive Profession) — School counseling: attendance problems, school refusal and truancy

► **Check:** Distinction follows Berg/Kearney; confirm the Gladding locator or cite a school-attendance source.

☐ Approve ☐ Revise ☐ Retire Notes:

Treatment Planning

17 items · NCE weight 9% (14 of 160 scored items)

108. nce-s-trt-035 · Counseling and Helping Relationships · evidence-based treatment selection · medium · In review

A 31-year-old graduate student has had recurrent unexpected panic attacks for six months. She now avoids crowded buses and worries constantly that her racing heart signals a heart attack. Medical causes have been ruled out. Which psychotherapy approach is the BEST first-line choice?

A. Supportive listening that steers away from discussing body sensations

Avoiding feared sensations maintains the fear of them; nondirective support is not a first-line treatment for panic disorder.

B. Cognitive behavioral therapy that includes interoceptive exposure ✓ KEY

CBT with psychoeducation, cognitive restructuring of catastrophic misinterpretations, and interoceptive and situational exposure is the best-supported psychotherapy for panic disorder.

C. Helping her plan routes that avoid the situations where attacks occur

Planned avoidance reinforces agoraphobic avoidance and the belief that the situations are dangerous.

D. Dream analysis aimed at uncovering the unconscious source of her fear

Dream analysis is not an evidence-based first-line treatment for panic disorder and does not target the panic cycle.

Rationale: Panic disorder is maintained by catastrophic misinterpretation of bodily sensations and by avoidance. Cognitive behavioral treatment, such as Panic Control Treatment, combines psychoeducation, cognitive restructuring, and exposure to feared body sensations (interoceptive exposure) and situations. Guidelines name CBT as the first-line psychological treatment.

References: Barlow PCT — Panic Control Treatment: interoceptive exposure and cognitive restructuring · NICE guidelines — CG113 Generalised anxiety disorder and panic disorder in adults: CBT for panic disorder

☐ Approve

☐ Revise

☐ Retire

Notes:

A 24-year-old client meets criteria for borderline personality disorder. She cuts herself several times a month during intense emotional storms, and reducing self-harm is her top priority. Which treatment has the strongest research support for this presentation?

A. Brief solution-focused therapy limited to three sessions

A three-session brief model does not provide the skills training and structure needed for recurrent self-harm in BPD.

B. Prolonged exposure therapy targeting traumatic memories

Prolonged exposure targets PTSD; starting it with uncontrolled self-harm is not the first-line approach for BPD.

C. A psychoeducational group on personality disorder myths

Psychoeducation alone does not teach the emotion regulation and distress tolerance skills that reduce self-harm.

D. Comprehensive dialectical behavior therapy (DBT) program ✓ KEY

Comprehensive DBT (individual therapy, skills group, phone coaching, consultation team) has the strongest evidence for reducing self-harm in BPD.

Rationale: Dialectical behavior therapy was developed for chronically suicidal and self-harming clients with borderline personality disorder. Its comprehensive form combines individual therapy, skills training in mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness, between-session coaching, and a therapist consultation team. Guidelines recommend it when reducing recurrent self-harm is a priority.

References: NICE guidelines — CG78 Borderline personality disorder: consider a comprehensive DBT programme when reducing recurrent self-harm is a priority

☐ Approve ☐ Revise ☐ Retire Notes:

A clinical supervisor is reviewing the common factors versus specific factors debate with a counselor-in-training. Across comparisons of well-established (bona fide) therapies, which finding is MOST consistent with the common factors research?

A. Alliance quality predicts outcome more than which model is used ✓ KEY

Meta-analyses find the alliance reliably predicts outcome, while differences among bona fide treatments are small.

B. Specific techniques explain most of the variance in outcomes

Specific ingredients account for a small share of outcome variance compared with relational and client factors.

C. Counselor years of experience is the strongest single predictor

Years of experience is a weak predictor of outcome; it does not outweigh the alliance.

D. Matching the model to the diagnosis removes therapist effects

Therapist effects persist across models; some counselors get better results regardless of the approach used.

Rationale: The common factors view (Rosenzweig, Frank, Lambert, Wampold) holds that elements shared by effective therapies, such as the alliance, empathy, expectancy and a credible rationale, account for much of the change. Comparisons of bona fide treatments show small differences, while the alliance consistently predicts outcome. This does not make specific methods irrelevant; many conditions, such as PTSD and panic, have treatments with specific evidence.

References: Corey (Theory & Practice) — Integrative approaches: common factors and the therapeutic relationship as a predictor of outcome

► **Check:** Common factors findings (Wampold & Imel; Lambert) are well established, but confirm the Corey edition discusses common factors research explicitly.

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old man with obsessive-compulsive disorder is nervous about exposure and response prevention and asks whether there are other options. Which response BEST reflects shared decision making?

A. Tell him ERP is the evidence-based choice, so the plan cannot change

Insisting on one option without exploring his concerns and preferences removes him from the decision.

B. Let him choose any approach he likes without discussing the evidence

Shared decision making requires the counselor to bring evidence; deferring entirely abandons that role.

C. Review the options, evidence and his concerns, then decide jointly ✓ KEY

Shared decision making combines the counselor's knowledge of the evidence with the client's values and preferences in a joint choice.

D. Postpone treatment planning until his anxiety about ERP has resolved

Delaying planning leaves him untreated; fear of ERP is something to discuss and plan for, not wait out.

Rationale: Evidence-based practice integrates the best research evidence, clinical expertise and client characteristics, values and preferences. In shared decision making the counselor explains the options with their benefits and burdens, explores the client's concerns, and arrives at a plan together. Here that might include explaining how ERP is graded and client-paced.

References: APA CPG — Evidence-based practice: integrating patient values and preferences into treatment decisions

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old Mexican American client with depression tells her counselor that decisions in her family are made together and that she wants her mother involved in her care. Which approach to the treatment plan is MOST culturally responsive?

A. Include her mother in planning as she wishes, keeping core methods ✓ KEY

Cultural adaptation honors her value of family involvement while keeping the active ingredients of effective treatment.

B. Keep the plan individual, since adult clients decide treatment alone

This imposes an individualistic worldview and ignores the client's own stated wish for family involvement.

C. Assume every Latina client prefers family therapy over individual work

This stereotypes the client; adaptation should follow her stated preferences, not group generalizations.

D. Ask her mother to set the goals because she is the family decision maker

The client remains the decision maker about her own care; involving family does not mean handing over her goals.

Rationale: Culturally adapted treatment adjusts how a plan is delivered (who is involved, language, metaphors, goals framed in the client's values) while preserving the core components of effective treatment. The adaptation should be driven by the individual client's stated values, here familismo and collective decision making, rather than by assumptions about a group.

References: Sue & Sue (Counseling the Culturally Diverse) — Culturally appropriate intervention strategies: collectivism and family involvement · MSJCC (Ratts et al., 2016) — Counseling and advocacy interventions: integrating client worldview into counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A group practice announces that it is adopting measurement-based care. Which practice BEST describes this approach?

A. Giving a symptom inventory at intake and again at discharge only

Pre-post measurement evaluates results after the fact but cannot guide treatment while it is happening.

B. Routinely scoring brief measures and using them to guide treatment ✓ **KEY**

Measurement-based care means regular standardized measurement whose results are reviewed with the client and used to adjust treatment.

C. Basing progress notes on the counselor's sense of client improvement

Clinical judgment alone tends to miss clients who are not improving or are getting worse.

D. Sending yearly satisfaction surveys to all of the practice's clients

Satisfaction surveys are program evaluation; they do not track an individual client's symptoms over time.

Rationale: Measurement-based care is the systematic administration of brief, validated measures (for example, the PHQ-9 or GAD-7) at regular intervals, with results reviewed with the client and used to inform clinical decisions. It improves detection of clients who are not responding.

References: Hays (Assessment) — Use of assessment in counseling: monitoring client progress with standardized measures

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling center begins giving the Outcome Questionnaire-45 (OQ-45) before every session and sends counselors color-coded alerts based on expected recovery curves. According to research on this kind of routine outcome monitoring with feedback, which clients benefit MOST?

A. Clients who are improving faster than their expected recovery curve

On-track clients do about as well with or without feedback; they are not where the largest gains appear.

B. Clients whose intake scores fell within the normal, nonclinical range

Clients starting in the functional range have little room to improve, so feedback adds little for them.

C. Clients in brief treatment who are seen for only one or two sessions

Feedback needs repeated measurement over time to identify a trajectory and prompt a change in course.

D. Clients flagged as not on track, who are at risk of deteriorating ✓ **KEY**

Feedback alerts help counselors notice and respond to not-on-track clients, improving their outcomes and reducing deterioration.

Rationale: The OQ-45 measures symptom distress, interpersonal relations and social role functioning. Lambert and colleagues found that giving counselors feedback on clients predicted to be not on track, especially paired with clinical support tools that review alliance, motivation and supports, improved outcomes and reduced deterioration. Counselors on their own tend to underestimate which clients are getting worse.

References: Lambert (Outcome Monitoring) — OQ-45 feedback and alerts for clients not on track · Hays (Assessment) — Outcome assessment and routine outcome monitoring

☐ Approve ☐ Revise ☐ Retire Notes:

A 56-year-old client's scores on a weekly anxiety measure have risen by more than the measure's reliable change index since intake, and she arrives at session five looking tense. How should the counselor respond to this deterioration?

A. Keep the current plan, since symptoms often worsen before improving

Assuming early worsening is expected ignores the warning sign; reliable deterioration predicts poor outcome if unaddressed.

B. Discharge her and refer elsewhere because treatment is not working

Abrupt discharge without exploring the cause is premature and risks abandonment.

C. Review scores with her, explore causes and revise the plan together ✓ **KEY**

Sharing the data, checking the alliance, stressors and fit of the plan, and adjusting collaboratively is the recommended response.

D. Stop giving the measure, because her rising scores may upset her more

Dropping measurement removes the information needed to respond; the scores should be discussed openly.

Rationale: Reliable deterioration on an outcome measure is a signal to act, not to wait. The counselor discusses the change with the client, assesses risk, explores the alliance, life events and treatment fit, and adjusts the plan, consulting or referring for added services if needed. Documenting the change and the response is part of good practice.

References: Hays (Assessment) — Monitoring progress: reliable change and responding to nonresponse · Wiger (Documentation) — Progress notes and treatment plan updates when progress stalls or reverses

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 28-year-old client had a suicidal crisis over the weekend. He did not use his safety plan, saying it "didn't feel real" and that he had forgotten where he put it. He is now safe and engaged. What should the counselor do with the plan?

A. Revise it with him, find the barriers and make each step more usable ✓ **KEY**

Safety plans are living documents; reviewing what got in the way and rewriting steps in his words makes the plan more usable.

B. Replace it with a written no-suicide contract that he agrees to sign

No-suicide contracts lack evidence of preventing suicide and do not give the client coping steps.

C. Set the plan aside, since he has shown that safety planning fails him

One unused plan points to barriers to fix, not a reason to abandon an evidence-supported intervention.

D. Keep the original plan but tell him to read it once a day as homework

Repeating an unchanged plan ignores why it failed; it needs collaborative revision first.

Rationale: The Stanley-Brown Safety Planning Intervention is developed collaboratively, written in the client's own words, kept somewhere easy to reach (such as a phone), and reviewed and revised after crises. Asking what got in the way of using each step lets the counselor and client make the plan realistic.

References: Stanley-Brown SPI — Reviewing and revising the safety plan; identifying barriers to using each step

☐ Approve ☐ Revise ☐ Retire Notes: _____

While adding crisis resources to a safety plan, a 17-year-old client asks what happens if she contacts 988. Which description is accurate?

A. It is a line that sends police to the caller's location on every contact

Most 988 contacts are resolved by phone, text or chat; emergency dispatch is reserved for imminent danger.

B. It connects her by call, text or chat to trained crisis counselors 24/7 ✓ **KEY**

988 Suicide and Crisis Lifeline offers call, text and chat access to trained crisis counselors at any hour.

C. It is reserved for people who are in the middle of a suicide attempt

988 serves anyone in suicidal, mental health or substance use crisis, or worried about someone else.

D. It is a text-only service that answers messages during business hours

988 is available by phone, text and chat around the clock.

Rationale: The 988 Suicide and Crisis Lifeline became the national three-digit number in the United States in July 2022. It routes calls, texts and chats to trained crisis counselors 24 hours a day. In a Stanley-Brown safety plan it belongs in the step listing professionals and agencies to contact in a crisis.

References: SAMHSA 988 Lifeline — 988 by call, text or chat, 24/7 · Stanley-Brown SPI — Step 5: professionals and agencies to contact in a crisis

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor writes this objective for a client with panic disorder: "Client will reduce panic attacks from four per week to one or fewer per week, as recorded in a daily panic log." Which SMART element is missing?

A. A specific behavior that is targeted for change

The objective names a specific behavior: the frequency of panic attacks.

B. A way to measure progress toward the objective

The daily panic log and the counts per week make the objective measurable.

C. A target that is realistic for this client

Reducing attacks to one or fewer per week is a reasonable, attainable target for treated panic disorder.

D. A time frame for reaching the stated target ✓ **KEY**

The objective gives no date or number of weeks, so it is not time-bound.

Rationale: SMART objectives are specific, measurable, achievable (attainable), relevant and time-bound. Adding a time frame, such as "within 10 weeks," tells the client and counselor when to judge whether the objective was met and supports documentation of medical necessity and progress.

References: Wiger (Documentation) — Treatment plans: writing behaviorally measurable, time-bound objectives

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client with severe depression has worsened despite three weeks in an intensive outpatient program (three hours, three days a week). He is not suicidal, lives with a supportive spouse, and can stay safe at night, but he needs structured treatment most of each weekday. Which level of care BEST fits?

A. Weekly individual outpatient counseling plus a medication review

This is less intensive than the IOP he is already failing in, so it would be a step down when he needs a step up.

B. Continued IOP at the same schedule for another three weeks

He is worsening at this intensity; continuing unchanged does not match his increased need.

C. A partial hospitalization program attended most of each weekday ✓ KEY

PHP offers several hours of daily structured treatment while he returns home at night, matching his need without 24-hour care.

D. Admission to a locked inpatient psychiatric unit for stabilization

He has no imminent risk and can stay safe at home, so 24-hour inpatient care would be more restrictive than needed.

Rationale: Levels of care run from outpatient to intensive outpatient (about 9 or more hours a week for adults), partial hospitalization (about 20 or more hours a week, usually most of each weekday), and 24-hour residential or inpatient care. The best match is the least restrictive level that meets current need; worsening in IOP without imminent danger points to PHP.

References: LOCUS (ACCP) — Level-of-care intensity: intensive outpatient versus partial hospitalization

☐ Approve ☐ Revise ☐ Retire Notes:

Which client presentation MOST clearly meets criteria for inpatient psychiatric hospitalization rather than a less restrictive level of care?

A. A man with chronic passive death wishes, no plan, and strong supports

Passive ideation without plan or intent, with supports, can usually be managed in outpatient care with safety planning.

B. A woman with a plan, intent and access to means who cannot stay safe ✓ KEY

Imminent risk with a plan, intent, access to means and inability to maintain safety requires 24-hour protective care.

C. A teen with worsening grades and conflict who refuses to go to school

Functional decline without imminent danger calls for outpatient or intensive outpatient services, not hospitalization.

D. A man with panic attacks who missed work twice this month from fear

Panic with some missed work is treated effectively on an outpatient basis.

Rationale: Inpatient hospitalization is reserved for clients who are an imminent danger to themselves or others, or who are so impaired they cannot care for themselves, and who cannot be kept safe at a less restrictive level. Suicidal ideation with a plan, intent, access to means and an inability to engage in safety planning is the classic indication.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: suicide risk and the decision to hospitalize

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old client in counseling for perfectionism mentions that she has been vomiting after most meals, has lost 15 pounds in two months, and feels dizzy when she stands up. The counselor has no eating disorder training. What is the BEST course of action?

A. Continue the current work and add eating concerns to future goals

Purging with rapid weight loss and dizziness signals possible medical danger that cannot wait for future sessions.

B. End counseling now and give her a list of eating disorder programs

Abruptly ending services without a coordinated transition risks abandonment.

C. Read about eating disorders and begin treating them in next session

Reading alone does not provide the training and supervision needed to practice within competence.

D. Arrange a prompt medical exam and specialist referral with a release ✓ **KEY**

Her symptoms need urgent medical evaluation, and care should be coordinated with a qualified specialist with her consent.

Rationale: Purging, rapid weight loss and dizziness can indicate electrolyte imbalance or other medical instability, so a prompt medical evaluation comes first. Because eating disorder treatment falls outside this counselor's competence, the counselor coordinates a referral to a qualified provider with a signed release, supporting the transition rather than abandoning the client.

References: ACA Code of Ethics — C.2.a Boundaries of Competence; A.11.a Competence Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 42-year-old client is completing an intensive outpatient program for depression and will step down to weekly counseling. Which aftercare step does the MOST to reduce the risk that he drops out after discharge?

A. Book his first outpatient visit with a named counselor before he leaves ✓ **KEY**

A confirmed appointment with a named provider, made before discharge, is a warm handoff that improves follow-through.

B. Give him a list of local counselors to call once he feels he needs one

Leaving the client to make contact on his own increases the chance that no follow-up happens.

C. Tell him to return to the program if symptoms come back after discharge

This plans only for relapse and leaves a gap in continuing care.

D. Mail a copy of his discharge summary to his home address after he leaves

A summary sent to the client does not connect him to the next provider.

Rationale: The period right after leaving a more intensive level of care carries elevated risk of disengagement and relapse. Effective aftercare planning schedules the next appointment before discharge, shares records with the receiving provider with consent, and updates the safety and relapse prevention plans.

References: Wiger (Documentation) — Discharge summaries and aftercare recommendations

► **Check:** Warm-handoff and pre-discharge scheduling are standard continuity-of-care practice; confirm Wiger is the best fit or cite a SAMHSA continuity-of-care source.

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 50-year-old client was referred by his physician for heavy cannabis use. He says, "It helps me relax. I don't see what the problem is." Which intervention BEST matches his stage of change?

A. Help him set a quit date and list strategies for handling cravings

Quit dates and coping plans are action-stage tasks; pushing them now tends to raise resistance.

B. Teach relapse prevention skills for high-risk situations he may face

Relapse prevention fits the action and maintenance stages, after change has begun.

C. Build rapport and offer nonjudgmental feedback to raise awareness ✓ **KEY**

In precontemplation, consciousness raising and nonconfrontational feedback help the client begin to consider change.

D. Confront his denial firmly so that he accepts that he has a problem

Confrontation increases defensiveness and is not supported for clients in precontemplation.

Rationale: A client who sees no problem is in precontemplation. Stage-matched work at this point builds the relationship, raises awareness through information and personalized feedback, and explores the client's own view of the behavior. Action-oriented tasks fit later stages.

References: Transtheoretical Model — Precontemplation: consciousness raising and stage-matched processes of change · Miller & Rollnick (MI) — Avoiding the righting reflex and reducing discord

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses a decisional balance exercise with a client who is weighing whether to stop gambling. According to research on the Transtheoretical Model, how does decisional balance typically shift as people move from precontemplation toward action?

A. The cons of changing rise steadily until they overtake the pros

This reverses the pattern; perceived cons of change tend to decrease as people progress.

B. The pros of changing increase until they come to outweigh the cons ✓ **KEY**

Pros of change rise across stages and typically cross over the cons before the action stage.

C. Pros and cons both stay stable, and only self-efficacy shifts

Self-efficacy does rise, but decisional balance also shifts in a predictable way across stages.

D. Pros and cons drop together as the client stops thinking about it

Movement toward action involves more, not less, weighing of the pros of change.

Rationale: Decisional balance is the weighing of the pros and cons of changing. In precontemplation the cons outweigh the pros; the two are roughly balanced in contemplation, which feeds ambivalence; and the pros come to outweigh the cons before action. Prochaska described the pros as rising more steeply than the cons fall (the strong and weak principles).

References: Transtheoretical Model — Decisional balance across the stages of change; strong and weak principles of progress

☐ Approve ☐ Revise ☐ Retire Notes:

Counseling Skills and Interventions

60 items · NCE weight 30% (48 of 160 scored items)

125. nce-s-cou-121 · Counseling and Helping Relationships · Adlerian therapy · medium · In review

During a lifestyle assessment, an Adlerian counselor asks a 38-year-old client to describe a single, specific incident from early childhood that she can picture clearly, including what she felt at the time. The counselor is LEAST concerned with whether the memory is factually accurate. What is the main purpose of gathering these early recollections?

A. To uncover repressed trauma so it can be worked through in transference

Recovering repressed material and working it through in the transference is a psychoanalytic goal, not the Adlerian use of early memories.

B. To reveal her private logic and core convictions about self, others and life ✓ **KEY**

Adlerians treat early recollections as projective: the memories a person keeps reflect present convictions, goals and basic mistakes in the lifestyle.

C. To identify the conditioned stimuli that first triggered her current anxiety

Locating conditioned stimuli is a behavioral assessment task; Adlerians are not seeking a learning history of specific triggers.

D. To confirm the accuracy of her family history before choosing a diagnosis

Adlerians value early recollections for what the client selects and how she tells it, not for historical accuracy or diagnosis.

Rationale: In Adlerian lifestyle assessment, early recollections are single, specific memories, usually from before about age 10. Because people remember events consistent with their current view of life, the memories serve as a projective window into private logic, goals and basic mistakes. Factual accuracy matters little; the themes and the client's stance within the memory matter most.

References: Corey (Theory & Practice) — Adlerian Therapy: lifestyle assessment and early recollections

☐ Approve ☐ Revise ☐ Retire Notes: _____

An Adlerian counselor asks a client who feels stuck in a low mood to close his eyes and recall a pleasant memory, notice the feelings that come with it, then recall a painful memory and notice how his feelings shift, and then return to the pleasant one. The aim is to show him that he can influence his feelings by what he chooses to think about. This is called:

A. Rational emotive imagery

Rational emotive imagery (REBT) has the client imagine a feared event and change the emotion by changing the belief, not alternate pleasant and painful memories.

B. Acting as if

Acting as if invites the client to behave as though he were already the person he wishes to be, not to switch between memories.

C. The push-button technique ✓ KEY

The push-button technique alternates pleasant and unpleasant images so the client sees he can "push the button" for the feeling he wants.

D. Systematic desensitization

Systematic desensitization pairs relaxation with a graded fear hierarchy; it is a behavioral fear-reduction method.

Rationale: The Adlerian push-button technique teaches that feelings are not simply things that happen to us. By deliberately calling up pleasant and then unpleasant memories and noticing the emotional shift, the client learns he can create or change feelings by what he chooses to focus on, which supports responsibility and encouragement.

References: Corey (Theory & Practice) — Adlerian Therapy: reorientation techniques (push-button technique)

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old who failed her first algebra test tells her school counselor she is "just bad at math" and wants to drop the class. She studied every evening for the next test and raised her score to a C. Which counselor response BEST reflects the Adlerian use of encouragement?

A. "You're so smart. I was sure you would get an A one of these days."

This is praise that rates the person and sets an outcome standard; Adlerians distinguish such praise from encouragement.

B. "A C is still a C. Let's set a firm goal of a B on the next test."

This focuses on the shortfall and raises the bar, which can deepen discouragement rather than build courage.

C. "Maybe math just isn't your thing. Many people do fine without it."

This colludes with her discouraged belief that she lacks ability, which is the mistaken conviction to be challenged.

D. "You studied each night, and your score rose. Your effort paid off." ✓ KEY

Encouragement recognizes effort and improvement, building the courage and faith in herself that she can grow.

Rationale: Adlerians view many problems as rooted in discouragement, and they see encouragement as the most powerful method for change. Encouragement focuses on effort, improvement and the person's strengths, and it is not contingent on success. Praise, by contrast, rewards outcomes and can teach that worth depends on performing well.

References: Corey (Theory & Practice) — Adlerian Therapy: encouragement as a core process

☐ Approve ☐ Revise ☐ Retire Notes:

Carl Rogers (1957) proposed six conditions that are necessary and sufficient for therapeutic personality change. Besides the counselor's congruence, unconditional positive regard and empathy, which condition describes the state of the client entering the relationship?

A. The client is incongruent, being vulnerable or anxious ✓ KEY

Rogers specified that the client is in a state of incongruence, vulnerable or anxious; this is one of the six conditions.

B. The client has insight into the childhood origins of distress

Insight into childhood origins is a psychodynamic aim and was not among Rogers's conditions.

C. The client agrees to specific, measurable treatment goals

Measurable goal setting is more typical of behavioral and brief approaches; Rogers did not require it.

D. The client has received an accurate diagnosis from the counselor

Rogers considered diagnosis unnecessary for change and potentially harmful to the relationship.

Rationale: Rogers's six conditions are: two persons are in psychological contact; the client is incongruent, vulnerable or anxious; the counselor is congruent in the relationship; the counselor experiences unconditional positive regard; the counselor experiences empathic understanding of the client's internal frame of reference; and the client at least minimally perceives the counselor's regard and empathy. He held that no particular technique, insight or diagnosis was required.

References: Corey (Theory & Practice) — Person-Centered Therapy: necessary and sufficient conditions for change

☐ Approve ☐ Revise ☐ Retire Notes:

A 60-year-old client caring for his wife with dementia asks his counselor, "Just tell me straight: should I move her into a memory care facility or not?" Which counselor response is MOST consistent with a person-centered, nondirective stance?

A. "Most families in your situation find a facility is the better choice."

This gives advice from the counselor's frame of reference and shifts the decision away from the client.

B. "Let's list the pros and cons this week and score each one at home."

A structured decision task is a directive, problem-solving move rather than following the client's experience.

C. "You feel torn, and you wish someone would tell you what is right." ✓ KEY

This empathic reflection stays in his frame of reference and trusts his capacity to reach his own decision.

D. "Perhaps your guilt about her illness is what makes deciding so hard."

This is an interpretation that imposes the counselor's hypothesis about the cause of his difficulty.

Rationale: Person-centered counselors trust the client's actualizing tendency and capacity for self-direction. Rather than advising, assigning tasks or interpreting, they offer accurate empathy, genuineness and unconditional positive regard, so the client can explore his own experience and arrive at his own choice.

References: Corey (Theory & Practice) — Person-Centered Therapy: nondirective stance and the client as expert

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student is summarizing common criticisms of the person-centered approach for a theories exam. Which criticism is MOST often raised?

A. It casts the counselor as an expert who interprets the client's hidden motives

This describes classical psychoanalysis; person-centered counselors avoid the interpretive expert role.

B. Its low structure may not suit clients who want or expect active direction ✓ **KEY**

Critics note that a nondirective stance can leave some clients, including many from cultures that expect an active helper, without enough guidance.

C. It depends heavily on standardized testing and diagnosis to guide treatment

Rogers de-emphasized testing and diagnosis, so this is the opposite of the approach.

D. It treats the therapeutic relationship as secondary to specific techniques

The relationship is the core of person-centered therapy, which places little weight on techniques.

Rationale: Person-centered therapy is praised for its emphasis on the relationship and the client's resources, and its core conditions are well supported by alliance research. Common criticisms are that it can be too passive or unstructured for clients in crisis or those who expect direction, and that some practitioners reduce it to simple reflection without real presence.

References: Corey (Theory & Practice) — Person-Centered Therapy: limitations and criticisms; multicultural perspective

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client has been offered a job abroad and says he feels anxious about the choice because no one can make it for him and he will have to live with the results. How would an existential counselor MOST likely view this anxiety?

A. As normal anxiety that comes with freedom and can spur growth and choice ✓ **KEY**

Existentialists see normal anxiety as an unavoidable part of facing freedom and responsibility, and as a potential source of growth.

B. As a symptom to be reduced quickly with relaxation and breathing skills

Symptom reduction is a behavioral aim; existential counselors do not try to eliminate normal anxiety.

C. As the result of repressed impulses breaking through his ego defenses

This is a psychoanalytic account of anxiety, not an existential one.

D. As the product of an irrational demand that he dispute in homework

Disputing irrational demands is REBT; existential therapy treats this anxiety as a meaningful response to real givens.

Rationale: Existential therapy distinguishes normal anxiety, a fitting response to freedom, responsibility, death and isolation, from neurotic anxiety, which is out of proportion and blocks living. Rollo May described normal anxiety as something to face, not remove, because it can motivate authentic choice. The counselor helps the client own his freedom rather than escape the discomfort.

References: Corey (Theory & Practice) — Existential Therapy: normal and neurotic anxiety; freedom and responsibility

☐ Approve ☐ Revise ☐ Retire Notes:

Which pairing of existential theorist and contribution is accurate?

A. Viktor Frankl: the four ultimate concerns of death, freedom, isolation and meaninglessness

The four ultimate concerns were described by Irvin Yalom, not Frankl.

B. Irvin Yalom: logotherapy and the will to meaning as the primary human motivation

Logotherapy and the will to meaning are Frankl's contributions.

C. Rollo May: the I-Thou relationship as the basis of genuine human encounter

The I-Thou relationship comes from the philosopher Martin Buber, though existential counselors draw on it.

D. Rollo May: bringing existential ideas to American therapy and writing on anxiety ✓ KEY

May was a key figure in introducing existential thought to U.S. psychotherapy and wrote extensively on anxiety, courage and freedom.

Rationale: Viktor Frankl founded logotherapy, emphasizing the will to meaning. Rollo May helped bring European existential thought to American psychotherapy, writing on normal anxiety and the courage to face freedom. Irvin Yalom identified four ultimate concerns: death, freedom, isolation and meaninglessness. Martin Buber, a philosopher, described the I-Thou relationship.

References: Corey (Theory & Practice) — Existential Therapy: key figures (Frankl, May, Yalom) and philosophical roots

☐ Approve

☐ Revise

☐ Retire

Notes:

A client dreads sweating heavily when giving presentations, and the more he fears it, the more he sweats. His counselor asks him, before his next talk, to try to sweat as much as he possibly can and to show the audience just how much he can sweat. Which Frankl technique is this?

A. Dereflection

Dereflection redirects attention away from oneself and the symptom toward other meanings; it does not ask the client to wish for the symptom.

B. Flooding

Flooding is a behavioral exposure method of prolonged, intense contact with the feared stimulus, not a humorous wish for the symptom.

C. Paradoxical intention ✓ KEY

Frankl's paradoxical intention asks clients to intend the very thing they fear, breaking the cycle of anticipatory anxiety.

D. Ordeal therapy

Ordeal therapy (Haley) attaches a burdensome task to the symptom; it is strategic, not logotherapeutic.

Rationale: In paradoxical intention, Frankl asked clients to wish for or deliberately produce what they fear, often with humor. Doing so interrupts anticipatory anxiety, the fear of the symptom that keeps the symptom going. Frankl paired it with dereflection, which moves attention away from hyper-reflection on oneself.

References: Corey (Theory & Practice) — Existential Therapy: Frankl's logotherapy, paradoxical intention and dereflection

☐ Approve

☐ Revise

☐ Retire

Notes:

A client speaks of her partner and herself as "we" when describing every feeling and opinion. She says they "never disagree," and she becomes anxious and vague when the counselor asks what she alone wants. In gestalt terms, this blurring of the boundary between self and other is called:

A. Introjection

Introjection is swallowing others' beliefs and standards whole without examining them.

B. Confluence ✓ **KEY**

Confluence blurs the self-other boundary so that differences are not felt, keeping conflict and separate wants out of awareness.

C. Projection

Projection attributes to others the traits or feelings one disowns in oneself.

D. Deflection

Deflection avoids contact through humor, vagueness or changing the subject.

Rationale: Gestalt therapy names several disturbances at the contact boundary: introjection, projection, retroflection, deflection and confluence. In confluence the person merges with others, seeks constant agreement and loses touch with personal wants. Counselors help such clients notice differences and ask, "What do you feel? What do you want?"

References: Corey (Theory & Practice) — Gestalt Therapy: contact and resistances to contact (confluence)

☐ Approve ☐ Revise ☐ Retire Notes:

A client gives a long, detached account of an argument he had with his brother two years ago. As he talks, his voice tightens and he taps his foot quickly. Which response is MOST consistent with gestalt therapy?

A. "What are you aware of right now as you tell me this? Notice your foot." ✓ **KEY**

This brings him into the here and now and directs attention to present, nonverbal experience, the heart of gestalt awareness work.

B. "Why do you think you and your brother end up fighting like that so often?"

Gestalt counselors avoid "why" questions, which invite intellectualizing rather than present awareness.

C. "What were you telling yourself during the argument that upset you?"

Identifying self-talk is a cognitive-behavioral move focused on the past event rather than present experience.

D. "It sounds as if your brother reminds you of a parent from childhood."

This is a psychodynamic interpretation; gestalt counselors favor client discovery over counselor interpretation.

Rationale: Gestalt therapy holds that awareness in the present moment is itself curative. Counselors shift clients from talking about past events to experiencing what is happening now, often by noticing body language, voice and breath. "What" and "how" questions are preferred to "why" questions, which tend to produce explanations instead of contact.

References: Corey (Theory & Practice) — Gestalt Therapy: the now, awareness, and attention to nonverbal behavior

☐ Approve ☐ Revise ☐ Retire Notes:

Arnold Beisser's paradoxical theory of change is a foundation of gestalt practice. Which statement BEST captures it?

A. Change happens when the counselor prescribes the symptom the client most wants to stop

Prescribing the symptom is a strategic family therapy directive, not Beisser's theory.

B. Change happens when clients replace irrational demands with flexible preferences

Replacing demands with preferences is the REBT model of change.

C. Change happens when the counselor interprets resistance until the client gains insight

Interpreting resistance to produce insight is a psychoanalytic method.

D. Change happens when one becomes what one is, not when one tries to be what one is not ✓ **KEY**

Beisser held that change comes through full awareness and acceptance of who one is now, not through striving to be different.

Rationale: Beisser's paradoxical theory of change states that the more a person tries to be who he or she is not, the more the person stays the same. Change occurs when the person fully experiences and owns current reality. Gestalt counselors therefore do not push for change; they heighten awareness of what is, trusting that integration follows.

References: Corey (Theory & Practice) — Gestalt Therapy: the paradoxical theory of change

☐ Approve ☐ Revise ☐ Retire Notes:

A client who believes it would be unbearable to look foolish is assigned homework: wear a brightly mismatched outfit to the grocery store and loudly ask a clerk where the milk is while standing next to it. She is to notice that she can tolerate others' reactions and still accept herself. This REBT technique is called:

A. Paradoxical intention

Paradoxical intention (Frankl) asks clients to wish for a feared symptom, such as trying to blush or sweat.

B. A shame-attacking exercise ✓ **KEY**

Shame-attacking exercises have clients do harmless "foolish" acts in public to learn that disapproval is tolerable and not a verdict on their worth.

C. Rational emotive imagery

Rational emotive imagery is done in imagination, not by acting in public.

D. Systematic desensitization

Systematic desensitization pairs relaxation with a graded hierarchy, usually in imagination, to reduce fear.

Rationale: Albert Ellis developed shame-attacking exercises as in vivo homework. Clients deliberately do something they consider shameful but that harms no one, then work on not putting themselves down. REBT relies heavily on such homework to practice new rational beliefs between sessions.

References: Corey (Theory & Practice) — REBT: behavioral techniques, homework and shame-attacking exercises

☐ Approve ☐ Revise ☐ Retire Notes:

After failing the bar exam, a 27-year-old law graduate says, "I failed, so I'm a failure." Her REBT counselor works toward unconditional self-acceptance (USA). Which goal BEST reflects this concept?

A. Restore her self-esteem by listing past successes that show her real worth

Ellis criticized self-esteem as conditional on achievement; rebuilding it through successes keeps worth tied to performance.

B. Help her see the exam was unimportant so the result need not upset her

REBT does not deny that the outcome is bad; appropriate disappointment is a healthy negative emotion.

C. Rate the failed exam as a poor result while accepting herself as fallible ✓ KEY

USA means rating behaviors and outcomes, not the whole self, and accepting oneself as a fallible human regardless of performance.

D. Trace how her parents' high standards first taught her to fear failure

REBT focuses on present beliefs; tracing childhood origins is not needed for unconditional self-acceptance.

Rationale: Ellis argued that global self-rating is irrational because a person is too complex to sum up with one score. Unconditional self-acceptance asks clients to evaluate their actions and outcomes, sometimes harshly, while accepting themselves without conditions. He contrasted this with self-esteem, which rises and falls with success and approval.

References: Corey (Theory & Practice) — REBT: unconditional self-acceptance versus self-esteem

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor asks a client to imagine vividly that he is turned down for a date, let himself feel the depressed panic that follows, and then, while keeping the image, change the feeling to disappointment. Afterward they discuss what he told himself to make the change. This REBT technique is:

A. Rational emotive imagery ✓ KEY

Rational emotive imagery has clients hold a vivid image of a feared event and shift an unhealthy emotion to a healthy one by changing the belief.

B. The push-button technique

The Adlerian push-button technique alternates pleasant and unpleasant memories; it does not keep one feared image constant.

C. Covert sensitization

Covert sensitization pairs an unwanted behavior with imagined aversive consequences to reduce that behavior.

D. Systematic desensitization

Systematic desensitization pairs relaxation with graded fear images; it does not ask the client to feel and then transform an emotion.

Rationale: In rational emotive imagery, the client imagines one of the worst things that could happen, allows the unhealthy emotion, and then changes it to a healthy negative emotion such as disappointment or concern. The change happens through a shift in belief, which the client then identifies and practices, often daily as homework.

References: Corey (Theory & Practice) — REBT: emotive techniques (rational emotive imagery)

☐ Approve ☐ Revise ☐ Retire Notes:

A client says she has felt sad "every single day" since moving to a new city. Her counselor asks, "Think about this past week. When was the sadness even a little lighter, and what was different then?" This is an example of:

A. A scaling question

Scaling questions ask the client to rate the problem or progress on a numerical scale, such as 0 to 10.

B. The miracle question

The miracle question asks the client to imagine the problem solved overnight and describe what would be different.

C. A coping question

Coping questions ask how the client has kept going despite difficulty, not when the problem was less present.

D. An exception question ✓ KEY

Exception questions search for times the problem was absent or less severe, so the client can repeat what worked.

Rationale: Solution-focused counselors assume that problems are never constant. Exception questions direct attention to times the problem was absent or milder and to what the client was doing then. These exceptions become the raw material for solutions: do more of what works.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: exception questions

☐ Approve ☐ Revise ☐ Retire Notes:

A father raising three children alone after his wife's death says nothing is going well, and he cannot recall a single time things were better. Which response is MOST consistent with a solution-focused coping question?

A. "Let's find out what in your past is keeping you so stuck right now."

Searching the past for the cause of the problem is problem-focused, which SFBT deliberately avoids.

B. "With so much on you, how have you managed to keep the kids fed and here?" ✓ KEY

A coping question highlights the strength and effort already present when the client sees no exceptions.

C. "On a scale of 0 to 10, how bad would you say things are for you today?"

This is a scaling question, and framing it around how bad things are keeps the focus on the problem.

D. "It sounds like the grief is still very raw. Tell me more about the loss."

This empathic invitation is appropriate in many models but is not an SFBT coping question.

Rationale: When clients cannot identify exceptions, solution-focused counselors use coping questions, such as "How have you managed to keep going?" These questions bring out the client's resources and small successes already present in a hard situation. They respect the difficulty while shifting attention toward competence.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: coping questions and client competence

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old is sent to counseling by his school after a fight. He says he has no problem and is only there because the principal required it. In de Shazer's terms, what is the counselor-client relationship, and what is the BEST response?

A. Complainant; ask him to watch for times when things go a bit better

A complainant sees a problem but not himself as part of the solution; this teen does not yet see a problem at all.

B. Customer; give him an action task to practice before the next session

A customer sees a problem and is ready to act; assigning tasks to an uncommitted client invites failure.

C. Visitor; compliment him for coming and avoid assigning him any tasks ✓ KEY

A visitor sees no problem to work on; the counselor offers sincere compliments, explores what he might want, and gives no tasks.

D. Customer; confront his denial and set firm goals for his anger at once

Confronting denial is not a solution-focused stance, and the teen is not in a customer relationship.

Rationale: De Shazer described three kinds of relationships: visitor (no complaint or goal yet), complainant (sees a problem but expects others to change) and customer (sees a problem and is willing to act). These describe the relationship at a moment, not fixed client traits. With visitors, counselors build rapport through compliments and curiosity about what the client might want, saving tasks for later.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: customer, complainant and visitor relationships; compliments

☐ Approve ☐ Revise ☐ Retire Notes:

A mother of twins says, "A good mother never needs help, so I'm failing." Her narrative counselor asks, "Where did you learn that idea about good mothers? Who benefits when mothers believe it? Are there times it doesn't fit your life?" The counselor is MAINLY engaged in:

A. Deconstructing a dominant discourse ✓ KEY

Deconstruction questions unpack the taken-for-granted discourses behind a problem story so the client can choose whether to accept them.

B. Disputing an irrational belief

Disputing tests a belief's logic and evidence (REBT); narrative questions trace a belief's cultural origins and effects instead.

C. Interpreting an unconscious conflict

Narrative counselors take a not-knowing stance and do not interpret hidden motives.

D. Naming an Adlerian basic mistake

Basic mistakes in the lifestyle are an Adlerian concept tied to private logic, not cultural discourse.

Rationale: Michael White and David Epston drew on the idea that problem stories are shaped by dominant cultural discourses. Deconstruction exposes where those discourses came from, whose interests they serve and how they have shaped the client's thin, problem-saturated description of herself. This opens space for a thicker, preferred story.

References: Corey (Theory & Practice) — Narrative Therapy: deconstruction and dominant discourses

☐ Approve ☐ Revise ☐ Retire Notes:

A young adult reclaiming her life from an eating disorder invites three friends to a session. While she talks with the counselor, the friends listen; then they describe what stood out to them, what it brought up in their own lives and how it moved them, and she responds. This narrative practice is known as:

A. Externalizing conversation

Externalizing separates the person from the problem in language; it does not depend on an audience of witnesses.

B. Family sculpting

Family sculpting (Satir) has members arrange one another physically to show relationships.

C. Reflecting team critique

Outsider witnesses do not critique or evaluate the client; reflecting teams also come from Andersen's work with clinicians.

D. Definitional ceremony ✓ KEY

In a definitional ceremony, outsider witnesses hear the preferred story and retell it, which authenticates and thickens it.

Rationale: White adapted the definitional ceremony from anthropologist Barbara Myerhoff. The client tells her preferred story, outsider witnesses retell what resonated with them, and the client then responds to the retellings. The audience helps acknowledge and thicken the new identity claims so they are not held by the client alone.

References: Goldenberg (Family Therapy) — Narrative therapy: definitional ceremonies and outsider witnesses

► **Check:** Confirm that Goldenberg (9th ed.) covers definitional ceremonies/outsider witnesses; Corey covers them only briefly. Reflecting-team distractor wording ("critique") chosen so it is clearly not the practice described.

☐ Approve ☐ Revise ☐ Retire Notes:

After a session, a narrative counselor mails a client a letter that quotes the client's own words about how he stood up to "the Anger," notes questions the counselor is still curious about and names the steps he took. What is the MAIN purpose of such therapeutic letters?

A. To give him the counselor's formal diagnosis and expert view of his case

Narrative counselors avoid the expert, diagnostic stance; letters center the client's words, not a diagnosis.

B. To document and thicken his emerging story so it lasts between sessions ✓ KEY

Letters record the alternative story in lasting form, extending and thickening it beyond the session.

C. To assign behavioral homework with rewards for each completed step

Contingent rewards for homework belong to behavior therapy, not narrative letter writing.

D. To keep a legal record of the session in place of case notes

Therapeutic letters are an intervention addressed to the client; they do not replace clinical records.

Rationale: David Epston in particular developed letter writing as a narrative practice. Letters capture unique outcomes and the client's preferred story in a durable, rereadable form, turning a thin problem story into a thicker, richly described one. White and Epston also used certificates and documents to celebrate new identities.

References: Corey (Theory & Practice) — Narrative Therapy: letters and documents that support the alternative story

☐ Approve ☐ Revise ☐ Retire Notes:

Which counselor practice BEST reflects the egalitarian relationship emphasized in feminist therapy?

A. Keeping a neutral blank screen so the client can project onto the counselor

The blank-screen stance is psychoanalytic and widens the power gap feminist counselors try to reduce.

B. Making the counselor's diagnosis the main basis for all treatment decisions

Feminist counselors are cautious about diagnostic labels and do not make the counselor's view the sole authority.

C. Demystifying counseling and treating the client as expert on her life ✓ KEY

Demystifying the process and honoring the client's expertise reduce the power difference and support collaboration.

D. Avoiding any talk of values or social issues so the counselor stays neutral

Feminist counselors see neutrality as impossible and openly address values and social context.

Rationale: Feminist therapy views the counseling relationship as egalitarian. Counselors demystify the process through informed consent, share their orientation and values, use appropriate self-disclosure and treat clients as experts on their own lives. This models healthier uses of power and supports empowerment.

References: Corey (Theory & Practice) — Feminist Therapy: the egalitarian therapeutic relationship

☐ Approve ☐ Revise ☐ Retire Notes:

A home health aide who immigrated five years ago blames herself for "being weak" because she has not confronted an agency that underpays her. Her counselor helps her map the differences in power between her and the agency, including economic, legal and gender factors, and explore ways she might access power. This feminist intervention is:

A. Power analysis ✓ KEY

Power analysis examines how unequal access to power shapes the client's situation and identifies ways to use or gain power.

B. Gender-role analysis

Gender-role analysis explores messages about how women and men should be; this work centers on broader power differences.

C. Cognitive reframing

Reframing offers a new meaning for the same facts but does not map structural power or plan to access it.

D. Assertiveness drills

Assertiveness training may follow, but the intervention described is an analysis of power, not skill rehearsal.

Rationale: In power analysis, feminist counselors help clients see the differences in power between groups in society and in their own relationships, and how these shape their options and self-blame. Clients then identify kinds of power they can use. Shifting the view from personal weakness to social context is a core feminist move.

References: Corey (Theory & Practice) — Feminist Therapy: power analysis and intervention

☐ Approve ☐ Revise ☐ Retire Notes:

Relational-cultural theory (RCT), developed by Jean Baker Miller, Judith Jordan and colleagues at the Stone Center, challenges traditional models of development. According to RCT, psychological growth occurs MAINLY through:

A. Separating from others to achieve independence and a firm, autonomous self

RCT critiques the separation-individuation model that treats autonomy as the goal of maturity.

B. Resolving unconscious drives and conflicts formed in early psychosexual stages

This describes Freudian theory, not RCT's focus on connection.

C. Learning new behaviors through reinforcement in structured, graded tasks

This is a behavioral account of change and omits the relational focus of RCT.

D. Growth-fostering relationships marked by mutual empathy and empowerment ✓ **KEY**

RCT holds that people grow through and toward connection, in relationships of mutual empathy and mutual empowerment.

Rationale: Relational-cultural theory argues that people grow through connection rather than separation. Growth-fostering relationships involve mutual empathy and mutual empowerment, and chronic disconnection, often linked to marginalization and power, leads to isolation and suffering. RCT is closely tied to feminist therapy and attends to culture and social justice.

References: Corey (Theory & Practice) — Feminist Therapy: relational-cultural theory (Miller, Jordan)

☐ Approve ☐ Revise ☐ Retire Notes:

Near the end of a session about her drinking, a client says, "I'm going to call the outpatient program on Monday and set up an intake." Using the DARN-CAT framework, which kind of change talk is this?

A. Desire

Desire statements express wanting or wishing ("I want to cut down"), not a pledge to act.

B. Commitment ✓ **KEY**

"I'm going to" signals an intention to act, which is commitment, a form of mobilizing change talk.

C. Reasons

Reasons statements give specific benefits of change ("I'd sleep better").

D. Taking steps

Taking steps describes an action already done ("I called them yesterday"); this call has not happened yet.

Rationale: Miller and Rollnick group change talk into preparatory talk (Desire, Ability, Reasons, Need) and mobilizing talk (Commitment, Activation, Taking steps). Commitment language such as "I will" or "I'm going to" predicts change more strongly than preparatory talk. Counselors listen for, reflect and strengthen change talk.

References: Miller & Rollnick (MI) — Evoking: preparatory (DARN) and mobilizing (CAT) change talk

☐ Approve ☐ Revise ☐ Retire Notes:

A father who smokes in the car with his young children says being a present, healthy dad is "the most important thing in my life." Which response BEST develops discrepancy in a way consistent with the spirit of motivational interviewing?

A. "Your kids matter most to you. How does smoking fit with that?" ✓ KEY

This reflects his stated value and invites him to voice the gap between it and his behavior, evoking his own reasons.

B. "You need to know that secondhand smoke in a car harms children's lungs."

Lecturing reflects the righting reflex and tends to provoke sustain talk rather than change talk.

C. "If your kids matter most, then you would have quit smoking long ago."

This is a confrontational argument that challenges his sincerity and is likely to increase discord.

D. "Let's set a quit date now, and I'll give you a list of nicotine aids."

Jumping to planning before he voices his own motivation skips evoking and risks resistance.

Rationale: Developing discrepancy means helping clients notice the gap between their values or goals and their current behavior, so the client, not the counselor, voices the argument for change. The MI spirit of partnership, acceptance, compassion and evocation rules out lecturing or confrontation, which tend to trigger sustain talk and discord.

References: Miller & Rollnick (MI) — MI spirit; developing discrepancy between values and behavior

☐ Approve ☐ Revise ☐ Retire Notes:

After building rapport with a client who has diabetes, depression and money worries, the counselor and client agree that they will concentrate on his eating and medication habits for now. In the four processes of motivational interviewing, this step is called:

A. Engaging

Engaging is establishing the working relationship, which has already occurred here.

B. Evoking

Evoking draws out the client's own motivation for change once a direction has been chosen.

C. Focusing ✓ KEY

Focusing is developing and keeping a specific direction or change goal for the conversation.

D. Planning

Planning comes last, when the client is ready to commit to a specific plan of action.

Rationale: Miller and Rollnick describe four overlapping processes: engaging (building the relationship), focusing (agreeing on a direction), evoking (drawing out the client's own motivation) and planning (developing commitment and a concrete plan). Each builds on the one before, and counselors may return to earlier processes as needed.

References: Miller & Rollnick (MI) — The four processes: engaging, focusing, evoking, planning

☐ Approve ☐ Revise ☐ Retire Notes:

After a client self-harms, her DBT therapist works with her to trace, step by step, her vulnerability factors that day, the prompting event, each thought, feeling and action leading to the behavior, and the consequences that followed. This procedure is called:

A. Radical acceptance

Radical acceptance is a distress tolerance skill of fully accepting reality as it is; it does not map links to a behavior.

B. A thought record

A thought record tracks situations, automatic thoughts and alternative responses (CBT), not a full behavioral sequence.

C. Wise mind practice

Wise mind is a mindfulness skill integrating emotion mind and reasonable mind, not an analysis of a behavior.

D. Behavioral chain analysis ✓ KEY

Chain analysis maps every link from vulnerability and prompting event to the target behavior and its consequences.

Rationale: In DBT, a behavioral chain analysis is done after a target behavior such as self-harm. Therapist and client identify vulnerabilities, the prompting event, the chain of thoughts, emotions, sensations and actions, the behavior and its consequences. Each link becomes a point where a skill could be used instead, and a solution analysis follows.

References: Corey (Theory & Practice) — Dialectical behavior therapy: behavioral chain analysis

☐ Approve ☐ Revise ☐ Retire Notes:

A client in a DBT skills group practices asking her roommate to pay his share of the rent. At one point she says, "If you get your half to me by Friday, I'll stop bringing it up and things can get back to normal between us." In the DEAR MAN skill, this statement illustrates:

A. Assert

Assert is clearly asking for what one wants or saying no ("I need your half of the rent").

B. Reinforce ✓ KEY

Reinforce means rewarding the person ahead of time by explaining the positive effects of getting what one asks for.

C. Negotiate

Negotiate is being willing to give to get, such as offering alternatives or a compromise.

D. Express

Express is sharing one's feelings and opinions about the situation using "I" statements.

Rationale: DEAR MAN is a DBT interpersonal effectiveness skill: Describe the situation, Express feelings, Assert the request, Reinforce the other person in advance, stay Mindful, Appear confident and Negotiate. Reinforcing explains what the other person gains by agreeing, which makes a yes more likely.

References: Linehan (DBT Skills Training Manual) — Interpersonal effectiveness: DEAR MAN skills

☐ Approve ☐ Revise ☐ Retire Notes:

A client says she freezes and goes silent whenever her manager raises his voice. Her DBT therapist responds, "Growing up, speaking up when your father yelled got you punished, so freezing makes sense given what you learned." In Linehan's levels of validation, this response BEST illustrates:

A. Validating in terms of past learning or biology ✓ KEY

This level explains the response as understandable given the client's history, even if it does not fit the present situation.

B. Normalizing in terms of present circumstances

This level says the response would make sense for anyone in the current situation, without reference to the client's history.

C. Articulating the client's un verbalized feelings

This level puts into words what the client has not said, such as a feeling she has not named.

D. Radical genuineness with the client as an equal

Radical genuineness treats the client as a capable equal rather than fragile; it does not explain a response by history.

Rationale: Linehan described six levels of validation: (1) staying awake and attentive, (2) accurate reflection, (3) articulating the un verbalized, (4) validating in terms of past learning or biology, (5) normalizing in terms of present context, and (6) radical genuineness. Validation balances DBT's change strategies, communicating that the client's responses make sense, which supports the acceptance side of the acceptance-change dialectic.

References: Linehan (DBT Skills Training Manual) — Validation and its levels · Corey (Theory & Practice) — Dialectical behavior therapy: validation strategies and the acceptance-change dialectic

► **Check:** The six validation levels are set out in Linehan (1997, "Validation and psychotherapy"); confirm the skills manual edition cited presents all six.

☐ Approve ☐ Revise ☐ Retire Notes:

After four sessions on grief, a 58-year-old client leaves a voicemail cancelling her next appointment and saying she 'just doesn't need counseling anymore.' The counselor believes important work remains. What is the BEST response?

A. Close the file without further contact, since the client has made her choice

Respecting autonomy does not require silence; a brief outreach to offer closure and options is good practice.

B. Reach out, invite a closing session to review progress and offer referral options ✓ KEY

This respects her decision while offering pretermination counseling, a chance to explore her reasons and a path back or elsewhere.

C. Call several times each week until she agrees to resume her regular appointments

Repeated pressure disregards client autonomy and could feel coercive.

D. Mail a letter stating that ending now will most likely cause her grief to worsen

Predicting harm to persuade her to return is a pressure tactic and is not supported by the facts at hand.

Rationale: Clients may end counseling early for many reasons, including satisfaction, cost, discomfort or a rupture in the relationship. The counselor respects the choice but reaches out once to explore the decision, offer a closing session to consolidate gains and provide referrals, leaving the door open to return.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Gladding (Counseling: A Comprehensive Profession) — Termination: premature termination initiated by clients

☐ Approve ☐ Revise ☐ Retire Notes:

After 12 sessions and several changes in approach, a client with social anxiety shows no measurable improvement, and the client and counselor agree that counseling is not helping. According to the ACA Code of Ethics, what should the counselor do?

A. Continue the same sessions, since ending now would amount to abandonment

Ending a relationship that is not helping, with proper preparation and referral, is not abandonment; continuing without benefit is the concern.

B. End services at the next visit without discussion to avoid more expense

An abrupt ending without discussion denies the client pretermination counseling.

C. Keep meeting monthly until the client's insurance benefits are used up

Continuing only to use available benefits does not serve the client and raises ethical concerns.

D. Provide pretermination counseling and recommend other service providers ✓ **KEY**

The Code directs counselors to end services that are not benefiting the client, with pretermination counseling and referral as needed.

Rationale: ACA Code A.11.c directs counselors to terminate when it is reasonably apparent that the client no longer needs help, is not likely to benefit or is being harmed. Counselors provide pretermination counseling and recommend other service providers when necessary, so the client leaves with a path to help that may fit better.

References: ACA Code of Ethics — A.11.c Appropriate Termination

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor's individual sessions with a 7-year-old will end in three weeks because he has met his goals. Which approach to ending is MOST developmentally appropriate?

A. Mark the remaining sessions on a calendar together and plan a goodbye ritual ✓ **KEY**

Concrete countdowns and a closing ritual give a young child advance notice, predictability and a sense of closure.

B. Tell him at the start of the final session so he has less time to be upset

Last-minute notice gives the child no time to prepare and can feel like abandonment.

C. Avoid mentioning the ending and simply stop scheduling him for sessions

An unexplained stop leaves the child confused and may feel like rejection.

D. Ask his teacher to explain the ending to him after the last session is over

The ending belongs in the relationship; handing it off to someone else after the fact avoids closure.

Rationale: Children understand time concretely, so endings are announced several sessions ahead and made visible, for example by counting down sessions on a calendar. A goodbye ritual, such as a drawing, a memory book or a review of what the child learned, honors the relationship and consolidates gains.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: preparing for and ending the relationship

☐ Approve ☐ Revise ☐ Retire Notes:

158. nce-s-cou-154 · Counseling and Helping Relationships · reflection, paraphrasing and summarizing · hard · In review

A client says, 'I keep picking up extra shifts even though I'm exhausted. My parents gave up everything to bring us to this country.' Which counselor response is the BEST example of a reflection of meaning?

A. You're feeling exhausted and a bit trapped by all the extra shifts you work.

This names emotions, which makes it a reflection of feeling rather than of meaning.

B. You've been working many extra shifts lately, even though you're worn out.

This restates content, which makes it a paraphrase.

C. Working this hard seems to be a way of honoring what your parents sacrificed. ✓ KEY

This captures the value and purpose that give the behavior its significance for the client, the core of reflecting meaning.

D. Perhaps you overwork because part of you fears you'll disappoint your parents.

This offers a new, counselor-generated explanation (a hidden fear), which is an interpretation.

Rationale: Reflection of meaning, in Ivey's microskills, goes beneath content and feeling to the values, purposes and significance a client attaches to experience. It stays close to what the client has implied, whereas interpretation offers a new frame drawn from the counselor's perspective.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: reflection of meaning versus interpretation

☐ Approve ☐ Revise ☐ Retire Notes:

159. nce-s-cou-155 · Counseling and Helping Relationships · reflection, paraphrasing and summarizing · easy · In review

A client says, 'Between my night classes and my daughter's soccer schedule, I haven't had a free evening in a month.' Which counselor response is the BEST paraphrase?

A. Your evenings have been taken up by school and family commitments for weeks. ✓ KEY

This restates the essential content in fresh, concise words, which is what a paraphrase does.

B. Between night classes and soccer, you haven't had a free evening in a month.

Repeating the client's words almost verbatim is parroting, which can sound mechanical.

C. You're feeling worn out and maybe resentful that there's no time for you.

This focuses on emotion, making it a reflection of feeling.

D. It sounds like you may have trouble saying no when others need something.

This adds a counselor hypothesis about the client, which is interpretation.

Rationale: A paraphrase restates the content of a client's message in the counselor's own, usually briefer, words. It shows accurate listening and invites the client to confirm or correct. Verbatim repetition (parroting) adds little and can feel mechanical.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: paraphrasing and the pitfall of parroting

☐ Approve ☐ Revise ☐ Retire Notes:

A client who was open in earlier sessions has arrived late three weeks in a row, answers in single words and avoids looking at the counselor. The counselor senses growing distance between them. Which response BEST demonstrates immediacy?

A. You've been late three times now, and our policy is that sessions can't run over.

Stating policy addresses logistics, not what is happening in the relationship.

B. Coming late may be your way of resisting the hard work we have started to do here.

This is an interpretation of motive and may increase defensiveness.

C. Let's go back to the goals we set in our first session and review your progress.

Shifting to goals sidesteps the relational shift the counselor is noticing.

D. I'm sensing some distance between us lately, and I wonder what's happening for you. ✓ **KEY**

The counselor tentatively names what is occurring between them right now and invites the client to explore it.

Rationale: Immediacy is the counselor's direct, tentative comment on what is happening in the counseling relationship in the present moment. Used when tension or distance develops, it brings the relationship itself into the conversation so ruptures can be explored and repaired.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling skills: relationship immediacy

☐ Approve ☐ Revise ☐ Retire Notes:

During intake, a client looks down or past the counselor most of the time and moves her chair back slightly when the counselor leans in. She explains that in her family, looking an elder or professional directly in the eye is considered disrespectful. What should the counselor do?

A. Gently encourage more eye contact to build trust and openness over time

This imposes the counselor's cultural norm on the client.

B. Adjust own eye contact and distance, following the client's comfort cues ✓ **KEY**

Attending behaviors are adapted to the client's cultural norms for gaze and personal space.

C. Note the gaze avoidance as a possible sign of depression or social anxiety

The client has given a cultural explanation; pathologizing it risks misdiagnosis.

D. Lean in closer to show warmth so that she feels more engaged in session

She has signaled discomfort with closeness; moving in would ignore her cue.

Rationale: Guidelines such as SOLER reflect a particular cultural norm. Comfort with eye contact (kinesics) and conversational distance (proxemics) varies across cultures and individuals, so culturally responsive attending means reading and following the client's cues rather than treating one style as the standard.

References: Sue & Sue (Counseling the Culturally Diverse) — Nonverbal communication: proxemics and kinesics

☐ Approve ☐ Revise ☐ Retire Notes:

During child-centered play therapy, a 5-year-old holds up a paintbrush and asks the counselor, 'What color should I make the sky?' Which response BEST fits this approach?

A. Blue, just like the real sky outside.

Answering for the child directs the play and takes away a chance for self-direction.

B. Why do you want me to pick for you?

"Why" questions ask young children to explain motives and can feel evaluative.

C. In here, you get to decide that. ✓ **KEY**

Returning responsibility communicates trust in the child and builds decision-making and self-control.

D. Let's paint it together and find out.

Joining to share the choice still shifts direction away from the child.

Rationale: In child-centered play therapy (Axline, Landreth), the counselor follows the child's lead. Key responses include tracking (describing what the child is doing, such as "You're putting that one on top") and returning responsibility, which hands decisions back to the child to foster autonomy and self-responsibility.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy responses

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor trains parents to hold weekly 30-minute nondirective play sessions with their own child at home, using reflective listening, tracking and limit setting, while the counselor supervises and gives feedback on their skills. Which approach is this?

A. Filial therapy ✓ **KEY**

Filial therapy, developed by Bernard and Louise Guerney, trains parents as the agents of child-centered play sessions.

B. Theraplay

Theraplay (Jernberg) is an adult-led, attachment-based approach using structured, playful engagement, not nondirective play.

C. Parent-child interaction therapy

PCIT (Eyberg) coaches parents live, usually by earpiece, and includes a parent-directed discipline phase.

D. Sandplay therapy

Sandplay (Kalff, building on Lowenfeld's World Technique) is a Jungian method in which the child builds scenes in a sand tray with the therapist.

Rationale: Filial therapy treats parents as partners: the counselor teaches child-centered play skills, supervises practice sessions and then helps parents generalize the sessions to home. It aims to strengthen the parent-child relationship as the vehicle of change.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: filial therapy and sand tray approaches

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client says he badly wants a close relationship, yet he pulls away whenever someone gets close because he expects to be hurt and doubts that he deserves love. In Bartholomew and Horowitz's four-category model of adult attachment, which style BEST fits?

A. Preoccupied

Preoccupied adults see themselves negatively but others positively, so they pursue closeness anxiously rather than withdraw.

B. Dismissing-avoidant

Dismissing adults view themselves positively and downplay any wish for closeness.

C. Secure

Secure adults hold positive views of self and others and are comfortable with intimacy.

D. Fearful-avoidant ✓ KEY

He wants closeness but avoids it, with negative views of both himself and others.

Rationale: Bartholomew and Horowitz crossed a model of self (worthy or unworthy) with a model of others (trustworthy or not). Secure is positive-positive, preoccupied is negative self and positive other, dismissing is positive self and negative other, and fearful is negative on both, producing a desire for closeness alongside avoidance.

References: Berk (Development Through the Lifespan) — Early adulthood: adult attachment styles and intimate relationships

► **Check:** Confirm Berk 7th ed covers Bartholomew and Horowitz's four-category adult model; the model itself is standard.

☐ Approve ☐ Revise ☐ Retire Notes:

Bowlby proposed that through repeated experiences with caregivers, a child builds mental representations of whether others are dependable and whether the self is worthy of care, which then guide expectations in later relationships. What did Bowlby call these representations?

A. Secure base

A secure base (emphasized by Ainsworth) is the caregiver from whom a child explores, not the mental representation itself.

B. Internal working models ✓ KEY

Bowlby's internal working models are expectations about self and others built from early caregiving.

C. Object constancy

Object constancy is Mahler's term for holding a stable inner image of the caregiver.

D. Cognitive schemas

Schemas are Piaget's and Beck's terms for organized knowledge or core beliefs, not Bowlby's attachment concept.

Rationale: Bowlby argued that attachment experiences are internalized as working models of self and others. These models operate largely outside awareness and shape how a person seeks support, interprets others' behavior and forms close relationships later in life.

References: Berk (Development Through the Lifespan) — Bowlby's ethological theory of attachment: internal working model

☐ Approve ☐ Revise ☐ Retire Notes:

For two years a Korean American graduate student rejected anything she saw as White culture and worked only within Asian American groups. Lately she feels uneasy with that rigid stance, wonders whether it limits her personal autonomy and is sorting out which values are truly her own. In the Racial/Cultural Identity Development (R/CID) model, she is MOST likely in which stage?

A. Dissonance

Dissonance is the earlier stage in which experiences first challenge a conforming, majority-oriented view.

B. Resistance and immersion

This describes her previous two years of rejecting the dominant culture and endorsing her own group.

C. Introspection ✓ KEY

Discomfort with rigid group views and a search for personal autonomy mark the introspection stage.

D. Integrative awareness

Integrative awareness reflects a secure, flexible identity that she is still working toward.

Rationale: The R/CID model (Atkinson, Morten and Sue, expanded by Sue and Sue) describes five stages: conformity, dissonance, resistance and immersion, introspection and integrative awareness. Introspection follows immersion, as the person questions rigid loyalty to the group and works out an individual identity.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial/Cultural Identity Development (R/CID) model

☐ Approve ☐ Revise ☐ Retire Notes:

After a classmate's remark about her family's background, a 15-year-old Mexican American student begins asking her grandparents about their lives in Mexico, joins a Latinx student club and reads Chicano history, though she has not settled what her heritage means to her. In Phinney's model of ethnic identity, she is in which stage?

A. Ethnic identity search (moratorium) ✓ KEY

Active exploration of heritage, often prompted by an encounter, without firm commitment marks the search stage.

B. Unexamined ethnic identity

This first stage involves little interest in or exploration of ethnicity; she is now actively exploring.

C. Achieved ethnic identity

Achievement requires a secure, committed sense of what ethnicity means, which she has not yet reached.

D. Integrative awareness

Integrative awareness is the final stage of the R/CID model, not part of Phinney's model.

Rationale: Phinney applied Marcia's identity statuses to ethnicity: an unexamined identity (diffuse or foreclosed), an ethnic identity search or moratorium often sparked by a significant experience, and an achieved identity marked by clarity and commitment. Exploration supported by family and community fosters well-being.

References: Berk (Development Through the Lifespan) — Adolescence: ethnic identity (Phinney)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who holds several privileged identities is working with a client from a marginalized racial and religious group. She sets aside time to explore with the client how their differences in power and privilege affect trust, communication and the work they are doing together. In the MSJCC, this falls MOST directly within which developmental domain?

A. Counselor self-awareness

Self-awareness concerns the counselor's own identities and biases; here the focus is how the two identities interact.

B. Counseling relationship ✓ KEY

This domain addresses how privileged and marginalized statuses of counselor and client shape the relationship.

C. Client worldview

Client worldview concerns understanding the client's identities, values and experiences of oppression.

D. Counseling and advocacy interventions

This domain covers intervening at individual through international levels, not exploring the dyad itself.

Rationale: The MSJCC places the counselor-client pair in one of four quadrants defined by privileged and marginalized statuses. It then describes four developmental domains: counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions, with attitudes and beliefs, knowledge, skills and action (AKSA) within each.

References: MSJCC (Ratts et al., 2016) — Quadrants and developmental domains: the counseling relationship

☐ Approve ☐ Revise ☐ Retire Notes: _____

An older counseling text lists ataque de nervios as a 'culture-bound syndrome.' DSM-5-TR uses a broader term that includes cultural syndromes, cultural idioms of distress and cultural explanations or perceived causes. What is that term?

A. Culture-specific disorders

DSM-5-TR moved away from framing these patterns as disorders tied to one culture.

B. Atypical presentations

This implies a deviation from a supposedly typical norm, the view the newer framework rejects.

C. Ethnic variant syndromes

This is not a DSM-5-TR term.

D. Cultural concepts of distress ✓ KEY

DSM-5-TR groups cultural syndromes, idioms of distress and explanations under cultural concepts of distress.

Rationale: DSM-5-TR replaced "culture-bound syndromes" with cultural concepts of distress, recognizing that all distress is culturally shaped and that these concepts may not map onto a single DSM disorder. The Cultural Formulation Interview helps counselors explore the client's own understanding of the problem.

References: DSM-5-TR — Section III: Culture and Psychiatric Diagnosis, cultural concepts of distress

☐ Approve ☐ Revise ☐ Retire Notes: _____

Using James and Gilliland's six-step model of crisis intervention, a counselor has defined the problem from the client's point of view, ensured the client's safety and provided support. What is the counselor's NEXT step?

A. Obtain commitment to a plan

Obtaining commitment is the sixth and final step.

B. Make a concrete plan

Making plans is the fifth step, after alternatives are explored.

C. Examine alternatives ✓ KEY

The fourth step is exploring options, supports and coping mechanisms with the client.

D. Complete a full diagnosis

Formal diagnosis is not a step in the model; assessment runs throughout.

Rationale: James and Gilliland's model has three listening steps (define the problem, ensure client safety, provide support) followed by three acting steps (examine alternatives, make plans, obtain commitment). Assessment of the client's state and safety continues throughout all six steps.

References: James & Gilliland (Crisis Intervention Strategies) — Six-step model: define the problem, ensure safety, provide support, examine alternatives, make plans, obtain commitment

☐ Approve ☐ Revise ☐ Retire Notes:

In Kanel's ABC model of crisis intervention, a counselor first builds rapport through attending and reflection. The counselor then identifies the precipitating event, the client's perception of it and the resulting distress and impaired functioning. What is this second phase called?

A. Boiling down the problem ✓ KEY

The B phase narrows the focus to the trigger, the client's perception and the subjective distress.

B. Coping

Coping is the C phase, which follows and focuses on current and new coping strategies.

C. Achieving contact

Achieving contact is the A phase, the rapport building described first in the stem.

D. Brief safety planning

Safety is assessed throughout but is not the name of a phase in the ABC model.

Rationale: Kanel's ABC model has three phases: A, achieving (developing and maintaining) contact; B, boiling down the problem by identifying the precipitating event, perceptions and distress; and C, coping, in which the counselor explores past and new coping and builds a plan. Its focus on perception reflects the view that crises arise from how events are interpreted.

References: Kanel (A Guide to Crisis Intervention) — ABC model: A, achieving contact; B, boiling down the problem; C, coping

☐ Approve ☐ Revise ☐ Retire Notes:

Before launching a support group at a refugee resettlement agency, a counselor studies the community's needs, the agency's resources and support, and cultural attitudes toward discussing personal problems in a group. In the ASGW Best Practice Guidelines, this planning task is called:

A. Pre-group screening

Screening assesses individual prospective members, not the community and organizational context.

B. Ecological assessment ✓ KEY

ASGW asks group workers to assess the community, agency and cultural context before forming a group.

C. Professional disclosure

Professional disclosure informs members about the leader and the group; it is not a needs assessment.

D. Group processing

Processing is the later, ongoing reflection on what happens in sessions.

Rationale: The ASGW Best Practice Guidelines organize group work into planning, performing and group processing. In planning, an ecological assessment of community needs, organizational support and cultural factors guides whether and how a group is offered. Processing, by contrast, means regular reflection on sessions with co-leaders, members or supervisors.

References: ASGW Best Practice Guidelines — Best Practice in Planning: ecological assessment

☐ Approve ☐ Revise ☐ Retire Notes:

The T-group (training group), first developed at the National Training Laboratories in Bethel, Maine, used here-and-now feedback to teach human relations skills. Whose field theory and group dynamics research led to this movement?

A. Carl Rogers

Rogers developed the basic encounter group, which grew alongside but separately from T-groups.

B. Jacob Moreno

Moreno developed psychodrama and is credited with coining the term group therapy.

C. Joseph Pratt

Pratt led early educational "classes" for patients with tuberculosis in the early 1900s.

D. Kurt Lewin ✓ KEY

Lewin's field theory and group dynamics work led his colleagues to found NTL and the T-group.

Rationale: Kurt Lewin studied how group forces shape individual behavior. His work led to the National Training Laboratories in 1947, where T-groups used here-and-now feedback to build interpersonal awareness. Rogers later extended the experiential group into the encounter group movement.

References: Gladding (Counseling: A Comprehensive Profession) — History of group work: Lewin, T-groups and encounter groups

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor suggests that a recently widowed client try a community bereavement group run by and for people who have lost a spouse, with no professional leader. Compared with a counseling or therapy group, what BEST characterizes this kind of group?

A. It uses a professional leader trained to treat diagnosable mental disorders

This describes a psychotherapy group, not a peer-run group.

B. It follows a strict time-limited format with formal screening of all members

Mutual-aid groups are usually open and ongoing, without formal clinical screening.

C. It is peer-led, with members who share a concern offering one another support ✓ KEY

Self-help and mutual-aid groups are led by members who share the problem and help one another.

D. It focuses mainly on reshaping personality through deep insight and analysis

Reconstructive personality change is a goal of some long-term therapy groups, not of mutual-aid groups.

Rationale: Self-help and mutual-aid groups, such as Alcoholics Anonymous or widow-to-widow programs, are organized around a shared concern and led by members rather than professionals. They emphasize support, hope, universality and practical help, and they can complement rather than replace professional counseling.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: self-help and mutual-aid groups

☐ Approve

☐ Revise

☐ Retire

Notes:

175. nce-s-cou-171 · Group Counseling and Group Work · group leadership skills and therapeutic factors · medium · In review

A 15-year-old in a court-mandated anger management group sits with his hood up and says, 'I'm only here because the judge made me.' What is the leader's BEST initial response?

A. Acknowledge he did not choose to come and invite him to say how that feels ✓ KEY

Openly naming involuntary status respects his autonomy and turns resistance into material for the group.

B. Explain that members who do not take part may be reported back to the court

Leading with threats reinforces the power struggle and deepens resistance.

C. Ask the other members to persuade him that the group will really help him

Recruiting peers to convince him puts him on the spot and can feel like ganging up.

D. Ignore the remark and move on so he gets no attention for his resistance

Ignoring a direct statement misses a chance to engage him and may model avoidance.

Rationale: Adolescents, and especially involuntary members, often test leaders and guard their autonomy. Acknowledging their reluctance openly, respecting choices about what to disclose and inviting them to talk about being sent helps build trust and often reduces resistance more than pressure or avoidance.

References: Gladding (Counseling: A Comprehensive Profession) — Group work with adolescents and involuntary members

☐ Approve

☐ Revise

☐ Retire

Notes:

In the ninth session of a cohesive interpersonal group, Malik tells Rosa that her advice-giving feels 'preachy,' and Rosa responds sharply. The room goes quiet. What should the leader do?

A. Move on to a new topic to protect the cohesion the group has built so far

Avoiding conflict teaches members that strong feelings are unsafe and wastes useful material.

B. Help both speak directly to each other and explore what the exchange stirs up ✓ **KEY**

Keeping the conflict in the here-and-now with direct communication makes it a source of learning.

C. Ask Malik and Rosa to work out their disagreement privately after the session

Moving conflict outside the group fosters subgrouping and removes it from shared work.

D. Point out that Malik's feedback was too harsh and ask him to rephrase it now

Taking sides shuts down exploration and positions the leader as judge.

Rationale: Yalom notes that conflict is inevitable and, in a cohesive group, can be highly productive. The leader keeps it in the here-and-now, encourages members to speak directly and own their reactions, and helps the group examine what the exchange reveals about each person's interpersonal patterns.

References: Yalom & Leszcz (Group Psychotherapy) — Conflict in the therapy group (the advanced group)

☐ Approve ☐ Revise ☐ Retire Notes:

177. nce-s-cou-173 · Group Counseling and Group Work · group leadership skills and therapeutic factors · medium · In review

A member describes feeling ignored at family dinners. The leader asks her, 'Is there anyone in this room who you feel might be ignoring you right now?' In Yalom's two-tiered approach to the here-and-now, this question is an example of:

A. Process illumination

Illumination is the second tier, in which the group reflects on and makes sense of what has happened.

B. Universality

Universality is a therapeutic factor, the relief of learning others share one's problems.

C. Catharsis

Catharsis is the expression of strong emotion, not a leader technique for shifting focus.

D. Here-and-now activation ✓ **KEY**

Steering an outside concern into members' relationships in the room is the first, activating tier.

Rationale: Yalom describes two tiers of here-and-now work. The leader first activates the here-and-now by moving the focus from outside, past events to members' reactions to one another in the room. The leader then illuminates the process, helping members reflect on and understand what just occurred; activation without illumination yields emotion without learning.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 6: The here-and-now, activation and illumination of process

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-year-old's frequent stomachaches keep his worried mother home from work. A therapist directs the boy to pretend to have a stomachache at set times and the mother to pretend to comfort him, turning the symptom into play while the mother takes back her caretaking role. This technique is MOST closely associated with:

A. Salvador Minuchin

Minuchin's structural therapy uses enactments, joining and boundary making rather than pretend directives.

B. Murray Bowen

Bowen emphasized differentiation, triangles and genograms, with the therapist as a calm coach.

C. Cloé Madanes ✓ KEY

Madanes developed pretend techniques within strategic family therapy.

D. Virginia Satir

Satir's experiential approach focuses on communication stances, self-esteem and sculpting.

Rationale: Madanes viewed some children's symptoms as attempts to help a parent that also upset the family hierarchy. Pretend techniques ask the child to pretend to have the symptom and the parent to pretend to help, making the symptom voluntary and playful so it loses its function and the parent resumes an appropriate caregiving role.

References: Goldenberg (Family Therapy) — Strategic family therapy: Madanes and pretend techniques

☐ Approve ☐ Revise ☐ Retire Notes:

In a group setting, a guide uses a client's family chronology to have members role-play scenes from the lives of the client's parents and grandparents, helping the client see them as people and let go of old learnings. This experiential technique, developed by Virginia Satir, is called:

A. Family reconstruction ✓ KEY

Satir's family reconstruction re-enacts multigenerational scenes to free the "star" from outdated patterns.

B. Psychodrama

Psychodrama, created by Jacob Moreno, uses role-play but is not Satir's multigenerational technique.

C. Family sculpting

Sculpting arranges members in a physical tableau of present relationships rather than re-enacting family history.

D. Genogram construction

A genogram maps multigenerational patterns on paper (associated with Bowen) and does not involve role-play.

Rationale: Satir's family reconstruction guides a "star" through key events in three generations, using a family chronology and role players. Family sculpting, also used by Satir and by Duhl and Kantor, positions members physically to show closeness, distance and power. Both are experiential techniques that make family patterns visible and felt.

References: Goldenberg (Family Therapy) — Experiential family therapy: Satir, family reconstruction and sculpting

☐ Approve ☐ Revise ☐ Retire Notes:

Parents of a 14-year-old have grounded him for longer and longer periods for breaking curfew, with no effect. In MRI brief therapy terms, which outcome would represent second-order change?

- A. The parents extend the grounding from one week to a full month
More of the same solution is first-order change in degree, not in kind.
- B. The family changes the rules governing how curfew is set and handled ✓ KEY
Altering the rules of the system itself is second-order change.
- C. The parents add the loss of his phone to the grounding as a penalty
Adding another penalty stays within the same punitive pattern, so it is first-order.
- D. The parents keep the same rules but enforce them more strictly
Stricter enforcement of unchanged rules is a first-order adjustment.

Rationale: Watzlawick, Weakland and Fisch distinguished first-order change, a change within a system whose rules stay the same, from second-order change, a change in the system's rules or structure. MRI therapists look for interventions such as reframing that interrupt "more of the same" attempted solutions.

References: Goldenberg (Family Therapy) — MRI interactional view: first-order and second-order change

☐ Approve ☐ Revise ☐ Retire Notes:

Parents decide to stop going into their 3-year-old's room when she cries at bedtime. On the second and third nights, her crying becomes louder and lasts longer than before. How should the counselor explain this?

- A. Spontaneous recovery; the plan has failed and should be stopped now
Spontaneous recovery is the return of a behavior after extinction appeared complete; the plan has not failed.
- B. Negative reinforcement; her crying is being made stronger by them
Nothing aversive is being removed after crying, so this is not negative reinforcement.
- C. Punishment; ignoring the crying is causing her new emotional harm
Withholding attention is extinction, not punishment, and the increase is predictable.
- D. Extinction burst; it is expected and fades if parents stay consistent ✓ KEY
A temporary surge in a behavior when reinforcement stops is an extinction burst.

Rationale: When a previously reinforced behavior no longer produces its reinforcer, it often increases in frequency or intensity before declining, a pattern called an extinction burst. If caregivers give in during the burst, they reinforce a more intense version of the behavior on an intermittent schedule, so consistency is essential.

References: Corey (Theory & Practice) — Behavior therapy: operant extinction and the extinction burst

☐ Approve ☐ Revise ☐ Retire Notes:

To reduce a child's hand-biting, a behavior analyst delivers a token at the end of every 10-minute interval in which no hand-biting occurs, regardless of what else the child is doing. Which procedure is this?

A. DRA (alternative behavior)

DRA reinforces a specific replacement behavior, such as asking for help, rather than mere absence of the target.

B. DRL (low rates of behavior)

DRL reinforces the target behavior when it occurs at or below a set rate, for behaviors to reduce but not eliminate.

C. DRO (other behavior) ✓ KEY

DRO reinforces the absence of the target behavior over an interval.

D. Premack principle

The Premack principle uses a high-probability activity to reinforce a low-probability one.

Rationale: Differential reinforcement of other behavior (DRO) delivers reinforcement when the target behavior has not occurred for a set interval, sometimes called differential reinforcement of zero rates. DRA reinforces a specified alternative behavior, DRI an incompatible one and DRL a low rate of the target itself.

References: Corey (Theory & Practice) — Behavior therapy: differential reinforcement procedures

☐ Approve ☐ Revise ☐ Retire Notes:

183. nce-s-cou-179 · Human Growth and Development · psychodynamic and psychoanalytic theory · medium · In review

An 18-month-old who recently toddled off happily to explore now repeatedly returns to his mother, shadows her, wants her to share each discovery and becomes upset when she leaves, yet sometimes pushes her away. In Mahler's separation-individuation theory, he is in which subphase?

A. Rapprochement ✓ KEY

Around 16 to 24 months, awareness of separateness brings renewed clinging alongside a push for autonomy.

B. Practicing

Practicing (about 10 to 16 months) is the earlier, exhilarated period of exploring with little concern for the mother.

C. Differentiation

Differentiation (about 5 to 10 months) is when the infant first begins to distinguish self from mother.

D. Object constancy

Object constancy (about 24 to 36 months) brings a stable inner image of the mother that eases separations.

Rationale: Mahler described separation-individuation subphases following the early autistic and symbiotic phases: differentiation, practicing, rapprochement and the move toward object constancy. The rapprochement crisis, with its mix of clinging and pushing away, is often linked to later difficulties with closeness and separateness.

References: Corey (Theory & Practice) — Psychoanalytic therapy: object relations theory and Mahler's separation-individuation

☐ Approve ☐ Revise ☐ Retire Notes:

A 2-year-old insists on bringing a worn stuffed rabbit to child care and clutches it when her father leaves. In Winnicott's object relations theory, what function does this transitional object serve?

A. It signals an insecure attachment that will need professional treatment

Transitional objects are a normal part of development, not a sign of pathology.

B. It soothes her and bridges her move from union with a parent to separateness ✓ **KEY**

Winnicott saw the transitional object as an intermediate space that helps the child tolerate separation.

C. It reflects fixation at the oral stage caused by weaning that came too early

This applies Freud's psychosexual fixation, not Winnicott's concept.

D. It is a part-object she splits into all-good and all-bad parts to manage fear

Splitting and part-objects come from Melanie Klein, not the transitional object concept.

Rationale: Winnicott described the transitional object as the child's first "not-me" possession, standing between inner and outer reality and helping the child manage the gradual move to independence. It develops within a holding environment provided by a good-enough mother, who meets needs well at first and then fails in small, tolerable ways.

References: Corey (Theory & Practice) — Psychoanalytic therapy: object relations (Winnicott, Klein)

► **Check:** Confirm Corey's object-relations section names Winnicott's transitional object and good-enough mother.

☐ Approve ☐ Revise ☐ Retire Notes:

Core Counseling Attributes

16 items · NCE weight 8% (13 of 160 scored items)

185. nce-s-cor-033 · Social and Cultural Diversity · counselor self-awareness and bias · medium · In review

A counselor who sincerely holds egalitarian values reviews her recorded sessions for supervision. She notices that with her Latino clients she offers fewer reflections, interrupts more often, and moves to advice sooner than with her White clients. She had not been aware of the difference. This pattern BEST illustrates

A. implicit bias operating outside her conscious awareness ✓ KEY

Automatic, unintended differences in behavior toward a group, despite consciously held egalitarian beliefs, are the hallmark of implicit (unconscious) bias.

B. explicit prejudice that she consciously holds but conceals

Explicit prejudice is consciously endorsed. The counselor sincerely holds egalitarian values and was unaware of the pattern.

C. cultural encapsulation produced by a narrow training program

Wrenn's cultural encapsulation describes defining reality through one's own cultural lens. It does not specifically explain unaware, group-specific differences in her microskills.

D. countertransference rooted in conflicts from her own family

Countertransference stems from the counselor's personal history with significant others. Nothing suggests a family conflict; the pattern tracks client ethnicity.

Rationale: Implicit bias refers to attitudes and associations that operate automatically and outside awareness, so well-intentioned counselors can treat groups differently without meaning to. Sue and Sue stress that good intentions do not protect against unintentional bias; recorded sessions, supervision and ongoing self-examination are how counselors detect it.

References: Sue & Sue (Counseling the Culturally Diverse) — Implicit bias and unintentional racism in well-intentioned helping professionals

☐ Approve

☐ Revise

☐ Retire

Notes: _____

A 47-year-old client in a larger body seeks counseling for grief after his mother's death. He has not raised any concern about his weight. Over three sessions, the counselor finds herself suggesting exercise plans and asking about his eating. Which action BEST reflects counselor self-awareness?

A. Add a weight-loss goal to the plan, since weight affects mood and grief

This imposes the counselor's agenda on a client who came for grief and acts on the bias rather than examining it.

B. Notice her assumptions about his weight and refocus on the grief he brought ✓ KEY

Recognizing the counselor-driven assumption and returning to the client's own goal is self-aware, client-centered practice consistent with ACA A.4.b.

C. Refer him to a dietitian so his eating habits can be addressed by an expert

A referral the client did not request still acts on the counselor's assumption about his body rather than on his presenting concern.

D. Ask him directly whether his weight played a part in his mother's death

This question extends the counselor's weight focus into his grief and may shame him; it reflects the bias rather than awareness of it.

Rationale: Self-awareness means noticing when personal values or assumptions, including those about body size, are steering the work. ACA Standard A.4.b directs counselors to avoid imposing their own values and to respect the client's goals. The counselor should examine where the weight focus came from and return to the grief the client brought.

References: ACA Code of Ethics — A.4.b Personal Values · Corey (Theory & Practice) — The counselor as a person: self-awareness and personal values

☐ Approve ☐ Revise ☐ Retire Notes:

The week after a national election, a client spends several minutes praising a candidate the counselor strongly opposes. The counselor notices her jaw tightening. The client then asks, "You agree with me, right?" Which counselor response BEST reflects self-aware, ethical practice?

A. Honestly, I see it differently, but I respect your right to your own view.

Sharing political disagreement shifts focus to the counselor and risks imposing values; congruence does not require disclosing every opinion.

B. I do. A lot of people feel that way, so I can see why you are hopeful now.

Feigning agreement is incongruent and also brings the counselor's supposed politics into the session.

C. I keep my politics out of here, but I want to hear what this means for you. ✓ KEY

This gently declines to disclose, keeps the focus on the client, and invites exploration, which manages the counselor's reaction without acting on it.

D. Since we differ on this, another counselor might be a better fit for you.

ACA A.11.b cautions against referral based solely on the counselor's personal values; a political difference is not grounds for referral.

Rationale: Counselors are expected to notice their own value reactions, including political ones, and keep them from steering the session (ACA A.4.b). Self-disclosure of opinions should serve the client, and a politically charged question is usually best met by declining warmly and exploring its meaning for the client. Referral solely because of a values difference is discouraged under A.11.b.

References: ACA Code of Ethics — A.4.b Personal Values · ACA Code of Ethics — A.11.b Values Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

At a community mental health center, a 36-year-old client referred by his caseworker glares at the counselor and says, "You're just another paid stranger. You don't actually care what happens to me." Which counselor response BEST conveys warmth and respect?

A. I have years of training, so you can trust that I know how to help you.

Defending credentials centers the counselor and argues with the client's experience rather than respecting it.

B. I can't work with you if you speak to me in that tone during sessions.

The client is distrustful, not abusive. An early limit on tone reads as punitive and confirms his expectation that no one cares.

C. Let's set that aside and get through the intake questions for today.

Ignoring the statement dismisses the client's concern and treats him as paperwork, the opposite of respect.

D. Sounds like people haven't earned your trust. I'd like to, at your pace. ✓ KEY

This reflects the meaning behind the hostility without defensiveness and communicates a respectful wish to earn trust over time.

Rationale: Warmth and respect are tested most when a client is hostile. A nondefensive response that reflects the client's distrust, treats it as understandable, and leaves the client in control of the pace communicates care. Defending oneself, setting premature limits, or ignoring the comment tend to confirm the client's expectations.

References: Gladding (Counseling: A Comprehensive Profession) — Warmth, respect and working with reluctant or resistant clients

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 60-year-old Vietnamese immigrant father comes to counseling about conflict with his teenage son. In the first session he says, "You are the expert. Please tell me what I should do." Which counselor approach BEST combines respect with responsiveness to his request?

A. Offer some concrete suggestions and structure, checking how each idea fits his family ✓ KEY

Honoring his expectation of an active, expert helper while asking how ideas fit his family respects both his cultural frame and his autonomy.

B. Explain that counselors do not give advice and that he must find his own answers

A strictly nondirective stance may be experienced as unhelpful or as a lack of competence, and it dismisses his stated preference.

C. Tell him exactly what rules to set for his son, since he asked for clear direction

Directing without his input ignores his knowledge of his own family and values; structure should still be collaborative.

D. Ask him to first explore his deeper feelings about his son before any guidance

Insisting on emotional exploration before addressing his practical request disregards what he came for and may lead to early dropout.

Rationale: Sue and Sue note that many clients, especially those from cultures that value hierarchy and practical help, expect the counselor to take an active, directive role, and that a purely nondirective style can lead to early termination. Respect means meeting the client's expectations with structure and concrete input while still checking that suggestions fit the client's values.

References: Sue & Sue (Counseling the Culturally Diverse) — Culturally appropriate helping styles: directive versus nondirective approaches

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 38-year-old Army veteran referred for military sexual trauma tells the counselor in her second session, "I'm not ready to talk about what happened. I don't know if I ever will be." Which counselor response BEST respects the client's pace?

A. Avoiding it keeps symptoms going, so let's try to cover the details today.

Pressing for details before the client consents overrides her choice and can feel retraumatizing.

B. That's okay. You decide what we talk about and when, and we can go slowly. ✓ **KEY**

This affirms her control over disclosure, a core trauma-informed principle, and conveys respect for her readiness.

C. Could you give me just a short outline so I know what we're working with?

Asking for an outline right after she declines still pressures disclosure and disregards her stated limit.

D. Unless you talk about it, I'm not sure that counseling can help you much.

Making help conditional on disclosure is coercive and undermines safety and trust.

Rationale: Respecting a client's pace is especially important with trauma survivors. SAMHSA's trauma-informed principles emphasize safety, trustworthiness, and client voice and choice; counselors let clients control when and how much they disclose, building stabilization and trust first.

References: SAMHSA TIP 57 — Trauma-informed principles: safety, choice and client control over disclosure

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old client in outpatient treatment for opioid use returns after relapsing following 90 days of abstinence. He looks down and says, "You must be so disappointed in me." Which counselor response BEST demonstrates unconditional positive regard?

A. I'm a bit disappointed, but I know you can do better than this next time.

This makes the counselor's regard contingent on his behavior, which is the opposite of unconditional positive regard.

B. Relapse is just part of recovery, so it's really not a big deal at all.

This minimizes a serious event and offers reassurance rather than acceptance of the person and his feelings.

C. My regard for you hasn't changed. I'm glad you came back to talk about it. ✓ **KEY**

This separates his worth from the relapse and welcomes him back, communicating acceptance that does not depend on his behavior.

D. Let's go over what you did wrong so this does not happen to you again.

Opening with what he did wrong frames the session around judgment; relapse review can come later from a stance of acceptance.

Rationale: Unconditional positive regard is acceptance of the client as a person, not approval of every behavior, and it does not rise and fall with the client's performance. After a relapse, communicating steady regard reduces shame and makes honest exploration of what happened more likely.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes:

According to Carl Rogers, incongruence between a person's self-concept and actual experience develops primarily when the person

A. fails to resolve unconscious conflicts between the id and superego

This is a psychoanalytic explanation of conflict, not Rogers's account of incongruence.

B. learns maladaptive behavior patterns through repeated reinforcement

This is a behavioral explanation; Rogers focused on the self-concept and conditions of worth.

C. holds irrational beliefs that they must win approval from others

Irrational beliefs about needing approval belong to Ellis's REBT, not person-centered theory.

D. internalizes conditions of worth set by significant others ✓ KEY

Rogers held that conditional regard from significant others leads people to internalize conditions of worth, which produces incongruence.

Rationale: Rogers proposed that when love and approval are conditional, people internalize conditions of worth and deny or distort experiences that do not fit them, creating incongruence. Unconditional positive regard in counseling counters this by allowing clients to accept all of their experience without losing the counselor's regard.

References: Corey (Theory & Practice) — Person-centered therapy: conditions of worth and incongruence

☐ Approve ☐ Revise ☐ Retire Notes:

In person-centered theory, a counselor who is congruent is one who

A. shares every thought and feeling with the client as soon as it arises

Congruence does not mean impulsively disclosing all reactions; expression should be appropriate and in the client's service.

B. is aware of inner experience and expresses it authentically when fitting ✓ KEY

Congruence is the match between the counselor's inner experience, awareness of it, and outward expression, without a professional facade.

C. hides personal reactions to maintain a neutral and professional stance

Hiding behind a professional facade is the opposite of congruence in Rogers's view.

D. mirrors the client's emotional tone and posture to build rapport fast

Mirroring is a rapport technique; it can be done without any genuineness at all.

Rationale: Rogers described congruence, or genuineness, as the counselor being real: inner experience, awareness and communication are consistent, with no facade. Corey notes that this does not mean impulsively sharing every feeling; genuine expression is appropriate and serves the relationship.

References: Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old client who says coworkers avoid her asks, "Be honest. Do you find me annoying? Everyone else does." In their sessions, the counselor has felt irritated when the client cuts her off mid-sentence. Which response BEST demonstrates congruence in the client's service?

A. Not at all. I truly enjoy every single minute of our time together in here.

This reassurance contradicts the counselor's actual experience, so it is incongruent and withholds useful feedback.

B. Honestly, yes. You can be pretty exhausting for most people to be around.

This is a global, hurtful judgment rather than a specific, caring observation; genuineness is not the same as bluntness.

C. At times I feel cut off when you jump in. I wonder if others feel that too. ✓ **KEY**

This honestly shares a specific, behavior-based reaction in the here and now and links it to the client's concern, which is genuine and therapeutic.

D. What makes you want to ask me that question at this point in our work?

Exploring the question can be useful, but used here it sidesteps a direct, sincere request for honest feedback.

Rationale: Congruence means the counselor's responses match inner experience, but it is expressed with care and for the client's benefit. Sharing a specific, behavioral reaction in the moment (immediacy) gives the client accurate interpersonal feedback she may not get elsewhere. False reassurance is incongruent, and blunt global judgments are not genuineness but a failure of respect.

References: Corey (Theory & Practice) — Person-centered therapy: congruence; appropriate expression of counselor reactions

☐ Approve ☐ Revise ☐ Retire Notes:

A clinic director wants an early indicator of which clients are likely to benefit from counseling, regardless of the counselor's theoretical approach. Based on psychotherapy outcome research, which measure is the MOST useful early predictor?

A. The specific theoretical orientation the counselor practices

Comparative outcome research finds that bona fide approaches produce broadly similar outcomes, so orientation is a weak predictor.

B. A demographic match between the counselor and the client

Demographic matching has shown inconsistent, generally small effects on outcome compared with the alliance.

C. The total length of the initial intake assessment session

Intake length is not an established predictor of counseling outcome.

D. The client's rating of the working alliance in early sessions ✓ **KEY**

Meta-analyses find that the alliance, especially as rated by the client early in treatment, predicts outcome across approaches.

Rationale: The working alliance is one of the most consistently replicated predictors of counseling outcome. Meta-analyses (e.g., Horvath and colleagues; Flückiger and colleagues) show a moderate, reliable relationship between alliance and outcome across theoretical approaches, and alliance ratings taken early in treatment, often by the third to fifth session, already predict results.

References: Gladding (Counseling: A Comprehensive Profession) — The working alliance and common factors in counseling outcome

► **Check:** Alliance-outcome meta-analytic findings (Horvath et al., 2011; Flückiger et al., 2018) are not themselves in the source library; SME should confirm the Gladding locator covers early alliance as an outcome predictor.

☐ Approve ☐ Revise ☐ Retire Notes:

In the third session of couples counseling, Dana interrupts her wife and says to the counselor, "Every time she talks, you nod along. You're clearly on her side." What should the counselor do FIRST?

A. Acknowledge Dana's experience, invite her to say more, and check in with both partners ✓ KEY

Taking the perceived split seriously and exploring it openly with both partners repairs the alliance with Dana without abandoning her wife.

B. Reassure Dana that counselors stay neutral, then return to where the session left off

Reassurance without exploration dismisses Dana's experience and leaves the split alliance unrepaired.

C. Explain to Dana that her wife's points have simply been clearer and more specific

This confirms Dana's perception that the counselor is siding with her wife and deepens the split.

D. Schedule separate sessions with Dana alone to rebuild her trust before joint work

Moving to private sessions before addressing the issue together can create new imbalances and secrets and delays repair in the room.

Rationale: In couple and family counseling the counselor holds an alliance with each member, and a split alliance, where one member feels less allied with the counselor, is linked to poorer engagement and dropout. The counselor should address the rupture openly and nondefensively in session, maintaining balanced (multipartial) alliances with all members.

References: Goldenberg (Family Therapy) — Therapeutic alliance in couple and family therapy; balanced alliances with each member

► **Check:** Split-alliance research (Pinsof; Friedlander and colleagues, SOFTA) is not itself in the source library; SME should confirm the Goldenberg locator.

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old Diné (Navajo) client being seen for anxiety mentions that her family is arranging a ceremony with a traditional healer next month. Which counselor response BEST reflects cultural humility?

A. Suggest she wait on the ceremony until counseling has had a chance to work

This ranks the counselor's approach above the client's healing tradition and imposes the counselor's worldview.

B. Describe what she has read about Navajo ceremonies to show her knowledge

Taking an expert stance on the client's culture is the opposite of humility and may be inaccurate for this client and family.

C. Ask what the ceremony means to her and how she wants it to fit with counseling ✓ KEY

This positions the client as the expert on her culture and invites collaboration between traditional and counseling approaches.

D. Note it as possible magical thinking to explore further in the assessment

Pathologizing a culturally normative healing practice reflects cultural bias, not humility.

Rationale: Cultural humility is an other-oriented, lifelong stance of self-reflection in which the counselor treats the client as the expert on her own culture and attends to power differences. Sue and Sue encourage counselors to respect and, where the client wishes, collaborate with Indigenous healing practices rather than dismiss or pathologize them.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling Indigenous clients; respecting traditional and Indigenous healing · MSJCC (Ratts et al., 2016) — Counseling relationship: client worldview and cultural healing practices

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old says, "I dropped out of college to stream video games full time. My parents keep calling me lazy." Which counselor response BEST reflects a nonjudgmental stance?

A. Dropping out this young could really limit your options later in life.

This evaluates the decision as a mistake and aligns the counselor with a negative judgment.

B. Your parents may have a point, since streaming is a hard way to earn money.

Siding with the parents judges the client's choice and is likely to harm trust.

C. Good for you. Following your passion is the right call at your age, I think.

Approval is also a judgment; a nonjudgmental stance neither condemns nor endorses the choice.

D. That's a big change. What drew you to streaming, and how is it going so far? ✓ **KEY**

This acknowledges the change and invites the client's own perspective with curiosity, neither praising nor criticizing.

Rationale: A nonjudgmental stance means suspending evaluation of the client's choices, positive or negative, so the client can explore them openly. Curious, open-ended responses keep the focus on the client's experience; both criticism and praise place the counselor's values at the center.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes: _____

199. nce-s-cor-047 · Social and Cultural Diversity · awareness of power and privilege in the counseling relationship · medium · In review

A client who works two hourly jobs has missed two sessions after short-notice shift changes. She apologizes repeatedly and seems afraid of being dropped. The agency discharges clients after three missed sessions. Which counselor response BEST reflects awareness of power and privilege?

A. Name the shift-work barrier and explore flexible times or telehealth together ✓ **KEY**

This recognizes a structural barrier rather than blaming the client and uses the counselor's power to adapt services with her.

B. Remind her of the attendance policy and that one more absence ends services

Leading with the policy uses institutional power punitively and ignores the economic constraints driving the absences.

C. Interpret the missed sessions as ambivalence about change to explore in session

Framing a work-schedule barrier as resistance pathologizes the client and overlooks her circumstances.

D. Suggest she ask her manager for a fixed schedule so she can attend reliably

This assumes a level of workplace control that hourly workers often lack, reflecting the counselor's own privileged frame.

Rationale: The MSJCC ask counselors to recognize how privileged and marginalized statuses, including socioeconomic status, shape the counseling relationship and access to services. Counselors hold institutional power and can use it to reduce barriers rather than interpret structural constraints as client resistance.

References: MSJCC (Ratts et al., 2016) — Counseling relationship; privileged and marginalized statuses and institutional barriers

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor working with a 68-year-old recently widowed man, with his permission, has begun calling his bank, drafting his letters, and booking his appointments. She thinks warmly, "He would be lost without me." Which risk does this pattern MOST directly create?

A. Turning the client's gratitude into hostile negative transference

Nothing in the scenario points to hostility; the more direct risk lies in what the client stops doing for himself.

B. Fostering dependence that undermines the client's own competence ✓ **KEY**

Rescuing driven by the counselor's need to be needed takes over tasks the client could learn to do, encouraging dependence and eroding self-efficacy.

C. Making termination easier because the alliance is so strong

Dependence typically makes termination harder, not easier.

D. Causing the counselor to grow emotionally detached from him

The pattern reflects overinvolvement, not detachment.

Rationale: Countertransference can be positive as well as negative. Corey describes the counselor's need to be needed and rescuing as countertransference that leads counselors to do for clients what clients can do for themselves. The result is dependence and reduced client self-efficacy, which supervision can help the counselor recognize.

References: Corey (Theory & Practice) — Countertransference: the need to be needed and rescuing clients

☐ Approve ☐ Revise ☐ Retire Notes: